

# Medicare Claims Processing Manual

## Chapter 23 - Fee Schedule Administration and Coding Requirements

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## **10 - Reporting ICD Diagnosis and Procedure Codes**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

Proper coding is necessary on Medicare claims because codes are generally used in determining coverage and payment amounts. CMS accepts only HIPAA approved ICD-9-CM or ICD-10-CM/ICD-10-PCS codes, depending on the date of service. The official ICD-9-CM codes which were updated annually through October 1, 2013 are posted at

<http://www.cms.gov/Medicare/Coding/ICD9ProviderDiagnosticCodes/codes.html>

The official annual updates and effective dates for any changes to ICD-10-CM and ICD-10-PCS codes are posted at <http://www.cms.gov/Medicare/Coding/ICD10/index.html> .

See the following sections (10.1 - 10.6) for additional instructions about coding ICD diagnoses for inpatient, outpatient, and other services.

### **10.1 - General Rules for Diagnosis Codes**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

The Official ICD-9-CM Coding Guidelines can be found at

[http://www.cdc.gov/nchs/icd/icd9cm\\_addenda\\_guidelines.htm](http://www.cdc.gov/nchs/icd/icd9cm_addenda_guidelines.htm)

The Official ICD-10-CM and ICD-10-PCS Coding Guidelines can be found with the annual ICD-10-CM and ICD-10-PCS updates at <http://www.cms.gov/Medicare/Coding/ICD10/index.html>

The CMS understands that physicians may not always provide suppliers of DMEPOS with the most specific diagnosis code, and may provide only a narrative description. In those cases, suppliers may choose to utilize a variety of sources to determine the most specific diagnosis code to include on the individual line items of the claim. These sources may include, but are not limited to: coding books and resources, contact with physicians or other health professionals, documentation contained in the patient's medical record, or verbally from the patient's physician or other healthcare professional.

Beneficiaries are not required to submit diagnosis codes on beneficiary-submitted claims. Beneficiary-submitted claims are filed on Form CMS-1490S. For beneficiary-submitted claims, the A/B MAC (B) must develop the claim to determine a current and valid diagnosis code and may enter the code on the claim.

### **10.2 - Inpatient Claim Diagnosis Reporting**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

On inpatient claims providers must report the principal diagnosis. The principal diagnosis is the condition established after study to be chiefly responsible for the admission. Even though another diagnosis may be more severe than the principal diagnosis, the principal diagnosis, as defined above, is entered. Entering any other diagnosis may result in incorrect assignment of a Medicare Severity - Diagnosis Related Group (MS-DRG) and an incorrect payment to a hospital under PPS. See Chapter 25, Completing and Processing the Form CMS-1450 Data Set, for instructions about completing the claim.

Other diagnoses codes are required on inpatient claims and are used in determining the appropriate MS-DRG. The provider reports the full codes for up to twenty four additional conditions if they coexisted at the time of admission or developed subsequently, and which had an effect upon the treatment or the length of stay.

The principal diagnosis should not under any circumstances be duplicated as an additional or secondary diagnosis. If the provider reports duplicate diagnoses they are eliminated in Medicare Code Editor (MCE) before GROUPER.

The Admitting Diagnosis Code is required for inpatient hospital claims subject to A/B MAC (A) review. The admitting diagnosis is the condition identified by the physician at the time of the patient's admission requiring hospitalization. For outpatient bills, the field defined as Patient's Reason for Visit is not required by Medicare but may be used by providers for nonscheduled visits for outpatient bills.

### **10.3 - Outpatient Claim Diagnosis Reporting**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

For outpatient claims, providers report the full diagnosis code for the diagnosis shown to be chiefly responsible for the outpatient services. For instance, if a patient is seen on an outpatient basis for an evaluation of a symptom (e.g., cough) for which a definitive diagnosis is not made, the symptom is reported. If, during the course of the outpatient evaluation and treatment, a definitive diagnosis is made (e.g., acute bronchitis), the definitive diagnosis is reported. If the patient arrives at the hospital for examination or testing without a referring diagnosis and cannot provide a complaint, symptom, or diagnosis, the hospital reports the encounter code that most accurately reflects the reason for the encounter.

Examples include:

- Z00.00 Encounter for general adult medical examination without abnormal findings
- Z00.01 Encounter for general adult medical examination with abnormal findings
- Z01.10 Encounter for examination of ears and hearing without abnormal findings
- Z01.118 Encounter for examination of ears and hearing with other abnormal findings

For outpatient claims, providers report the full diagnosis codes for up to 24 other diagnoses that coexisted in addition to the diagnosis reported as the principal diagnosis. For instance, if the patient is referred to a hospital for evaluation of hypertension and the medical record also documents diabetes, diabetes is reported as another diagnosis.

Additional information and training is available on CMS Web site:

<http://www.cms.gov/Medicare/Coding/ICD10/index.html>

### **10.4 - ICD Procedure Codes**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

ICD procedure codes are required for inpatient hospital Part A claims only. Healthcare Common Procedure Code System (HCPCS) codes are used for reporting procedures on other claim types. Inpatient hospital claims require reporting the principal procedure if a significant procedure occurred during the hospitalization. For information of the selection of the principal procedure, see the Official ICD-10-PCS coding guidelines posted with the annual updates to ICD-10-PCS posted at <http://www.cms.gov/Medicare/Coding/ICD10/index.html>

The principal procedure code and other procedure codes shown on the bill **must** be the full ICD-9-CM, Volume 3, or ICD-10-PCS procedure code, including all applicable digits, up to seven digits.

Up to twenty four significant procedures other than the principal procedure may be reported.

### **10.5 - Coding for Outpatient Services and Physician Offices**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

The Official ICD-10-CM Coding Guidelines include a section for Outpatient Services (hospital-based and physician office). These guidelines can be found in the annual updates to ICD-10-CM posted at <http://www.cms.gov/Medicare/Coding/ICD10/index.html>

A/B MACs (A), (B), (HHH), and DME MACs, physicians, hospitals, and other health care providers must comply with the Official ICD-10-CM Coding Guidelines.

## **10.6 - Relationship of Diagnosis Codes and Date of Service**

**(Rev. 3081, Issued: 09-26-14, Effective: Upon Implementation of ICD-10, Implementation: Upon Implementation of ICD-10)**

Diagnosis codes must be reported based on the date of service (including, when applicable, the date of discharge) on the claim and not the date the claim is prepared or received. A/B MACs (A), (B), (HHH), and DME MACs are required to edit claims on this basis, including providing for annual updates each October.

Shared systems must maintain date parameters for diagnosis editing. Use of actual effective and end dates is required when new diagnosis codes are issued or current codes become obsolete with the annual updates.

The Health Insurance Portability and Accountability Act (HIPAA) requires that medical code sets must be date-of-service compliant. Since ICD diagnosis codes are a medical code set, effective for dates of service on and after October 1, 2004, CMS does not provide any grace period for providers to use in billing discontinued diagnosis codes on Medicare claims. The updated codes are published in the Federal Register each year as part of the Proposed Changes to the Hospital Inpatient Prospective Payment Systems in table 6 and effective each October 1.

All MACs will return claims containing a discontinued diagnosis code as unprocessable. For dates of service beginning October 1, 2004, physicians, practitioners, and suppliers must use the current and valid diagnosis code that is then in effect for the date of service. After the updated codes are published in the Federal Register, CMS places the new, revised and discontinued codes on the ICD-9 or ICD-10 Web site as applicable.

<http://www.cms.gov/Medicare/Coding/ICD9ProviderDiagnosticCodes/codes.html>  
or <http://www.cms.gov/Medicare/Coding/ICD10/index.html>.

The CMS sends the updated codes to All MACs on an annual basis via a recurring update notification instruction. This is normally released to MACs each June, and contains the new, revised, and discontinued diagnosis codes which are effective for dates of service on and after October 1<sup>st</sup>.

## **20 - Description of Healthcare Common Procedure Coding System (HCPCS)**

**(Rev. 10320, Issued: 08-28-2020, Effective: 12-01-2020, Implementation: 12-01-2020)**

The HCPCS has been adopted as the code set for use in Health Insurance Portability and Accountability Act (HIPAA) transactions, for reporting outpatient procedures, items, and services.

The HCPCS originated from the American Medical Association's (AMA) "Physicians' Current Procedural Terminology, Fourth Edition" (CPT-4). It includes two levels of codes and modifiers. Level I contains only the AMA's CPT-4 codes. This level consists of all numeric codes. Level II contains alpha-numeric codes primarily for items and nonphysician services not included in CPT-4, e.g., ambulance, DME, orthotics, and prosthetics. These are alpha-numeric codes maintained by CMS.

The CMS monitors the system to ensure uniformity.

### **20.1 - Use and Maintenance of CPT-4 in HCPCS**

**(Rev. 10320, Issued: 08-28-2020, Effective: 12-01-2020, Implementation: 12-01-2020)**

There are over 7,000 service codes, plus titles and modifiers, in the CPT-4 section of HCPCS, which is copyrighted by the AMA. The AMA and CMS have entered into an agreement that permits the use of HCPCS codes and describes the manner in which they may be used. See §20.7 below.

- The AMA permits CMS, its agents, and other entities participating in programs administered by CMS to use CPT-4 codes/modifiers and terminology as part of HCPCS;
- CMS shall adopt and use CPT-4 in connection with HCPCS for the purpose of reporting services under Medicare and Medicaid;
- CMS agrees to include a statement in HCPCS that participants are authorized to use the copies of CPT-4 material in HCPCS only for purposes directly related to participating in CMS programs, and that permission for any other use must be obtained from the AMA;
- HCPCS shall be prepared in format(s) approved in writing by the AMA, which include(s) appropriate notice(s) to indicate that CPT-4 is copyrighted material of the AMA;
- Both the AMA and CMS will encourage health insurance organizations to adopt CPT-4 for the reporting of physicians' services in order to achieve the widest possible acceptance of the system and the uniformity of services reporting;
- The AMA recognizes that CMS and other users of CPT-4 may not provide payment under their programs for certain procedures identified in CPT-4. Accordingly, CMS and other health insurance organizations may independently establish policies and procedures governing the manner in which the codes are used within their operations; and
- The AMA's CPT-4 Editorial Panel has the sole responsibility to revise, update, or modify CPT-4 codes.

The AMA updates and republishes CPT-4 annually and provides CMS with the updated data. The CMS updates the alpha-numeric (Level II) portion of HCPCS and incorporates the updated AMA material to create the HCPCS code file. The CMS provides the file to A/B MACs (A), (B), (HHH), and DME MACs and Medicaid State agencies. The file is also available via the CMS website.

It is the MAC's responsibility to develop payment screens and limits within Federal guidelines and to implement CMS' issuances. The coding system is one of the tools used to achieve national consistency in claims processing.

MACs may edit and abridge CPT-4 terminology within their claims processing area. However, MACs are not allowed to publish, edit, or abridge versions of CPT-4 for distribution outside of the claims processing structure. This would violate copyright laws. MACs may furnish providers/suppliers AMA and CMS Internet addresses, and may issue newsletters with codes and approved narrative descriptions that instruct physicians, suppliers and providers on the use of certain codes/modifiers when reporting services on claims forms, e.g., need for documentation of services, handling of unusual circumstances. The CMS acknowledges that CPT is a trademark of the AMA, and the newsletter must show the following statement in close proximity to listed codes and descriptors:

"CPT Copyright 2017 American Medical Association. All rights reserved.  
CPT® is a registered trademark of the American Medical Association."

(Please note that the use or reprinting of CPT content in any product or publication requires a license. For more information about the licenses available, see CPT License Information.)

Applicable FARS/DFARS Restrictions Apply to Government Use

Fee schedules, relative value units, conversion factors and/or related components are not assigned by the AMA, are not part of CPT, and the AMA is not recommending their use. The AMA does not directly or indirectly practice medicine or dispense medical services. The AMA assumes no liability for data contained or not contained herein.

If only a small portion of the terminology is used, MACs do not need to show the copyright legend. MACs may also print the code and approved narrative description in development requests relating to individual cases.

The CMS provides MACs a quarterly update file of HCPCS codes and instructions to retrieve the update via CMS mainframe telecommunication system.

## **20.2 – RESERVED**

### **20.3 - Use and Acceptance of HCPCS Codes and Modifiers**

**(Rev. 10320, Issued: 08-28-2020, Effective: 12-01-2020, Implementation: 12-01-2020)**

The HCPCS is updated quarterly to reflect changes in the practice of medicine and provision of health care. The CMS provides a file containing the updated HCPCS codes to A/B MACs (A), (B), (HHH), and DME MACs and Medicaid State agencies 60 to 90 days in advance of the implementation of each update. Distribution consists of an electronic file of the updated HCPCS codes, file characteristics, record layout, and a listing of added, revised and deleted codes. MACs are required to update their HCPCS codes file and map all new, revised, and deleted codes to appropriate payment information.

An updated list of the HCPCS codes for Durable Medical Equipment Medicare Administrative Contractors (DME MAC) and A/B MAC (B) jurisdictions is updated quarterly to reflect codes that have been added or discontinued (deleted) during each year. A recurring update notification will be published to notify the DME MACs and A/B MACs (B) that the list has been updated and is available on the CMS Web site. <https://www.cms.gov/Medicare/Coding/HCPCSReleaseCodeSets/Alpha-Numeric-HCPCS.html>.

Both the DME MACs and the A/B MACs (B) publish this list to educate providers on which MAC they should bill for codes provided on this list.

Physicians and suppliers must use HCPCS codes on the Form CMS-1500 or its electronic equivalent and providers must use HCPCS codes on the Form CMS-1450 or its electronic equivalent for most outpatient services. The procedure, item, or service can be further described by using 2-position modifiers contained in HCPCS.

Modifiers to HCPCS Level I codes for medicine, anesthesia, surgery, radiology, and pathology are on the HCPCS codes file from CMS. Modifiers for Level II alpha-numeric codes are with the Level II codes published by CMS. Alpha-numeric and CPT-4 modifiers may be used with either alpha-numeric or CPT-4 codes. A/B MACs (A, B and HHH) and DME MACs are required to accept up to four 2-position numeric or alpha modifiers and process both modifiers completely through the claims processing system (including any manual portion) as far as payment history. It is not acceptable merely to be able to accept multiple modifiers and then drop one before complete systems processing. Dropping of a modifier leads to incomplete and inaccurate pricing profiles.

Series “Q,” “K,” and “G” in the Level II coding are reserved for CMS assignment. “Q,” “K,” and “G” codes are **temporary** national codes for items or services requiring uniform national coding between one year’s update and the next. Sometimes “temporary” codes remain for more than one update. If “Q,” “K,” or “G” codes are not converted to permanent codes in the Level I or Level II series in the following update, they will remain active until converted or until CMS notifies MACs to delete them. All active “Q,” “K,” and “G” codes at the time of update will be included on the update file for MACs. In addition, deleted codes are retained on the file for informational purposes, with a deleted indicator, for four years.

Series “S” Level II codes are reserved for use by the BCBSA and other private insurers. These codes provide for reporting needs unique to those organizations. Each State defines its own Medicaid coverage, payment, and utilization levels. The CMS does not impose Medicare requirements on Medicaid programs. The HCPCS simply provides a system for identifying services that can be expanded to meet everyone’s needs.

## **20.4 - Deleted HCPCS Codes/Modifiers**

**(Rev. 10320, Issued: 08-28-2020, Effective: 12-01-2020, Implementation: 12-01-2020)**

Claims for services in a prior year are reported and processed using the HCPCS codes/modifiers in effect during that year. For example, a claim for a service furnished in November 2019 but received by an A/B MAC (A), (B), or (HHH), or DME MAC in 2020, should contain codes/modifiers valid in 2019 and is processed using the prior year’s pricing files.

HCPCS codes (Level I CPT-4 and Level II alpha-numeric) are updated on a quarterly basis. Each quarter (January, April, July and October), CMS releases the HCPCS file to A/B MACs (A), (B), (HHH), and DME MACs. The HCPCS file contains the CPT-4 and the alpha-numeric updates. All MACs are notified of the release date via a one-time notification instruction. The file contains new, deleted, and revised HCPCS codes which are effective for the upcoming quarter.

The Health Insurance Portability and Accountability Act (HIPAA) requires that medical code sets must be date of service compliant.

Providers can purchase the American Medical Association’s CPT-4 coding book that is published each October that contains new, revised, and discontinued CPT-4 codes for the upcoming year. In addition, CMS posts on its Web site the alpha-numeric HCPCS file. Providers are encouraged to access CMS web site to see the new, revised, and discontinued alpha-numeric codes. The CMS web site to view the HCPCS update is <https://www.cms.gov/Medicare/Coding/HCPCSReleaseCodeSets/Alpha-Numeric-HCPCS.html>.

A/B MACs (B) and DME MACs must reject services submitted with discontinued HCPCS codes. A/B MACs (A) and (HHH) must return to the provider (RTP) claims containing deleted codes.

See the Medicare Claims Processing Manual, Chapter 22, “Remittance Notices to Providers.”

## **20.5 - The HCPCS Codes Training**

**(Rev. 1, 10-01-03)**

**HO-442.5**

A large number of changes to HCPCS codes may necessitate the training of physician, supplier and provider personnel. If this is necessary, MACs must schedule the sessions in conjunction with the State medical records association or the State hospital or medical/physician associations. All MACs should schedule sessions in the major population centers of their service area.

All MACs should use the following guidelines in planning training sessions.

- Limit the training sessions to a manageable size to encourage questions and answers.
- A/B MACs (B) should limit sessions to office staffs of homogenous medical specialties, e.g., do not include DME suppliers with staff from physicians’ offices.
- Invite office personnel who complete bills and claims to attend.
- In conducting the training, emphasize the use of the new HCPCS/CPT-4 manuals and the proper completion of the claim forms using the new codes.

Be alert to a specific physician/supplier/provider having difficulty with HCPCS. Provide follow up training focused to the individual situation. Review and discuss specific billing problems over the phone or by mail. If many problems develop with a specific provider, the MAC schedules a visit to provide the specific training needed. Training is the A/B MACs (A)'s, (B)'s, (HHH)'s, and DME MACs' responsibility.

## **20.6 - Professional/Public Relations for HCPCS**

**(Rev. 1, 10-01-03)**

A/B MACs (A), (B), (HHH), and DME MACs must perform the following professional/public relations activities:

- Designate a knowledgeable person as a focal point for physician, supplier and provider inquiries. This person must be able to address coding and payment questions.
- Notify physicians, suppliers and providers by newsletter and Web site at least 30 days before codes are changed, added or deleted. Also, notify them when general problems arise including coding issues.
- A/B MACs (B) should share Level III codes/modifiers with the Medicaid State agency in the A/B MAC (B)'s jurisdiction.
- Advise physicians and providers to obtain new copies of CPT-4 annually from the AMA. Orders can be placed by calling 1-800-621-8335 or order online at [www.ama-assn.org/catalog](http://www.ama-assn.org/catalog)
- A/B MACs (A) and (HHH) supply providers with CMS' alpha-numeric code updates and with any local codes. See §20 for sources. A/B MACs (B) supply the current HCPCS local codes/modifiers (alpha-numeric W-Z) since they are the only source.
- A/B MACs (A), (B), (HHH), and DME MACs inform physicians, suppliers, and providers when the annually and quarterly updated HCPCS codes become available and effective in the claims processing system.
- All codes/modifiers contained in the current version of the HCPCS will be recognized and processed unless they have been deleted or are indicated as not valid for Medicare.
- It is important for physicians, practitioners, suppliers, and providers to note that code/modifier recognition does not imply that a service is covered by Medicare. In addition, a separate code does not mean that the payment level will be different from similar services identified by different codes.
- Since local changes can occur throughout the year, continue professional relations activities to provide as much information as possible to providers.

## **20.7 - Use of the American Medical Association's (AMA's) Physicians' Current Procedural Terminology (CPT) Fourth Edition Codes, and Use of the American Dental Association's (ADA's) Current Dental Terminology-Fourth Edition (CDT) Codes, on A/B MACs (A)'s, (B)'s, (HHH)'s, and DME MACs' Web Sites and Other Electronic Media**

**(Rev. 12573, Issued: 04-09-24; Effective:08-10-20; Implementation: 08-10-20)**

See Business Requirements at [http://www.cms.hhs.gov/manuals/pm\\_trans/R43CP.pdf](http://www.cms.hhs.gov/manuals/pm_trans/R43CP.pdf)

## **CMS AMA-ADA-AHA Medical Codes**

### **Overview**

The Centers for Medicare & Medicaid Services (CMS) staff may access the following **copyright protected** medical codes and descriptors:

- American Medical Association's Current Procedural Terminology – AMA/CPT
- American Dental Association's Current Dental Terminology – ADA/CDT
- American Hospital Association's National Uniform Billing Codes – AHA/NUBC UB-04

CMS signed licensing agreements with the AMA, ADA, and AHA giving staff limited use of CPT, CDT, and NUBC data codes and descriptors.

### **Access the CPT, CDT, or NUBC codes**

Prior to accessing the codes, CMS staff must accept the terms and conditions of an association's licensing agreement.

- American Medical Association's Current Procedural Terminology – AMA/CPT
  - [AMA Terms and Conditions](#)
  - [AMA CPT Exhibits Directory](#)
- American Dental Association's Current Dental Terminology – ADA/CDT
  - [ADA Terms and Conditions](#)
  - [ADA CDT Exhibits Directory](#)
- American Hospital Association's National Uniform Billing Codes – AHA/NUBC
  - [AHA Terms and Conditions](#)
  - [AHA NUBC Exhibits Directory](#)

Any proposed usage not covered by the current agreement requires the completion of a [https://surveys.cms.gov/jfe/form/SV\\_4UsBJoFDNs9qQD4](https://surveys.cms.gov/jfe/form/SV_4UsBJoFDNs9qQD4). CMS, then, will negotiate your “new use” with the appropriate association.

## **20.8 - Payment, Utilization Review (UR), and Coverage Information on CMS Quarterly HCPCS Codes Update File** (Rev. 10320, Issued: 08-28-2020, Effective: 12-01-2020, Implementation: 12-01-2020)

The file CMS provides for the quarterly update of HCPCS codes contains fields for payment, UR, and coverage information to assist in developing front-end edit screens. Coverage information is not all inclusive, but should be used mainly as a guide in establishing specific review limits. A/B MACs (B) must establish reasonable developmental guidelines, review screens, and relative value units, as appropriate. A/B MACs (B) must assure that their system processes claims in accordance with CMS policies and procedures, including changes that may occur between HCPCS codes updates.

Where CMS determines that nationally uniform temporary codes/modifiers are needed to implement policy/legislation between HCPCS codes updates, the codes/modifiers, definitions and policy are issued as Level II codes/modifiers prefixed with “Q” or “K” or “G.” Questions may arise in updating that require A/B MAC (B) staff to refer to a physician's or supplier's pricing history. Therefore, keep an electronic backup of HCPCS codes for the two prior years with linkages to pricing profiles. Perform required computer analysis as necessary.

The HCPCS terminology seldom includes a place of service designation. Where place of service affects pricing, pricing is obtained from the place of service field on the claim record.

A/B MACs (A) and (HHH) also develop editing screens using HCPCS based on payment and coverage policies from CMS. A/B MAC

## **20.9 - National Correct Coding Initiative (NCCI)**

*(Rev. 14428, Issued:06-16-26; Effective:07-16-26; Implementation:07-16-26)*

The Centers for Medicare & Medicaid Services (CMS) developed the NCCI program to promote national correct coding methodologies and to control improper coding leading to inappropriate payment in Part B claims. CMS developed its coding policies based on coding conventions defined in the American Medical Association's *CPT Professional codebook*, CMS national and local policies and edits, coding guidelines developed by national societies, analysis of standard medical and surgical practices, and a review of current coding practices. An overview of the NCCI program for Procedure-to-Procedure (PTP) edits, Medically Unlikely Edits (MUEs), Add-on Code (AOC) edits and additional information sources are found on the [Medicare NCCI Edits webpage](#).

The [Medicare NCCI Policy Manual](#) (also known as the Coding Policy Manual) shall be used by Medicare Administrative Contractors (MACs) as a general reference tool that explains the rationale for NCCI edits.

The purpose of the NCCI PTP edits is to prevent improper payment when incorrect Healthcare Common Procedure Coding System (HCPCS)/Current Procedural Terminology (CPT) code combinations are reported. The [Medicare NCCI Edits webpage](#) contains separate tables of edits for physicians/practitioners and outpatient hospital services. Additional information regarding types of tables is available in the [How to Use The Medicare National Correct Coding Initiative \(NCCI\) Tools](#) MLN booklet.

The purpose of the NCCI MUE program is to prevent improper payments when services are reported with incorrect units of service (UOS). CMS developed MUEs to reduce the paid claims error rate for Part B claims. An MUE for a HCPCS/CPT code is the maximum UOS that a provider or supplier would report under most circumstances for a single beneficiary on a single date of service. The [Medicare NCCI Medically Unlikely Edits \(MUEs\) webpage](#) contains separate tables of edits for physicians/practitioners, outpatient hospital services, and durable medical equipment.

An AOC is a HCPCS/CPT code that describes a service that, with rare exception, is performed in conjunction with another primary service by the same practitioner. An AOC is rarely eligible for payment if it is the only procedure reported by a practitioner.

CMS posted the Correspondence Language Manual for Medicare Services on the [Medicare NCCI Correspondence Language Manual webpage](#) for use by the Medicare Contractors to answer routine correspondence inquiries about the NCCI PTP and MUE edits. The general correspondence language paragraphs explain the rationale for the edits. The section-specific examples add further explanation to the PTP or MUE edits and are sorted by edit rationale and HCPCS/CPT code section (00000, 10000, 20000, etc.). Please refer to the Introduction of the Correspondence Language Manual for additional guidance about its use.

### **20.9.1 - Correct Coding Modifier Indicators (CCMI) and HCPCS Codes Modifiers**

*(Rev . 10878, Issued: 07-15-2021, Effective:08-16-2021; Implementation:08-16-2021)*

The National Correct Coding Initiative (NCCI) File Formats continue to include a Correct Coding Modifier Indicator (CCMI) for the Column One / Column Two Correct Coding edit file. This indicator determines whether an NCCI PTP-associated modifier causes the code pair to bypass the edit. The CCMI will be either a "0," "1," or a "9." The definitions of each are:

0 = an NCCI PTP-associated modifier is not allowed and will not bypass the edit.

1 = an NCCI PTP-associated modifier is allowed and will bypass the edit.

9 = The use of NCCI PTP-associated modifiers is not specified. This indicator is used for all code pairs that have a deletion date that is the same as the effective date. This indicator prevents blank spaces from appearing in the indicator field.

### **20.9.1.1 - Instructions for Codes With Modifiers (A/B MACs (B) Only)** **(Rev. 13216; Issued:05-09-25; Effective: 06-03-25; Implementation:06-03-25)**

#### **A. General**

MACs subject all line items for the same beneficiary, same NPI, and same date of service to NCCI edits.

All line items for the same beneficiary, same NPI, and same date of service shall be subject to NCCI PTP edits. If the CCMI of a PTP edit is “0”, the Column Two code is not eligible for payment even if an NCCI PTP-associated modifier is appropriately appended to one of the codes. If the CCMI of a PTP edit is “1”, the edit may be bypassed and the Column Two code of the edit may be eligible for payment if an NCCI PTP-associated modifier is appropriately appended to one of the codes. If the 2 codes of a code pair edit have the same NCCI PTP-associated anatomic modifier, the edit will not be bypassed unless an additional NCCI PTP-associated modifier is appended to 1 of the codes indicating the reason to bypass the edit.

The use of modifiers that are not NCCI PTP-associated modifiers shall not bypass an NCCI PTP edit.

NCCI PTP-associated modifiers are the following:

Anatomic modifiers : E1-E4, FA, F1-F9, TA, T1-T9, LT, RT, LC, LD, RC, LM, RI

Global surgery modifiers: 24, 25, 57, 58, 78, 79

Other modifiers: 27, 59, 91, XE, XS, XP, XU

#### **B. Modifiers 59 or XE, XP, XS, XU**

Modifiers 59 or XE, XP, XS, XU and other NCCI PTP-associated modifiers shall not be used to bypass a PTP edit unless the proper criteria for use of the modifier are met. Documentation in the medical record must satisfy the criteria required by any NCCI PTP-associated modifier that is used. Find further information on modifiers 59 or XE, XP, XS, XU in the Coding Policy Manual available on the CMS website.

Use of modifiers 59 or XE, XP, XS, XU does not require a different diagnosis for each HCPCS/CPT coded procedure. Conversely, different diagnoses are not adequate criteria for use of modifiers 59 or XE, XP, XS, XU.

Modifiers 59 or XE, XP, XS, XU shall not be used with the following codes:

- 77427 Radiation treatment management, 5 treatments
- Evaluation & Management (E&M) services

When a provider or supplier submits a claim for any of the codes specified above with modifiers 59 or XE, XP, XS, XU, the A/B MAC must process the claim as if the modifier were not present. In addition to those messages specified in §20.9.1 above, A/B MACs shall convey additional messaging per instructions in Pub. 100-09, Chapter 6 and Pub. 100-04, Chapter 22.

Examples of appropriate use of modifiers 59 and XE, XP, XS, XU can be found in the Fact Sheet [Proper Use of Modifiers 59, XE, XP, XS, & XU](#).

1. Modifier 59 or XE are used appropriately when the procedures are performed in different encounters on the same day.

2. Modifier 59 or XP are used appropriately when the procedures are performed by different practitioners.

3. Modifier 59 or XS are used appropriately for different anatomic sites during the same encounter only when procedures are performed on different organs, or different anatomic regions, or in limited situations on different, non-contiguous lesions in different anatomic regions of the same organ.

4. Other specific appropriate uses of modifiers 59 or XE, XU.

There are 3 other limited situations in which 2 services may be reported as separate and distinct because they are separated in time and describe non-overlapping services even though they may occur during the same encounter.

a. Modifier 59 or XE is used appropriately for 2 services described by timed codes provided during the same encounter only when they are performed sequentially. There is an appropriate use for modifiers 59 or XE that is applicable only to codes for which the unit of service is a measure of time (e.g., per 15 minutes, per hour). If 2 timed services are provided in blocks of time that are separate and distinct (i.e., the same time block is not used to determine the unit of service for both codes), modifier 59 may be used to identify the services.

b. Modifier 59 or XU is used appropriately for a diagnostic procedure, which precedes a therapeutic procedure only when the diagnostic procedure is the basis for performing the therapeutic procedure. When a diagnostic procedure precedes a surgical procedure or on-surgical therapeutic procedure and is the basis on which the decision to perform the surgical procedure is made, that diagnostic test may be considered to be a separate and distinct procedure as long as (a) it occurs before the therapeutic procedure and is not interspersed with services that are required for the therapeutic intervention; (b) it clearly provides the information needed to decide whether to proceed with the therapeutic procedure; (c) it does not constitute a service that would have otherwise been required during the therapeutic intervention; and (d) it is not specifically prohibited. If the diagnostic procedure is an inherent component of the surgical procedure, it should not be reported separately.

c. Modifier 59 or XU is used appropriately for a diagnostic procedure, which occurs subsequent to a completed therapeutic procedure only when the diagnostic procedure is not a common, expected, or necessary follow-up to the therapeutic procedure. When a diagnostic procedure follows the surgical procedure or non-surgical therapeutic procedure, that diagnostic procedure may be considered to be a separate and distinct procedure as long as (a) it occurs after the completion of the therapeutic procedure and is not interspersed with or otherwise commingled with services that are only required for the therapeutic intervention, and (b) it does not constitute a service that would have otherwise been required during the therapeutic intervention. If the post-procedure diagnostic procedure is an inherent component or otherwise included (or not separately payable) post-procedure service of the surgical procedure or non-surgical therapeutic procedure, it should not be reported separately.

5. Modifiers 59 or XE, XP, XS, XU are used inappropriately if the basis for their use is that the narrative description of the 2 codes is different.

## C. Modifier 91

Modifier 91 may be appended to laboratory procedure(s) or service(s) to indicate a repeat test or procedure on the same day when appropriate. If a HCPCS/CPT code has an MUE that is adjudicated as a claim line edit, (i.e., MUE Adjudication Indicator (MAI) equal to "1") appropriate use of CPT modifiers (i.e., 59 or XE, XP, XS, XU, 76, 77, 91, anatomic) may be used to report the same HCPCS/CPT code on separate lines of a claim. This modifier indicates to the Medicare contractors that the physician had to perform a repeat clinical diagnostic laboratory test that was distinct or separate from a lab panel or other lab services performed on the same day, and was performed to obtain medically necessary subsequent reportable test values. This modifier must not be used to report repeat laboratory testing due to laboratory errors, quality control, or confirmation of results.

For example, if a laboratory performs all tests included in a panel of laboratory tests and repeats one of these component tests as a medically reasonable and necessary service on the same date of service, the HCPCS/CPT code corresponding to the repeat laboratory test may be reported with modifier 91 appended.

## D. Reserved for future use

## E. Coding for Noncovered Services and Services Not Reasonable and Necessary

For information on this topic, see the Claims Processing Manual, Chapter 1 and MLN Booklet: Medicare Advance Written Notices of Noncoverage ICN 006266.

## Use of HCPCS Code A9270

HCPCS code A9270 (Non-covered item or service), will not be accepted under any circumstances for services or items billed to A/B MACs. However, in cases where there is no specific procedure code for an item or supply and no appropriate Not Otherwise Classified (NOC) code available, A9270 must continue to be used by suppliers to bill DME MACs for statutorily non-covered items or supplies and items or supplies that do not meet the definition of a Medicare benefit.

## Claims Processing Instructions

At A/B MAC and DME MAC discretion, claims submitted using the GY modifier may be auto denied. If the GZ and GA modifiers are submitted for the same item or service, treat the item or service as having an invalid modifier and therefore unprocessable.

Effective for dates of service on and after July 1, 2011, A/B MACs shall automatically deny claim line(s) items submitted with a GZ modifier. A/B MACs shall not perform complex medical review on claim line(s) items submitted with a GZ modifier. All MACs shall make all language published in educational outreach materials, articles, and on their websites, consistent to state all claim line(s) items submitted with a GZ modifier shall be denied automatically and will not be subject to complex medical review.

### **20.9.2 - Reserved for future use**

**(Rev. 4465; Issued: 11-15-19, Effective: 01-30-19, Implementation: 01-30-19)**

### **20.9.3 – Appeals**

**(Rev. 13216; Issued:05-09-25; Effective: 06-03-25; Implementation:06-03-25)**

When a request for review is received as a result of an initial determination based on a correct coding initiative edit, and after determining that the reviews were coded correctly, the reviewer must come to the same conclusion as the initial determination (i.e., the review does not result in an increase in payment). If the review determines that a correct coding modifier not submitted with the initial claim could have been appended to either code of an edit code pair, the reviewer may change the initial determination only if the

correct coding initiative edit has a modifier indicator of “1.” If the correct coding initiative edit modifier indicator is a “0,” the reviewer must come to the same conclusion as the initial determination. If the conclusion is the same as the initial determination, the review determination must repeat the generic language that appears in the Medicare Summary Notice (MSN) or remittance advice notice pertaining to the correct coding edit. In addition, MACs must include the more detailed explanation of the correct coding initiative edit which can be found in the standard correspondence language for MACs in the Medicare NCCI Correspondence Language Manual on the Medicare NCCI Edits webpage.

- MUEs are set high enough to allow for medically likely daily frequencies of services provided in most settings. Because MUEs are based on current coding instructions and practices, MUEs are prospective edits applicable to the time period for which the edit is effective. A change in an MUE is not retroactive and has no bearing on prior services unless specifically updated with a retroactive effective date. In the unusual case of a retroactive MUE change, MACs are not expected to identify claims but should reopen impacted claims that providers or suppliers bring to their attention.
- Since MUEs are auto-deny edits, denials may be appealed. Appeals shall be submitted to the appropriate MAC not the NCCI contractor. MACs adjudicating an appeal for a claim denial for a HCPCS/CPT code with an MAI of “1” or “3” may pay correctly coded correctly counted medically necessary UOS in excess of the MUE value.
- A denial of services due to an MUE is a coding denial, not a medical necessity denial. The presence of an ABN shall not shift liability to the beneficiary for UOS denied based on an MUE. If during reopening or redetermination medical records are provided with respect to an MUE denial for an edit with an MAI of “3”, MACs will review the records to determine if the provider or supplier actually furnished units in excess of the MUE, if the codes were used correctly, and whether the services were medically reasonable and necessary. If the units were actually provided, but one of the other conditions is not met, a change in denial reason may be warranted (for example, a change from the MUE denial based on incorrect coding to a determination that the item/service is not reasonable and necessary under section 1862(a)(1)). This may also be true for certain edits with an MAI of “1.” The CMS interprets the notice delivery requirements under Section 1879 of the Social Security Act (the Act) as applying to situations in which a provider or supplier expects the initial claim determination to be a reasonable and necessary denial. Consistent with NCCI program guidance, denials resulting from MUEs are not based on any of the statutory provisions that give liability protection to beneficiaries under section 1879 of the Social Security Act. Thus, ABN issuance based on an MUE is NOT appropriate. A provider or supplier may not issue an ABN in connection with services denied due to an MUE and cannot bill the beneficiary for UOS denied based on an MUE.
- If a procedure is performed bilaterally and the HCPCS/CPT code descriptor does not state that it is a unilateral or bilateral procedure, report bilateral surgical procedures on a single claim line with modifier 50 and one (1) unit of service. For specific instructions for Ambulatory Surgical Centers, see the Medicare Claims Processing Manual, Chapter 14, Section 40.5.

When modifier 50 is required by manual or coding instructions, claims submitted with 2 lines or 2 units and anatomic modifiers will be denied for incorrect coding. MACs may reopen or allow resubmission of those claims in accordance with their policies and with the policy in the Medicare Claims Processing Manual, Chapter 34, Section 10.1.

Clerical errors (which include minor errors and omissions) may be treated as reopenings.

- Providers or suppliers may change and resubmit their own claims where possible but during reopening MACs may, when necessary, correct the claim to modifier 50 from an equivalent 2 units of bilateral anatomic modifiers. The original submitted version of the claim is retained in the Medicare IDR (Integrated Data Repository).
- Providers or suppliers shall use anatomic modifiers (e.g., RT, LT, FA, F1-F9, TA, T1-T9, E1-E4) and report procedures with differing modifiers on individual claim lines when appropriate. Many MUEs are based on the assumption that correct modifiers are used.
- A/B MACs shall include with the review determination the more detailed explanation of the correct coding initiative edit, which can be found in the standard correspondence language for A/B MACs in the Medicare NCCI Correspondence Language Manual.
- MACs shall assign MSN 15.6. CARC 151 with Group Code CO for claims that fail the MUE edits, when the UOS on the claim exceeds the MUE value, and deny the entire claim line(s) for the relevant HCPCS/CPT code.

### **20.9.3.1- Procedure-to-Procedure (PTP) Edits**

*(Rev. 14428, Issued:06-16-26; Effective:07-16-26; Implementation:07-16-26)*

All PTP edits have a CCMI.

A denial of services due to a PTP edit is a coding denial, not a medical necessity denial. The presence of an Advance Beneficiary Notice (ABN) shall not shift liability to the beneficiary for UOS denied based on a PTP.

PTP edits with a CCMI of “0”:

On appeal, if the CCMI is a “0”, and the provider or supplier coded the claim correctly, there are no circumstances in which both procedures of the PTP code pair should be paid for the same beneficiary on the same day by the same provider or supplier. If the reviewer determines that the claim was coded incorrectly then, the review determination must repeat the generic language that appears in the MSN or remittance advice notice pertaining to the NCCI edit. In addition, MACs must include the more detailed explanation of the NCCI edit, which can be found in the standard correspondence language for MACs in the [Medicare NCCI Correspondence Language Manual](#).

PTP edits with a CCMI of “1”:

On appeal, if the correct coding initiative edit modifier indicator is a “1”, the reviewer must determine whether the claim was coded correctly. For example, the reviewer should determine whether the provider or supplier reported an incorrect code, a medically unnecessary service, or simply neglected to use a modifier. The reviewer may change the initial determination only if the correct coding initiative edit has a modifier indicator of “1” and the reviewer determines that an NCCI *PTP*-associated modifier could have been appended to either code of a correctly coded edit code pair. If the reviewer determines that the claim was coded incorrectly then, the review determination must repeat the generic language that appears in the MSN or remittance advice notice pertaining to the NCCI edit. In addition, MACs must include the more detailed explanation of the NCCI edit, which can be found in the standard correspondence language for MACs in the [Medicare NCCI Correspondence Language Manual](#).

## General Instructions on PTPs:

- MACs shall assign CARC 236 with Group Code CO and MSN 16.8 for claims that fail the PTP edits, and deny when this procedure or procedure/modifier combination is not compatible with another procedure or procedure/modifier combination provided on the same day according to the NCCI program or workers compensation state regulations/ fee schedule requirements.

### 20.9.3.2- Medically Unlikely Edits (MUEs)

*(Rev. 14428, Issued:06-16-26; Effective:07-16-26; Implementation:07-16-26)*

All HCPCS/CPT codes with MUE values have an MAI.

MUEs for HCPCS/CPT codes with an MAI of “1”: MUEs for HCPCS/CPT codes with an MAI of “1” will be adjudicated as a claim line edit.

MUEs for HCPCS/CPT codes with an MAI of “2”: MUEs for HCPCS/CPT codes with an MAI of “2” are absolute date of service edits. These are “per day edits based on policy”. HCPCS/CPT codes with an MAI of “2” have been rigorously reviewed and vetted within CMS and obtain this MAI designation because UOS on the same date of service in excess of the MUE value would be considered contrary to statute, regulation, or subregulatory guidance. Subregulatory guidance includes clear correct coding policy that is binding on both providers or suppliers and the MACs. As stated in [CR 8853](#), while Qualified Independent Contractors (QICs) are not bound by subregulatory guidance, they should understand the policy nature of the MAI “2” indicator when considering whether to pay UOS in excess of the MUE value if claim denials based on these edits are appealed.

Limitations created by anatomical or coding restrictions are incorporated in correct coding policy, both in the Health Insurance Portability & Accountability Act of 1996 (HIPAA) mandated coding descriptors and CMS approved coding guidance as well as specific guidance in the CMS and NCCI Policy manuals. For example, it would be contrary to correct coding policy to report more than 1 unit of service for "ventilation assist and management . . . initial day" because such usage could not accurately describe 2 initial days of management occurring on the same date of service as would be required by the code descriptor.

**CMS** establishes edits with an MAI of “2” based directly on regulation, statute or subregulatory guidance.

MUEs for HCPCS/CPT codes with an MAI of “3”: MUEs for HCPCS/CPT codes with an MAI of “3” are date of service edits. These are “per day edits based on clinical benchmarks”. If claim denials based on these edits are appealed, MACs may pay UOS in excess of the MUE value if there is adequate documentation of medical necessity of correctly reported units. If MACs have pre-payment evidence (e.g. medical review) that UOS in excess of the MUE value were actually provided, were correctly coded, and were medically necessary, the MACs may bypass the MUE for a HCPCS/CPT code with an MAI of “3” during claim processing, reopening, or redetermination, or in response to effectuation instructions from a reconsideration or higher level appeal.

## General Instructions on MUEs:

- MUEs are set high enough to allow for medically likely daily frequencies of services provided in most settings. Because MUEs are based on current coding instructions and practices, MUEs are prospective edits applicable to the time period for which the edit is effective. A change in an MUE is not retroactive and has no bearing on prior services unless specifically updated with a retroactive effective date. In the unusual case of a retroactive MUE change, MACs are not expected to identify claims but should reopen impacted claims that providers or suppliers bring to their attention.

- Since MUEs are auto-deny edits, denials may be appealed. Appeals shall be submitted to the appropriate MAC not the NCCI contractor. MACs adjudicating an appeal for a claim denial for a HCPCS/CPT code with an MAI of “1” or “3” may pay correctly coded correctly counted medically necessary UOS in excess of the MUE value.
- A denial of services due to an MUE is a coding denial, not a medical necessity denial. The presence of an ABN shall not shift liability to the beneficiary for UOS denied based on an MUE. If during reopening or redetermination medical records are provided with respect to an MUE denial for an edit with an MAI of “3”, MACs will review the records to determine if the provider or supplier actually furnished units in excess of the MUE, if the codes were used correctly, and whether the services were medically reasonable and necessary. If the units were actually provided, but one of the other conditions is not met, a change in denial reason may be warranted (for example, a change from the MUE denial based on incorrect coding to a determination that the item/service is not reasonable and necessary under section 1862(a)(1)). This may also be true for certain edits with an MAI of “1.” CMS interprets the notice delivery requirements under Section 1879 of the Social Security Act (the Act) as applying to situations in which a provider or supplier expects the initial claim determination to be a reasonable and necessary denial. Consistent with NCCI program guidance, denials resulting from MUEs are not based on any of the statutory provisions that give liability protection to beneficiaries under section 1879 of the Social Security Act. Thus, ABN issuance based on an MUE is NOT appropriate. A provider or supplier may not issue an ABN in connection with services denied due to an MUE and cannot bill the beneficiary for UOS denied based on an MUE.
- If a procedure is performed bilaterally and the HCPCS/CPT code descriptor does not state that it is a unilateral or bilateral procedure, report bilateral surgical procedures on a single claim line with modifier 50 and one (1) unit of service. For specific instructions for Ambulatory Surgical Centers, see the Medicare Claims Processing Manual, [Chapter 14](#), Section 40.5.

When modifier 50 is required by manual or coding instructions, claims submitted with 2 lines or 2 units and anatomic modifiers will be denied for incorrect coding. MACs may reopen or allow resubmission of those claims in accordance with their policies and with the policy in the Medicare Claims Processing Manual, [Chapter 34](#), Section 10.1.

Clerical errors (which include minor errors and omissions) may be treated as reopenings.

- Providers or suppliers may change and resubmit their own claims where possible but during reopening MACs may, when necessary, correct the claim to modifier 50 from an equivalent 2 units of bilateral anatomic modifiers. The original submitted version of the claim is retained in the Medicare IDR (Integrated Data Repository).
- Providers or suppliers shall use anatomic modifiers (e.g., RT, LT, FA, F1-F9, TA, T1-T9, E1-E4) and report procedures with differing modifiers on individual claim lines when appropriate. Many MUEs are based on the assumption that correct modifiers are used.
- A/B MACs shall include with the review determination the more detailed explanation of the correct coding initiative edit, which can be found in the standard correspondence language for A/B MACs in the [Medicare NCCI Correspondence Language Manual](#).

- MACs shall assign MSN 15.6. CARC 151 with Group Code CO for claims that fail the MUE edits, when the UOS on the claim exceeds the MUE value, and deny the entire claim line(s) for the relevant HCPCS/CPT code.

## **20.9.4 Reserved for future use**

**(Rev. 4465; Issued: 11-15-19, Effective: 01-30-19, Implementation: 01-30-19)**

### **20.9.4.1 Reserved for future use**

**(Rev. 4465; Issued: 11-15-19, Effective: 01-30-19, Implementation: 01-30-19)**

## **20.9.5 - Adjustments**

**(Rev. 1, 10-01-03)**

A/B MACs (B) adjust for underpayment if the wrong, lower paying code is paid on the first of multiple claims submitted. If the wrong, higher paying code is paid on the first of multiple claims submitted, A/B MACs (B) pay the subsequent claim(s) and initiate recovery action on the previously paid claim(s).

## **20.9.6 - Correct Coding Edit (CCE) File Record Format**

**(Rev. 10233; Issued: 07-24-20, Effective: 06-16-20, Implementation: 06-16-20)**

The following record layout for the Correct Coding Edit (CCE) File is available to the Shared Systems, A/B MACs (B), and the Regional Offices via Network Data Mover and CMS Data Center.

A/B MAC (B)/Shared Systems Record Format

| <b>Field</b>                                                                                                               | <b>Type</b> | <b>Record Position</b> | <b>Length</b> |
|----------------------------------------------------------------------------------------------------------------------------|-------------|------------------------|---------------|
| Comprehensive Column 1 Code or<br>Mutually Exclusive Column 1 Code                                                         | Character   | 1                      | 5             |
| Component Column 2 Code or<br>Mutually Exclusive Column 2 Code                                                             | Character   | 6                      | 5             |
| Prior Rebundled Code Indicator<br>“*” rebundled prior to 1996 edits<br>“•” rebundled 1/1/1996 or later                     | Character   | 11                     | 1             |
| Correspondence Language Reference                                                                                          | Character   | 12                     | 12            |
| Effective Date (4 position year followed by<br>Julian day)                                                                 | Numeric     | 24                     | 7             |
| Deletion Date (4 position year followed by<br>Julian day)                                                                  | Numeric     | 31                     | 7             |
| Modifier Indicators<br>“0” No CCE modifier allowed<br>“1” CCE modifier acceptable<br>“9” Use of CCE modifier not specified | Numeric     | 38                     | 1             |

| Field                       | Type      | Record Position | Length |
|-----------------------------|-----------|-----------------|--------|
| Savings Type Indicator Edit | Character | 39              | 1      |
| “1” CCE                     |           |                 |        |
| “2” Mutually Exclusive      |           |                 |        |

### **20.9.7 - National Correct Coding Initiative (NCCI) Edits Quarterly Updates (Rev. 10878, Issued: 07-15-2021, Effective:08-16-2021; Implementation:08-16-2021)**

Medicare Administrative Contractors (MACs) receive quarterly updates to National Correct Coding Initiative (NCCI) edits, indicating the version and the effective date, through a recurring update notification. At this time, the official method for providers or suppliers to receive the National Correct Coding Initiative (NCCI) edits is through the CMS website.

### **30 - Services Paid Under the Medicare Physician’s Fee Schedule (Rev. 1717, Issued: 04-24-09, Effective: 07-01-09, Implementation: 07-06-09)**

Following is a general description of services paid under the Medicare Physicians’ Fee Schedule (MPFS).

#### **A. Physician’s Services**

Effective with services furnished on or after January 1, 1992, A/B MACs (B) pay for physicians’ services based on the MPFS. The Medicare allowed charge for such physicians’ services is the lower of the actual charge or the fee schedule amount. The Medicare payment is 80 percent of the allowed charge after the deductible is met for most services paid based on the fee schedule. Exceptions to the rule, e.g., services for which deductible is not applicable, are specifically identified for the service where the exception applies.

The Physicians Fee Schedule is used when paying for the following physicians’ services.

- Professional services (including attending physicians’ services furnished in teaching settings) of doctors of medicine and osteopathy (including osteopathic practitioners), doctors of optometry, doctors of podiatry, doctors of dental surgery and dental medicine, and chiropractors;
- Services covered incident to physicians’ services other than certain drugs covered as incident to services;
- Physical and occupational therapy, and speech-language pathology services furnished by physical therapists, occupational therapists, and speech-language pathologists in private practices;
- Diagnostic tests other than clinical laboratory tests. See chapter 16 for payment for clinical diagnostic laboratory tests;
- Radiology services; and
- Monthly capitation payment (MCP) for physicians’ services associated with the continuing medical management of end stage renal disease (ESRD) services.

The fee schedule is not used to pay for direct medical and surgical services of teaching physicians in hospitals that have elected cost payment under §1861(b)(7) of the Act.

When processing a claim, A/B MACs (B) continue to determine if a service is reasonable and necessary to treat illness or injury. If a service is not reasonable and necessary to treat illness or injury for any reason (including lack of safety and efficacy because it is an experimental procedure, etc.), A/B MACs (B) consider the service noncovered notwithstanding the presence of a payment amount for the service in the Medicare fee schedule. The presence of a payment amount in the MPFS and the Medicare physician fee schedule database (MPFSDB) does not imply that CMS has determined that the service may be covered by Medicare. The nature of the status indicator in the database does not control coverage except where the status is N for noncovered.

A/B MACs (B) pay the above-mentioned physician services according to the physician fee schedule when billed by the following entities:

- A physician or physician group including optometrists, dentists, oral and maxillofacial surgeons, podiatrists, and chiropractors;
- A privately practicing physical therapist, (for outpatient physical therapy services);
- A privately practicing speech-language pathologist (for outpatient speech-language services);
- A privately practicing occupational therapist (for outpatient occupational therapy services);
- A nonphysician practitioner including a nurse practitioner, a physician assistant, and a clinical nurse specialist beginning January 1, 1998, with respect to services these practitioners are authorized to furnish under state law: payment is equal to 85 percent of the participating physician fee schedule amount for the same service;
- A nurse midwife: payment is equal to 65 percent of the participating physician fee schedule amount for the same service;
- A registered dietitian or nutrition professional, for medical nutrition therapy services provided as of January 1, 2002: payment is equal to 85 percent of the participating physician fee schedule amount for the same service;
- An audiologist, for services rendered to beneficiaries not in a skilled nursing facility (SNF) Part A covered stay;
- A clinical psychologist who renders services in community mental health centers (CMHCs) on or after July 1, 1988, and in all settings on or after July 1, 1990;
- A clinical social worker: The fee schedule for CSW services is set at 75 percent of the fee schedule for comparable services furnished by clinical psychologists;
- Another entity that furnishes outpatient physical therapy, occupational therapy, and speech- language pathology services. This could be a rehabilitation agency, a public health agency, a clinic, a skilled nursing facility, a home health agency (for beneficiaries who are not eligible for home health benefits because they are not home bound beneficiaries entitled to home health benefits), hospitals (when such services are provided to an outpatient or to a hospital inpatient who is entitled to benefits under Part A but who has exhausted benefits during a spell of illness, or who is not entitled to Part A benefits) and comprehensive outpatient rehabilitation facilities (CORFs). The fee schedule also applies to outpatient rehabilitation services furnished under an arrangement with any of the enumerated entities that are to be paid on the basis of the physician fee schedule;
- The supplier of the technical component of any radiology or diagnostic service;
- An independent laboratory doing anatomic pathology services; and

- Services billed by entities authorized to bill for physicians, suppliers, etc. under the reassignment rules.

## **B. Hospice Services**

The Physicians Fee Schedule is used when paying for hospice physician's services by the A/B MAC (HHH). Regular hospice services are paid under the hospice rate schedule (see chapter 11.)

## **C. Outpatient Rehabilitation Services**

Effective with services furnished on or after January 1, 1999, A/B MACs (A) pay for outpatient rehabilitation services based on the MPFS. Services included are physical therapy, speech-language pathology, occupational therapy, and certain audiology and CORF services.

Effective with services furnished on or after July 1, 2000, A/B MACs (A) pay for all CORF services under the MPFS.

Effective with claims with dates of service on or after July 1, 2003, OPTs/Outpatient Rehabilitation Facilities (ORFs), (74X bill type) are required to report all their services utilizing HCPCS. A/B MACs (A) are required to make payment for these services under the MPFS unless the item or service is currently being paid under the orthotic fee schedule or the item is a drug, biological, supply or vaccine.

The MPFS applies when these services are furnished by rehabilitation agencies, (outpatient physical therapy providers and CORFs), hospitals (to outpatients and inpatients who are not in a covered Part A stay), SNFs (to residents not in a covered Part A stay and to nonresidents who receive outpatient rehabilitation services from the SNF), and HHAs (to individuals who are not homebound or otherwise are not receiving services under a home health plan of treatment). The MPFS is used as a method of payment for outpatient rehabilitation services furnished under arrangement with any of these providers. The MPFS allowed charge for these services is the lower of the actual charge or the fee schedule amount. The Medicare payment for the services is 80 percent of the allowed charge after the Part B deductible is met. This is a final payment. The MPFS does not apply to outpatient rehabilitation services furnished by critical access hospitals (CAHs). CAHs are paid on a reasonable cost basis.

### **Application of the Outpatient Mental Health Treatment Limitation (A/B MACs (A))**

In accordance with §1833 of the Act, payment is made at 62½ percent of the approved amount for outpatient mental health treatment services. This provision will continue to be implemented in accordance with the Act when these services are furnished to beneficiaries by CORFs. Therefore, make payment at 62½ percent of 80 percent of the approved amount (or in effect 50 percent) for outpatient mental health treatment services.

## **D. SNF Services**

Effective with services furnished on or after April 1, 2001, A/B MACs (A) pay for Part B services furnished to SNF Part B inpatients and outpatients (22X and 23X types of bill) under the MPFS and other applicable fee schedules. Thus, where a fee schedule exists for the type of service, the fee amount (or charge if less than the applicable fee amount) is paid. Fee schedules made effective for SNF on this date include: Therapy, Lab, and DMEPOS.

Effective for services furnished by a SNF on and after January 1, 2002, A/B MACs (A) pay SNFs for radiology, other diagnostic, and other services under the MPFS. Payment is the lower of billed charges or the fee schedule amount. In either case, any applicable deductible and coinsurance amounts are subtracted from the payment amount prior to payment. Coinsurance is calculated on the Medicare payment amount after the subtraction of any applicable deductible amount.

If there is no fee schedule for the service or item being billed, A/B MACs (A) and (HHH) are to make payment based on cost. Consequently, all services billed under Part B are to be billed using HCPCS codes, whether the beneficiary resides in a certified bed or a noncertified bed.

### **30.1 - Maintenance Process for the Medicare Physician Fee Schedule Database (MPFSDB)**

**(Rev. 1, 10-01-03)**

The CMS calculates the fee schedule payment amounts and releases them to the A/B MACs (B) in the Medicare Physician Fee Schedule Database (MPFSDB). A/B MACs (B) implement those payment amounts on January 1 of each year. The CMS maintains the payment files centrally and is responsible for recalculating any revised payment amounts. Any revisions initiated by Central Office (fee schedule amounts or payment policy indicators) are issued to the A/B MACs (B) on a quarterly basis.

The information for the ongoing maintenance of the MPFSDB is stated below.

- CMS calculates the new fee schedule amounts. The CMS Central Office issues the revised data to the ROs in the same format of the MPFSDB.
- A/B MACs (B) receive a file containing data with revisions for the quarter. This file is released electronically via CMS' Mainframe Telecommunications System.
- A/B MACs (B) must allow providers 30 days notification before revised payment amounts are implemented. The revised payment amounts are implemented the beginning of the following quarter.
- CMS furnishes the recalculated payment amounts to the A/B MACs (B) in data files to ensure accuracy. A/B MACs (B) overlay these files into their existing file, to eliminate the potential for errors.
- A/B MACs (B) must make adjustments on those claims that were processed incorrectly if the adjustment is requested by the physician/supplier. Adjustments are made retroactively to January 1 of the current year, unless otherwise specified. This directive applies in all instances unless the situation requires special consideration. In those instances, instructions on handling adjustments will be provided on a case-by-case basis.

Separate instructions are issued describing the data exchange for the A/B MACs (A) and (HHH)

- In summary, A/B MACs (A) and (HHH) receive the revised payment amounts two to three weeks after the A/B MACs (B) receive the data from CMS. A/B MACs (A) and (HHH) do not implement the revised payment amounts prior to the A/B MACs (B)'s implementation date.
- A/B MACs (B) are required to furnish the revised payment information to the State Medicaid Agencies upon their request one month following receipt of the data from CMS. Those State agencies with Internet access capability will download the data directly from CMS Web site.

The CMS publishes a schedule for Medicare Physician Fee Schedule updates and participation physician enrollment procedures annually for all MACs.

### **30.2 - MPFSDB Record Layout**

**(Rev. 4298, Issued: 05-03-19, Effective: 01-01-19, Implementation 10-07-19)**

The CMS MPFSDB includes the total fee schedule amount, related component parts, and payment policy indicators. The record layout is provided in the Addendum below.

### 30.2.1 - Payment Concerns While Updating Codes (Rev. 1, 10-01-03)

The following instructions apply in situations where the CMS CO does NOT provide pricing guidance via the Medicare Physician Fee Schedule Database (MPFSDB) for physicians' services.

If a new code appears, A/B MACs (B) make every effort to determine whether the procedure, drug or supply has a pricing history and profile. If there is a pricing history, map the new code to previous customary and prevailing charges or fee schedule amounts to ensure continuity of pricing.

Since there are different kinds of coding implosions and explosions, the way the principle is applied varies. For example, when the code for a single procedure is exploded into several codes for the components of that procedure, the total of the separate relative value unit or other charge screens established for the components must not be higher than the relative value units or other charge screens for the original service. However, when there is a single code that describes two or more distinct complete services (e.g., two different but related or similar surgical procedures), and separate codes are subsequently established for each, continue to apply the payment screens that applied to the single code to each of the services described by the new codes.

If there is no pricing history or coding implosion and explosion, A/B MACs (B) must make an individual consideration determination for pricing and payment of a covered service.

Conversely, when the codes for the components of a single service are combined in a single global code, A/B MACs (B) establish the payment screens for the new code by totaling the screens used for the components (i.e., use the total of the customary charges for the components as the customary charge for the global code; use the total of the prevailing charges for the components adjusted for multiple surgical rules if applicable as the prevailing charge for the global code, etc.). However, when the codes for several different services are imploded into a single code, A/B MACs (B) set the payment screens at the average (arithmetic mean), weighted by frequency, of the payment screens for the formerly separate codes.

### 30.2.2 - MPFSDB Status Indicators

(Rev. 4418, Issued: 10-18- 19, Effective: 01-01- 20 Implementation: 11- 19 19)

|      |                                                                                                                                                                                                                                                                                                                                                                                                            |
|------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| A =  | Active code. These codes are separately paid under the physician fee schedule if covered. There will be RVUs and payment amounts for codes with this status. The presence of an "A" indicator does not mean that Medicare has made a national coverage determination regarding the service; A/B MACs (B) remain responsible for coverage decisions in the absence of a national Medicare policy.           |
| B =  | Payment for covered services are always bundled into payment for other services not specified. There will be no RVUs or payment amounts for these codes and no separate payment is ever made. When these services are covered, payment for them is subsumed by the payment for the services to which they are incident (an example is a telephone call from a hospital nurse regarding care of a patient). |
| C =  | A/B MACs (B) price the code. A/B MACs (B) will establish RVUs and payment amounts for these services, generally on an individual case basis following review of documentation such as an operative report.                                                                                                                                                                                                 |
| D =* | Deleted/discontinued codes.                                                                                                                                                                                                                                                                                                                                                                                |
| E =  | Excluded from physician fee schedule by regulation. These codes are for items and/or services that CMS chose to exclude from the fee schedule payment by regulation. No RVUs or payment amounts are shown and no payment may be                                                                                                                                                                            |

|      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|      | made under the fee schedule for these codes. Payment for them, when covered, continues under reasonable charge procedures.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
| F =  | Deleted/discontinued codes. (Code not subject to a 90 day grace period.) These codes are deleted effective with the beginning of the year and are never subject to a grace period. This indicator is no longer effective beginning with the 2005 fee schedule as of January 1, 2005.                                                                                                                                                                                                                                                                                                                                                                                                                            |
| G =  | Not valid for Medicare purposes. Medicare uses another code for reporting of, and payment for, these services. (Code subject to a 90 day grace period.) This indicator is no longer effective beginning with the 2005 fee schedule as of January 1, 2005.                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| H =* | Deleted modifier. For 2000 and later years, either the TC or PC component shown for the code has been deleted and the deleted component is shown in the data base with the H status.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| I =  | Not valid for Medicare purposes. Medicare uses another code for reporting of, and payment for, these services. (Code NOT subject to a 90 day grace period.)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| J=   | Anesthesia services (no relative value units or payment amounts for anesthesia codes on the database, only used to facilitate the identification of anesthesia services.)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| L =  | Local codes. A/B MACs (B) will apply this status to all local codes in effect on January 1, 1998 or subsequently approved by central office for use. A/B MACs (B) will complete the RVUs and payment amounts for these codes.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| M=   | Measurement codes, used for reporting purposes only.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| N =  | Non-covered service. These codes are carried on the HCPCS tape as noncovered services.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| P =  | <p>Bundled/excluded codes. There are no RVUs and no payment amounts for these services. No separate payment is made for them under the fee schedule.</p> <p>If the item or service is covered as incident to a physician service and is provided on the same day as a physician service, payment for it is bundled into the payment for the physician service to which it is incident (an example is an elastic bandage furnished by a physician incident to a physician service).</p> <p>If the item or service is covered as other than incident to a physician service, it is excluded from the fee schedule (for example, colostomy supplies) and is paid under the other payment provision of the Act.</p> |
| Q =  | Therapy functional information code (used for required reporting purposes only). This indicator is no longer effective beginning with the 2020 fee schedule as of January 1, 2020.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
| R =  | Restricted coverage. Special coverage instructions apply.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| T =  | There are RVUs and payment amounts for these services, but they are only paid if there are no other services payable under the physician fee schedule billed on the same date by the same provider. If any other services payable under the physician fee schedule are billed on the same date by the same provider, these services are bundled into the physician services for which payment is made.                                                                                                                                                                                                                                                                                                          |

|     |                                                                                                                                                                                                                                                                                                                                                                    |
|-----|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| X = | Statutory exclusion. These codes represent an item or service that is not in the statutory definition of “physician services” for fee schedule payment purposes. No RVUs or payment amounts are shown for these codes and no payment may be made under the physician fee schedule. (Examples are ambulances services and clinical diagnostic laboratory services.) |
|-----|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

\*Codes with these indicators had a 90 day grace period before January 1, 2005

### **30.3 - Furnishing Pricing Files**

**(Rev. 1, 10-01-03)**

The CMS provides a schedule for activities related to furnishing these pricing files in advance each year. The CMS will provide the completed physician fee schedule, the Durable Medical Equipment and Prosthetics and Orthotics and Supplies (DMEPOS) fee schedules and clinical lab data to United Mine Workers and Indian Health Services. A/B MACs (A), (B), (HHH), and DME MACs are informed where to access the files for FTP (File Transfer Protocol) download.

#### **30.3.1 - RESERVED**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

#### **30.3.2 - A/B MAC (A), (B), or (HHH) Furnishing Physician Fee Schedule Data for National Codes**

**(Rev. 4131, Issued: 09-14-18, Effective: 12-17-18, Implementation: 12-17-18)**

A/B MACs (A), (B), and (HHH) are responsible for furnishing fee schedules to related providers, suppliers, physicians, and practitioners with billing relationships.

A/B MACs (A), (B), and (HHH) may also receive requests for this information from the State Agencies. The data is available on the CMS website. The MACs shall direct State Agencies to the appropriate pages on the website to download the data directly.

The CMS will provide the Medicare physician, ambulance, DMEPOS, and clinical laboratory fee schedules and the zip code file to the A/B MACs (A) and (HHH), Palmetto GBA (the Specialty MAC or S MAC for Railroad Retirement Board), the Indian Health Services, and the United Mine Workers. MACs DO NOT NEED TO SEND FILES TO THOSE ENTITIES.

Fee schedules also are available for the public on the CMS Web site.

#### **30.3.3 - Furnishing Other Fee Schedule, Prevailing Charge, and Conversion Factor Data**

**(Rev. 1, 10-01-03)**

**B3-4620.3**

A/B MACs (B) use the file format in §30.3.5 to furnish fee schedule (Excluding Physician Fee Schedule), prevailing charge, and conversion factor information to Palmetto GBA (S MAC for RRB), A/B MACs (A) or (HHH), State Agencies, Indian Health Services, and United Mine Workers.

A/B MACs (B) furnish statewide (or A/B MAC (B)-wide for areas less than an entire State) pricing data for certified registered nurse anesthetist conversion factors. Furnish all fee schedules and conversion factors on tape unless the receiving entity agrees that a paper listing is acceptable.

Clinical Lab pricing files subject to national limitation amounts and DMEPOS pricing files subject to national floor and ceiling limitation amounts will be furnished by CMS to all entities except the State

Medicaid Agencies. The Center for Medicaid and State Operations will provide those pricing files to the State Medicaid Agencies.

In addition to the above pricing files, A/B MACs (B) furnish Palmetto GBA (the S MAC for RRB) with a tape file of locality prevailing charges for ambulance services and inflation indexed prevailing charges for non-physician services subject to the IIC.

Send pricing files for the RRB S MAC to:

Medicare Systems RRB - AG-430  
Palmetto GBA  
Building One  
2300 Springdale Drive  
Camden, SC 29020

For Indian Health Services, send pricing files to:

IHS Contract Health Services  
12800 Indian School Road North East  
Albuquerque, NM 87112

For releasing nonphysician pricing files to State Agencies, contact the RO to obtain the name and mailing address of the individual to whom the file should be addressed.

For the United Mine Workers, send the pricing files to:

Government Programs Manager  
UMWA Health and Retirement Funds  
2121 K Street, NW  
Washington, DC 20037, or  
MedPricing@umwafunds.org

You may negotiate agreements with the receiving entity to use an alternate medium (e.g., paper, diskette) or a tape file format other than that specified in §30.3.5. However, such agreements must be in writing and signed by the affected entities (e.g., A/B MAC (A), (B), (HHH), or DME MAC, RRB S MAC, UMWA, etc.). Furnish your RO with a copy of written agreements for using mediums other than tape or tape file formats other than that in §30.3.5.

### **30.3.4 - Responsibility to Obtain and Implement Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedules**

(Rev. 1, 10-01-03)

**B3-4622**

The A/B MACs (A), (B), (HHH), and DME MACs must download and implement the DMEPOS fee schedules for the items and services within their jurisdiction. DME MACs must also forward the DMEPOS fee schedule to Medicaid State Agencies. The CMS releases new fee schedules on an annual basis, with updates being issued quarterly, if needed. The CMS issues separate instructions for downloading and implementing fee schedule updates as appropriate. Central office (CO) will issue annual updates describing the maintenance process for each year. These instructions provide A/B MACs (A), (B), (HHH), and DME MACs the due dates. The instructions will also list the date the release will be made available and an implementation date for each release.

### **30.3.5 - File Specifications**

(Rev. 1, 10-01-03)

**B3-4621**

The following guidelines are used to create the pricing files.

1. Recommended Physical File Specifications-Magnetic Tape - Tape characteristics-9 track, 8 2" to 10 2" reels with silver mylar reflector (standard reels) with write ring removed.

Parity – Odd

Recording Density - 6250 bytes per inch

Recording Code - Extended Binary Coded Decimal

File Label - IBM standard label. The tape must have an end of file mark. The first data record on the file identifies the submitter and the receiver and serves the function of a file label.

Physical Record Length - 60 characters

Blocking Factor - 100 records per block

The external label on the reel must appear as:

From

To

Reel number HCPCS (see footnote (a) below for entry) number (the unique number by which the tape is identified in your library).

Date (MMDDYYYY)

2. Record Specifications. The logical HCPCS record is made up of a series of 60 character physical records.

3. Blocking factor of 100 (100 records per data block).

4. Header Type Specifications

| Field No. | Field Name        | Size | Picture | Specification | Field Remarks    |
|-----------|-------------------|------|---------|---------------|------------------|
| 1         | Label             | 3    | X(3)    | L             | See Footnote (b) |
| 2         | Filler            | 7    | X(7)    | L             |                  |
| 3         | A/B MAC (B) #     | 5    | 9(5)    | L             |                  |
| 4         | Filler            | 1    | X(1)    |               |                  |
| 5         | Interm. #         | 5    | 9(5)    | L             |                  |
| 6         | Filler            | 1    | X(1)    | L             |                  |
| 7         | Date Fee Updated  | 8    | X(8)    | L             | MMDDYYYY         |
| 8         | Filler            | 22   | X(22)   |               |                  |
| 9         | Date File Created | 8    | X(8)    | L             | MMDDYYYY         |

## 5. Detail Record

| Field No. | Field Name    | Size | Picture | Specification | Field Remarks                            |
|-----------|---------------|------|---------|---------------|------------------------------------------|
| 1         | HCPCS         | 5    | X(5)    | L             |                                          |
| 2         | Filler        | 2    | XX      | L             |                                          |
| 3         | Filler        | 2    | XX      | L             |                                          |
| 4         | Fee/CF/Prev.  | 7    | 9(5)V99 | R             | See Footnote (c) for applicable category |
| 5         | Fee           | 7    | 9(5)V99 | R             | See Footnote (d)                         |
| 6         | Fee           | 7    | 9(5)V99 | R             | See Footnote (e)                         |
| 7         | A/B MAC (B) # | 5    | X(5)    | L             |                                          |
| 8         | Locality      | 2    | 99      | L             |                                          |
| 9         | Filler        | 23   | X(23)   |               |                                          |

(a) Identify the type of information furnished:

- DME fee schedule (includes supplies, inexpensive/routinely purchased, frequency service, and capped rental);
- Surgical Dressings;
- Prevailing charges applicable to hospice physician services;
- Lab fee schedule;
- Other diagnostic service prevailing charges;
- Oxygen fee schedule;
- Prosthetic/orthotic fee schedule (includes ostomy, tracheostomy, and urologicals);
- Radiology conversion factors;
- Portable radiology fee schedule; or
- Certified registered nurse anesthetist conversion factors.

(b) Show the label for category as:

- DME - durable medical equipment;
- S/D - surgical dressings;
- HPH - prevailing charges for pricing hospice physician services;
- LAB - laboratory;
- ODX - other diagnostic services;
- OXY - oxygen;
- P/O - prosthetic/orthotic;
- RAD - radiology;
- PRF - portable radiology; or
- CNA - certified nurse anesthetist.

(c, d, e) DME

(c) purchase new

(d) purchase used

(e) monthly rental

**NOTE:** When a HCPCS code has multiple fees, list all the fees. A/B MACs (HHH) are not able to accept procedure codes with modifiers.

HPH

(c) locality prevailing charge for physician services

LAB

(c) fee schedule amount - 60%

(d) fee schedule amount - 62%

ODX

(c) locality prevailing charge amounts

OXY

(c) purchase new/purchased oxygen contents

(d) purchase used

(e) monthly rental

P/O

(c) purchase new

RAD

(c) conversion factors

**NOTE:** HCPCS codes are not necessary for radiology.

PRF

(c) fee schedule amount

CNA

(c) conversion factor - medically directed CRNA services

(d) conversion factor - non-medically directed CRNA services

## **40 - Clinical Diagnostic Laboratory Fee Schedule**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

The Medicare Claims Processing Manual, Chapter 16, “Laboratory Services From Independent Labs, Physicians, and Providers,” provides background and additional information for payment of laboratory services.

Clinical diagnostic laboratory tests - whether performed in a physician’s office, by an independent laboratory, or by a hospital laboratory for its outpatients - are paid based on fee schedules. This section sets out rules for use of these schedules.

The fee schedule amounts are adjusted annually to reflect changes in the Consumer Price Index (CPI) for all Urban Consumers (U.S. city average), or as otherwise specified by legislation. Adjustments are applied and amounts are determined by CMS and published for A/B MAC (A), (B), and (HHH) use and also on CMS Web site. A/B MACs (A), (B), and (HHH) are notified when and where updates are published.

For a cervical or vaginal smear test (pap smear), payment is the lesser of the local fee or the national limitation amount, but not less than the national minimum payment amount. However, in no case may payment for these tests exceed actual charges. The Part B deductible and coinsurance do not apply.

Regardless of whether a diagnostic laboratory test is performed in a physician's office, by an independent laboratory, or by a hospital laboratory for its outpatients or nonpatients, it is considered a laboratory service. When a hospital laboratory performs diagnostic laboratory tests for nonhospital patients, the laboratory is functioning as an independent laboratory.

National minimum limitation amounts are established each year for cervical or vaginal smear clinical laboratory tests. These payment amounts are published each year in a Recurring Update Notification issued by CMS. The affected CPT laboratory test codes for the national minimum payment amount are also identified in the annual Recurring Update Notification. National maximum limitation amounts may also be established for certain services and are also published each year this Recurring Update Notification.

A/B MACs (A), (B), and (HHH) pay the lowest of the applicable current fee schedule, the actual charge, or the NLA. This applies to all clinical diagnostic laboratory tests except:

- Laboratory tests furnished to a hospital inpatient whose stay is covered under Part A;
- Laboratory tests performed by a Skilled Nursing Facility (SNF) for its own SNF inpatients and reimbursed under Part A or Part B and any laboratory tests furnished under arrangements to an SNF inpatient with Part A coverage. (The only covered source for laboratory services furnished under Part A is the SNF itself or a hospital with which the facility has a transfer agreement in effect.)
- Laboratory tests furnished by hospital-based or independent ESRD dialysis facilities that are included under the ESRD composite rate payment;
- Laboratory tests furnished by hospitals in States or areas which have been granted demonstration waivers of Medicare reimbursement principles for outpatient services. The State of Maryland has been granted such demonstration waivers;
- Laboratory tests furnished to inpatients of a hospital with a waiver under §602(k) of the 1983 Amendments to the Act. This section of the Act provides that an outside supplier may bill under Part B for laboratory and other nonphysician services furnished to inpatients that are otherwise paid only through the hospital;
- Laboratory tests furnished to patients of rural health clinics (RHCs) under an all inclusive rate;
- Laboratory tests provided by a participating health maintenance organization (HMO) or health care prepayment plan (HCPP) to an enrolled member of the plan; and
- Laboratory tests furnished by a hospice.

#### **40.1 - Access to Clinical Diagnostic Lab Fee Schedule Files**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

The annual laboratory fee schedule data file should be retrieved electronically through CMS' mainframe telecommunications system.

For each test code, if the MAC's system retains only the pricing amount, they should load the data from the field named "60% Pricing Amt." For each test code, if the MAC's system has been developed to retain the local fee and the NLA, they may load the data from the fields named "60% Local Fee Amt" and "60% Natl Limit Amt" to use to determine payment. For clinical laboratory test codes for cervical or vaginal smear tests (listed in Chapter 16, "Laboratory Services from Independent Labs, Physicians, and Providers," §80.3)

load the data from the field named “60% Pricing Amt” to reflect the lower of the local fee or the NLA, but not less than the national minimum payment amount. The fields named “62% Local Fee Amt,” “62% Natl Limit Amt,” and “62% Pricing Amt” should be used by A/B MACs (A) for payment of clinical laboratory tests performed by a sole community hospital’s qualified laboratory.

The annual laboratory fee schedule data is available via the CMS website. It is available in multiple formats: Excel, text, and comma delimited.

## **40.2 - A/B MAC (B) Record Layout for Clinical Laboratory Fee Schedule**

**(Rev. 1, 10-01-03)**  
**PM AB-01-162, AB-02-163**

The instructions for each annual update contain the actual data set name for that year. The Data Set Name is included with each annual update instructions.

| <b>Data Element Name</b> | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                            |
|--------------------------|----------------|-----------------|-------------------------------------------------------------------------------------------------------------------------------------------|
| HCPCS Code               | X(05)          | 1-5             |                                                                                                                                           |
| A/B MAC (B) Number       | X(05)          | 6-10            |                                                                                                                                           |
| Locality                 | X(02)          | 11-12           | 00 = Denotes Single A/B MAC (B) State<br>01 = North Dakota<br>02 = South Dakota<br>20 = Puerto Rico<br>40 = New Hampshire<br>50 = Vermont |
| 60% Local Fee            | 9(05)V99       | 13 - 19         |                                                                                                                                           |
| 62% Local Fee            | 9(05)V99       | 20 - 26         |                                                                                                                                           |
| 60% Natl Limit Amt       | 9(05)V99       | 27 - 33         |                                                                                                                                           |
| 62% Natl Limit Amt       | 9(05)V99       | 34 - 40         |                                                                                                                                           |
| 60% Pricing Amt          | 9(05)V99       | 41 - 47         |                                                                                                                                           |
| 62% Pricing Amt          | 9(05)V99       | 48 - 54         |                                                                                                                                           |
| Gap - Fill Indicator     | X(01)          | 55 - 55         | 0 = No Gap-fill Required<br>1 = A/B MAC (B) Gap-fill<br>2 = Special Instructions Apply                                                    |
| Modifier                 | X(02)          | 56 - 57         | Where modifier is shown, QW denotes a CLIA waive test.                                                                                    |
| Filler                   | X(03)          | 58 - 60         |                                                                                                                                           |

## **40.3 - Institutional Claim Record Layout for Clinical Laboratory Fee Schedule**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

The CMS will provide the specific file names when the prices are released. The file name will contain the label CLAB.

Record Length = 60  
Record Format = FB  
Block Size = 6000  
Character Code = EBCDIC  
Sort Sequence = A/B MAC (B), Locality, HCPCS Code

#### Header Record

| Data Element Name   | Picture | Location | Comment     |
|---------------------|---------|----------|-------------|
| 1-Label             | X(03)   | 1 - 3    | Value = Lab |
| 2-Filler            | X(07)   | 4 - 10   |             |
| 3-Filler            | X(08)   | 11 - 15  |             |
| 4-Filler            | X(04)   | 16 - 22  |             |
| 5-Date Fee Update   | X(08)   | 23 - 30  | YYYYMMDD    |
| 6-Filler            | X(22)   | 31 - 52  |             |
| 7-Date File Created | X(08)   | 53 - 60  | YYYYMMDD    |

#### Data Record

| Data Element Name          | Picture  | Location | Comment                                                                                     |
|----------------------------|----------|----------|---------------------------------------------------------------------------------------------|
| 1-HCPCS                    | X(05)    | 1 - 5    |                                                                                             |
| 2-Filler                   | X(04)    | 6 - 9    |                                                                                             |
| 3-60% Fee                  | 9(05)V99 | 10 - 16  |                                                                                             |
| 4-62% Fee                  | 9(05)V99 | 17 - 23  |                                                                                             |
| 5-Filler                   | X(07)    | 24 - 30  |                                                                                             |
| 6- A/B MAC (B)<br>Number   | X(05)    | 31 - 35  |                                                                                             |
| 7- A/B MAC (B)<br>Locality | X(02)    | 36 - 37  | 00 = Single State A/B MAC (B)<br>01 = North Dakota<br>02 = South Dakota<br>20 = Puerto Rico |

| Data Element Name | Picture | Location | Comment                                                                       |
|-------------------|---------|----------|-------------------------------------------------------------------------------|
| 8-State Locality  | X(02)   | 38 - 39  | Separate instructions will be used for the use of this field at a later date. |
| 9-Filler          | X(02)   | 40 - 41  |                                                                               |
| 10-Effective Date | X(08)   | 42 - 49  | Update effective date (YYYYMMDD)                                              |
| 11-Filler         | X(11)   | 50 - 60  |                                                                               |

#### **40.4 - Gap-Filled Fees Submitted to CMS by A/B MACs (B)**

**(Rev. 1, 10-01-03)**

**AB-01-162, AB-02-163**

In accordance with §531(b) of the Benefits Improvement and Protection Act of 2000 (BIPA), CMS solicits public comments on determining payment amounts for new laboratory tests. The CMS hosts an annual public meeting to allow parties the opportunity to provide input to the payment determination process. The CMS employs one of two approaches to establishing payment amounts for new laboratory test codes, crosswalking and gap-filling. After considering public input regarding the new test codes, CMS determines which approach is most appropriate for each new test code.

If the new test is comparable to an existing test, the new test is “crosswalked” to the existing test, and it is assigned the local fee for the existing test and the corresponding NLA. The new test code and payment amounts are included in the updated laboratory fee schedule annually.

If CMS determines that the laboratory fee schedule includes no sufficiently comparable test to permit crosswalking, CMS instructs A/B MACs (B) to “gap-fill” the payment amount for the new test code. Gap-filling is an empirical process of determining a payment amount in a locality using available information sources. Usually the period during which gap-filled payment amounts are instructed is the year following the introduction of a new code. During this period, A/B MACs (B) establish and use these payment amounts; they may be revised in the course of the year. Also during this period, A/B MACs (B) must report the gap-fill amounts to their ROs which are then forwarded to CMS CO. The CMS considers the gap-fill amounts and uses them to establish the fees for the new test code in the next update of the laboratory fee schedule.

In determining gap-fill amounts, the sources of information A/B MACs (B) should examine, if available, include: charges for the test and routine discounts to charges; resources required to perform the test; payment amounts determined by other payers; and charges, payment amounts, and resources required for other tests that may be comparable or otherwise relevant. A/B MACs (B) may consider other sources of information as appropriate, including clinical studies and information provided by clinicians practicing in the area, manufacturers, or other interested parties. To assist each A/B MAC (B) in establishing a gap-fill amount, A/B MACs (B)’ Medical Directors may meet and share information regarding the new test, though without reaching a formal consensus.

Establishing payment amounts for new laboratory tests is inherently difficult, precisely because these tests are new and as a result the types and extent of information available about them may be limited. Because the circumstances of different tests may vary significantly, specifying in detail a method of using the various information sources outlined above does not appear appropriate at this time. However, CMS designates a new test code for gap-filling in instances where no test code seems sufficiently similar to make a crosswalk approach appropriate. Accordingly, A/B MACs (B) should not determine a gap-fill amount by crosswalking to the payment amount for another test code.

After determining a gap-fill amount, an A/B MAC (B) may consider if a least costly alternative to a new test exists. If an A/B MAC (B) determines a least costly alternative test exists, the A/B MAC (B) may adopt the payment amount of the least costly alternative test as the gap-fill amount for the new test code. The least costly alternative amount will be considered the local fee, and CMS will use this payment amount in establishing the NLA. However in this case, the A/B MAC (B) must report two payment amounts, the gap-fill amount prior to determination of a least costly alternative and the payment amount that the A/B MAC (B) has determined to be the least costly alternative.

A/B MACs (B) should also communicate the gap-fill amounts to corresponding A/B MACs (A) and (HHH). A/B MACs (B) can seek assistance from RO staff to facilitate communication of the gap-fill amounts to A/B MACs (A) and (HHH). The list of codes which A/B MACs (B) are required to gap-fill each year are communicated in the annual instructions.

A/B MACs (B) provide their RO with gap-fill fees according to the date communicated by CMS (usually May), to be used by CMS-Central for the development of subsequent or later laboratory fee schedules. A/B MACs (B) submit the gap-fill fees in a right-justified format. These gap-fill data should be transmitted in an ASCII file with the following file specifications to MStevenson@cms.hhs.gov with a copy to Agreenberg@cms.hhs.gov to assist with coordinated collection of the gap-fill fees.

Data Set Name: CLXXXXX.TXT\* (ASCII File)

(\*Denotes A/B MAC (B) 5 - digit number)

#### Gap-filled Fees Record Layout

| <b>Data Element Name</b>           | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                               |
|------------------------------------|----------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------|
| Year                               | X(4)           | 1 - 4           | Set to Year (e.g., 2003)                                                                                                                     |
| HCPCS Code                         | X(5)           | 5 - 9           |                                                                                                                                              |
| Modifier                           | X(2)           | 10 - 11         |                                                                                                                                              |
| A/B MAC (B)<br>Number              | X(5)           | 12 - 16         |                                                                                                                                              |
| Locality                           | X(2)           | 17 - 18         | 00 = Denotes Single State<br>A/B MAC (B)<br>01 = North Dakota<br>02 = South Dakota<br>20 = Puerto Rico<br>40 = New Hampshire<br>50 = Vermont |
| Gap-fill Amount                    | 9(5)V99        | 19 - 25         | Prior to any determination of<br>a least costly alternative                                                                                  |
| Least Costly<br>Alternative Amount | 9(5)V99        | 26-32           |                                                                                                                                              |
| Least Costly<br>Alternative Code   | X(5)           | 33-37           |                                                                                                                                              |

#### **40.4.1 - A/B MACs (B) Forward HCPCS Gap Fill Amounts to A/B MACs (A) and (HHH)** **(Rev. 1, 10-01-03)**

##### **CMS Memo 9-13-02 HCPCS Gap-fillR3.doc**

Any A/B MAC (B) that establishes a HCPCS gap fill payment rate for its own use, or at the request of an A/B MAC (A) or (HHH), must forward the information to the contacts at all A/B MACs (A) or (HHH) with providers in the A/B MAC (B) service area.

A/B MACs (B) that establish HCPCS gap-fill payment rates for their own needs should forward the information to A/B MACs (A) and (HHH) as soon as practicable. A/B MACs (A) and (HHH) are to contact the appropriate A/B MAC (B) to request a gap fill payment rate with 5 working days of the suspension of claims containing HCPCS codes(s) for which payment rate is needed and has not been forwarded. A/B MACs (B) must provide the payment rate to the requesting A/B MAC (A) or (HHH) (and all other A/B MACs (A) or (HHH) with providers in that common jurisdiction) within 10 working days of receipt of the request.

Requests for policy clarification in A/B MAC (B) establishing the HCPCS gap fill payment rates or A/B MAC (A) or (HHH) procedures for the use thereof should be directed to the respective Regional Office HCPCS business function experts (BFE). Examples of the need for A/B MAC (A) or (HHH) consultation with the BFE are coverage questions and receipt of unresolved significant inconsistencies in fee amounts from different A/B MACs (B) for the same HCPCS. The A/B MAC (A) or (HHH) should consult the A/B MACs (B) with disparate fees before contacting the BFE.

An A/B MAC (A) or (HHH) experiencing delay in replies or nonresponses from an A/B MAC (B) should alert its respective Consortium Contractor Management Staff (CCMS) Contract Manager (CM). The CM will coordinate with his/her counterpart assigned to the A/B MAC (B) to achieve more timely action by the A/B MAC (B).

It is the A/B MAC (A)'s or (HHH)'s responsibility to ascertain the amount or consult with the A/B MAC (B), CCMS-CM, or BFE timely. A/B MAC (A) or (HHH) requests to apply CC 15 to claims which are paid untimely solely because of a delay in ascertaining a HCPCS gap fill amount from the A/B MAC (B) should be addressed to the respective CM. The CM will contact the HCPCS BFE for a decision. Requests for application of CC 15 to claims for which the A/B MAC (A) or (HHH) did not request the gap fill payment rate within 5 working days of the claim suspension and/or did not seek timely advice from the RO BFE concerning application of gap fill amount will not be honored. Additionally, if the A/B MAC (B) replied at least five days before the expiration of the payment floor, CC 15 requests will not be honored.

#### **50 - Fee Schedules Used by Medicare A/B MACs (A) and (HHH) Processing Institutional Claims** **(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

A/B MACs (A) and (HHH) processing institutional claims retrieve multiple files from the CMS mainframe telecommunications system. The HCPCS data files include deleted codes for the upcoming year. A/B MACs (A) and (HHH) need to identify deleted codes using the HCPCS files because they are not identifiable solely from the fee schedules. HCPCS files are also obtained from CMS annually. New fee schedules are effective for dates of service on and after January 1 of each year. Quarterly and emergency updates to the fee schedules are also sometimes released. In that case, A/B MACs (A) and (HHH) implement them according to the instructions accompanying the release.

Two HCPCS files are furnished by CMS. They are:

- The annual HCPCS file update including procedure and modifier codes and deleted codes; and

- A print file of the new year HCPCS codes.

The following fee schedules are furnished by CMS for use in processing institutional claims:

- Fees for hospice claims for Part B services provided by the hospice beneficiary's attending physician;
- Medicare Physician Fee Schedule;
- Clinical Laboratory Fee Schedule discussed in §40.3 above;
- Durable Medical Equipment, Prosthetics/Orthotics and Supplies (DMEPOS) Fee Schedule. A/B MACs (HHH) with home health workloads retrieve data from all categories on this file. All MACs retrieve data from categories prosthetic/orthotics and surgical dressings;
- Outpatient Rehabilitation (Therapy) and CORF Services Fee Schedule Payment Amounts (Therapy/CORF Abstract File);
- CORF, outpatient Critical Access Hospital (CAH and Indian Health Services not part of the Outpatient Rehabilitation (therapy) file; and
- Skilled Nursing Facility (SNF) extract file for radiology, other diagnostic and other SNF services.

## **50.1 - Institutional Claim Record Layout for Hospice, Radiology and Other Diagnostic Prices and Local HCPCS Codes**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

This file contains hospice fee schedule prices extracted from the Physician Fee schedule. This file contains pricing data for A/B MAC (B)-priced and local HCPCS codes for radiology, other diagnostic services, and hospice services paid under the physician fee schedule. This file contains some high volume services such as portable x-rays. The file is also used to pay claims for Critical Access Hospitals that have elected the optional method. The CMS will provide the specific file names when the prices are released. The file name will contain the label HHH.

|                |   |                                             |
|----------------|---|---------------------------------------------|
| Record Length  | - | 60                                          |
| Record Format  | - | FB                                          |
| Block Size     | - | 6000                                        |
| Character Code | - | EBCDIC                                      |
| Sort Sequence  | - | A/B MAC (B), Locality, HCPCS Code, Modifier |

| <b>Data Element Name</b> | <b>Picture</b> | <b>Location</b> | <b>Comment</b> |
|--------------------------|----------------|-----------------|----------------|
| 1-HCPCS                  | X(05)          | 1 - 5           |                |
| 2-Modifier               | X(02)          | 6 - 7           |                |
| 3-Non-Facility Fee       | 9(05)V99       | 8 - 14          |                |

| <b>Data Element Name</b>                   | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                                                                                                       |
|--------------------------------------------|----------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 4-PCTC Indicator                           | X(01)          | 15 - 15         | This field is only applicable when pricing Critical Access Hospitals (CAHs) that have elected the optional method (Method 2) of payment.                                                                             |
| 5-Facility Fee                             | 9(05)V99       | 16 - 22         |                                                                                                                                                                                                                      |
| 6-Effective Date                           | X(08)          | 23 - 30         |                                                                                                                                                                                                                      |
| 7- A/B MAC (B) Number                      | X(05)          | 31 - 35         |                                                                                                                                                                                                                      |
| 8-Locality                                 | X(02)          | 36 - 37         |                                                                                                                                                                                                                      |
| 9-**Label**                                | X(03)          | 38 - 40         | HPH = Hospice Physician Services<br>ODX = Other Diagnostic Services<br>PRF = Portable Radiology<br>RAD = Radiology                                                                                                   |
| 10-Filler                                  | X(2)           | 41 - 42         |                                                                                                                                                                                                                      |
| 11-Status Code                             | X(1)           | 43 - 43         | Separate instructions will be used for the use of this field at a later date. This field indicates whether the code is in the physician fee schedule and whether it is separately payable if the service is covered. |
| 12-Multiple Surgery                        | X(01)          | 44 - 44         | Indicator indicates which payment adjustment rule for multiple procedures apply to the service.                                                                                                                      |
| 13-Non Facility PE                         | 9(05)V99       | 45 - 51         | Non-facility practice expense RVU                                                                                                                                                                                    |
| 14-Filler                                  | X(01)          | 52 - 52         |                                                                                                                                                                                                                      |
| 15- A/B MAC (A) or (HHH) Therapy Reduction | 9(05)V99       | 53 - 59         | Reduces payment amount for multiple surgery                                                                                                                                                                          |
| 16-Filler                                  | X(01)          | 60 - 60         |                                                                                                                                                                                                                      |

## **50.2 - Institutional Claim Record Layout for the Durable Medical Equipment, Prosthetic, Orthotic and Supply Fee Schedule**

**(Rev. 3350, Issued: 09-11-15, Effective: 01-01-16, Implementation: 01-04-16)**

This file contains services subject to national Floors and Ceilings under the DMEPOS Fee Schedules including Surgical Dressings. The CMS will provide the specific file names when the prices are released. The file name will contain the label DMEPOS.

|               |   |    |
|---------------|---|----|
| Record Length | - | 60 |
| Record Format | - | FB |

|                |   |                               |
|----------------|---|-------------------------------|
| Block Size     | - | 6000                          |
| Character Code | - | EBCDIC                        |
| Sort Sequence  | - | Label, HCPCS Code, MOD, State |

| Data Element Name            | Picture  | Location | Comment                                                                                                                                                                                                                                              |
|------------------------------|----------|----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1-HCPCS                      | X(05)    | 1 - 5    |                                                                                                                                                                                                                                                      |
| 2-MOD                        | X(02)    | 6 - 7    |                                                                                                                                                                                                                                                      |
| 3-MOD 2                      | X(02)    | 8 - 9    |                                                                                                                                                                                                                                                      |
| 4-Fee Schedule Amt           | 9(05)V99 | 10 - 16  |                                                                                                                                                                                                                                                      |
| 5-Filler                     | X(01)    | 17       |                                                                                                                                                                                                                                                      |
| 6-Rural Fee Amt              | 9(05)v99 | 18 - 24  |                                                                                                                                                                                                                                                      |
| 7-Filler                     | X(01)    | 25       |                                                                                                                                                                                                                                                      |
| 8-Rural Fee indicator        | X(01)    | 26-      | R = Rural Fee is present                                                                                                                                                                                                                             |
| 9-Filler                     | X(04)    | 27-30    |                                                                                                                                                                                                                                                      |
| 10-State                     | X(02)    | 31 - 32  |                                                                                                                                                                                                                                                      |
| 11-Filler                    | X(05)    | 33 - 37  |                                                                                                                                                                                                                                                      |
| 12-*Label*                   | X(3)     | 38 - 40  | DME = Durable Medical Equipment<br>(other than oxygen)<br>OXY = Oxygen<br>P/O = Prosthetic/Orthotic<br>S/D = Surgical Dressings                                                                                                                      |
| 13-Filler                    | X(4)     | 41 - 44  |                                                                                                                                                                                                                                                      |
| 14-*Pricing Change Indicator | X(1)     | 45 - 45  | 0 = No change to Update Fee Schedule Amount since previous release<br>1 = A change has occurred to the Update Fee Schedule Amount since the previous release.<br>NOTE: In the initial release of the annual update, this field is initialized to >0' |
| 15-Filler                    | X(02)    | 46 - 47  |                                                                                                                                                                                                                                                      |
| 16-Effective Date            | X(08)    | 48 - 55  | Update effective date (YYYYMMDD)                                                                                                                                                                                                                     |
| 17-Filler                    | X(05)    | 56 - 60  |                                                                                                                                                                                                                                                      |

### 50.3 - Institutional Claim Record Layout for the Outpatient Rehabilitation and CORF Services Fee Schedule

(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)

This is a physician fee schedule abstract file for outpatient rehabilitation and CORF services payment. The CMS will provide the specific file names when the prices are released. The file name will contain the label ABSTR.

|                |   |                                            |
|----------------|---|--------------------------------------------|
| Record Length  | - | 60                                         |
| Record Format  | - | FB                                         |
| Block Size     | - | 6000                                       |
| Character Code | - | EBCDIC                                     |
| Sort Sequence  | - | A/B MAC (B), Locality HCPCS Code, Modifier |

| <b>Data Element Name</b>                   | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                                                                                                       |
|--------------------------------------------|----------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1-HCPCS                                    | X(05)          | 1 - 5           |                                                                                                                                                                                                                      |
| 2-Modifier                                 | X(02)          | 6 - 7           |                                                                                                                                                                                                                      |
| 3-Non-Facility Fee                         | 9(05)V99       | 8 - 14          |                                                                                                                                                                                                                      |
| 4-PCTC Indicator                           | X(01)          | 15 - 15         | This field is only applicable when pricing Critical Access Hospitals (CAHs) that have elected the optional method (Method 2) of payment.                                                                             |
| 5-Facility Fee                             | 9(05)V99       | 12 - 22         |                                                                                                                                                                                                                      |
| 6-Effective Date                           | X(08)          | 23 - 30         | Update effective date                                                                                                                                                                                                |
| 7- A/B MAC (B) Number                      | X(05)          | 31 - 35         |                                                                                                                                                                                                                      |
| 8-Locality                                 | X(02)          | 36 - 37         |                                                                                                                                                                                                                      |
| 9-Filler                                   | X(03)          | 38 - 40         |                                                                                                                                                                                                                      |
| 10-Fee Indicator                           | X(1)           | 41 - 41         | R = Rehab/Audiology function test/CORF services                                                                                                                                                                      |
| 11-Outpatient Hospital                     | X(1)           | 42 - 42         | 0 = Fee applicable in hospital outpatient setting<br>1 = Fee not applicable in hospital outpatient setting                                                                                                           |
| 12-Status Code                             | X(1)           | 43 - 43         | Separate instructions will be used for the use of this field at a later date. This field indicates whether the code is in the physician fee schedule and whether it is separately payable if the service is covered. |
| 13-Multiple Surgery                        | X(01)          | 44 - 44         | Indicator indicates which payment adjustment rule for multiple procedures apply to the service.                                                                                                                      |
| 14-Non Facility PE                         | 9(05)V99       | 45 - 51         | Non-facility practice expense RVU                                                                                                                                                                                    |
| 15-Filler                                  | X(01)          | 52 - 52         |                                                                                                                                                                                                                      |
| 16- A/B MAC (A) or (HHH) Therapy Reduction | 9(05)V99       | 53 - 59         | Reduces payment amount for multiple surgery                                                                                                                                                                          |
| 17-Filler                                  | X(01)          | 60 - 60         |                                                                                                                                                                                                                      |

**50.4 - Institutional Claim Record Layout for the Skilled Nursing Facility Fee Schedule**  
**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

This section contains the record layout for the SNF extract for radiology Services, other diagnostic services, and other SNF services priced on the MPFS. The CMS will provide the specific file names when the prices are released. The file name will contain the label SNF.

|                |   |        |
|----------------|---|--------|
| Record Length  | - | 60     |
| Record Format  | - | FB     |
| Block Size     | - | 6000   |
| Character Code | - | EBCDIC |

| <b>Data Element Name</b>                   | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                                                                                                       |
|--------------------------------------------|----------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1-HCPCS                                    | X(05)          | 1 - 5           |                                                                                                                                                                                                                      |
| 2-Modifier                                 | X(02)          | 6 - 7           |                                                                                                                                                                                                                      |
| 3-Non-Facility Fee                         | 9(05)V99       | 8 - 14          | The SNF fee schedule amount is based on the “non-facility rate” which is the fee that physicians may receive if performing the service in the physician’s office.                                                    |
| 4-PCTC Indicator                           | X(01)          | 15 - 15         |                                                                                                                                                                                                                      |
| 5-Facility Fee                             | 9(05)V99       | 16 - 22         |                                                                                                                                                                                                                      |
| 6-Effective Date                           | X(08)          | 23 - 30         | Update effective date                                                                                                                                                                                                |
| 7- A/B MAC (B) Number                      | X(05)          | 31 - 35         |                                                                                                                                                                                                                      |
| 8-Locality                                 | X(02)          | 36 - 37         |                                                                                                                                                                                                                      |
| 9-Filler                                   | X(05)          | 38 - 42         |                                                                                                                                                                                                                      |
| 10-Status Code                             | X(1)           | 43 - 43         | Separate instructions will be used for the use of this field at a later date. This field indicates whether the code is in the physician fee schedule and whether it is separately payable if the service is covered. |
| 11-Multiple Surgery                        | X(01)          | 44 - 44         | Indicator indicates which payment adjustment rule for multiple procedures apply to the service.                                                                                                                      |
| 12-Non Facility PE                         | 9(05)V99       | 45 - 51         | Non-facility practice expense RVU                                                                                                                                                                                    |
| 13-Filler                                  | X(01)          | 52 - 52         |                                                                                                                                                                                                                      |
| 14- A/B MAC (A) or (HHH) Therapy Reduction | 9(05)V99       | 53 - 59         | Reduces payment amount for multiple surgery                                                                                                                                                                          |

| <b>Data Element Name</b> | <b>Picture</b> | <b>Location</b> | <b>Comment</b> |
|--------------------------|----------------|-----------------|----------------|
| 15-Filler                | X(01)          | 60 - 60         |                |

## **50.5 - RESERVED**

**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

## **50.6— Physician Fee Schedule Payment Policy Indicator File Record Layout**

**(Rev.12326; Issued:10-26-23; Effective: 01-29-24; Implementation:01-29-24)**

When a Medicare beneficiary elects hospice services, hospices must complete the data elements identified below for the Uniform (Institutional Provider) Bill (Form CMS-1450) or its electronic equivalent, which is a Notice of Election (NOE).

The information on the Physician Fee Schedule Payment Policy Indicator file record layout is used for processing Method II CAH professional services with revenue codes 96X, 97X or 98X.

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | LENGTH & PIC | Position |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|----------|
| <b>File Year</b><br>This field displays the effective year of the file.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | 4 Pic x(4)   | 1-4      |
| <b>HCPCS Code</b><br><br>This field represents the procedure code. Each Current Procedural Terminology (CPT) code and alpha-numeric HCPCS codes that are currently returned on the MPFS supplemental file will be included when the MPFS Status Indicator is A, C, T, and some with R. The standard sort for this field is blanks, alpha, and numeric in ascending order.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | 5 Pic x(5)   | 5-9      |
| <b>Modifier</b><br><br>For diagnostic tests, a blank in this field denotes the global service and the following modifiers identify the components:<br><br>26 = Professional component; and<br>TC = Technical component.<br><br>For services other than those with a professional and/or technical component, a blank will appear in this field with one exception: the presence of CPT modifier -53 which indicates that separate Relative Value Units (RVUs) and a fee schedule amount have been established for procedures which the physician terminated before completion. This modifier is used only with colonoscopy code 45378 and screening colonoscopy codes G0105 and G0121. Any other codes billed with modifier -53 are subject to medical review and priced by individual consideration.<br><br>Modifier-53 = Discontinued Procedure - Under certain circumstances, the physician may elect to terminate a surgical or diagnostic procedure. Due to extenuating circumstances, or those that | 2 Pic x(2)   | 10-11    |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | LENGTH & PIC | Position |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|----------|
| threaten the well being of the patient, it may be necessary to indicate that a surgical or diagnostic procedure was started but discontinued.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |              |          |
| <b>Code Status</b><br><br>This 1 position field provides the status of each code under the full fee schedule. Each status code is explained in Pub. 100-04, Chapter 23, §30.2.2.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | 1 Pic x(1)   | 12       |
| <b>Global Surgery</b><br><br>This field provides the postoperative time frames that apply to payment for each surgical procedure or another indicator that describes the applicability of the global concept to the service.<br><br>000 = Endoscopic or minor procedure with related preoperative and postoperative relative values on the day of the procedure only included in the fee schedule payment amount; evaluation and management services on the day of the procedure generally not payable.<br><br>010 = Minor procedure with preoperative relative values on the day of the procedure and postoperative relative values during a 10-day postoperative period included in the fee schedule amount; evaluation and management services on the day of the procedure and during this 10-day postoperative period generally not payable.<br><br>090 = Major surgery with a 1-day preoperative period and 90-day postoperative period included in the fee schedule payment amount.<br><br>MMM = Maternity codes; usual global period does not apply. | 3 Pic x(3)   | 13-15    |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                             | LENGTH & PIC | Position |
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| <p>XXX = Global concept does not apply.</p> <p>YYY = A/B MAC (A) determines whether global concept applies and establishes postoperative period, if appropriate, at time of pricing.</p> <p>ZZZ = Code related to another service and is always included in the global period of the other service. (NOTE: Physician work is associated with intra-service time and in some instances the post service time.)</p>                                                    |              |          |
| <p><b>Preoperative Percentage (Modifier 56)</b></p> <p>This field contains the percentage (shown in decimal format) for the preoperative portion of the global package. For example, 10 percent will be shown as 010000. The total of the preoperative percentage, intraoperative percentage, and the postoperative percentage fields will usually equal one. Any variance is slight and results from rounding.</p>                                                  | 6 Pic 9v9(5) | 16-21    |
| <p><b>Intraoperative Percentage (Modifier 54)</b></p> <p>This field contains the percentage (shown in decimal format) for the intraoperative portion of the global package including postoperative work in the hospital. For example, 63 percent will be shown as 063000. The total of the preoperative percentage, intraoperative percentage, and the postoperative percentage fields will usually equal one. Any variance is slight and results from rounding.</p> | 6 Pic 9v9(5) | 22-27    |
| <p><b>Postoperative Percentage (Modifier 55)</b></p> <p>This field contains the percentage (shown in decimal format) for the postoperative portion of the global package that is provided in the office after discharge from the hospital. For example, 17 percent will be shown as 017000. The</p>                                                                                                                                                                  | 6 Pic 9v9(5) | 28-33    |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | LENGTH & PIC | Position |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|----------|
| total of the preoperative percentage, intraoperative percentage, and the postoperative percentage fields will usually equal one. Any variance is slight and results from rounding.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |              |          |
| <p><b>Professional Component (PC)/Technical Component (TC) Indicator</b></p> <p>0 = Physician service codes: This indicator identifies codes that describe physician services. Examples include visits, consultations, and surgical procedures. The concept of PC/TC does not apply since physician services cannot be split into professional and technical components. Modifiers 26 &amp; TC cannot be used with these codes. The total Relative Value Units (RVUs) include values for physician work, practice expense and malpractice expense. There are some codes with no work RVUs.</p> <p>1 = Diagnostic tests or radiology services: This indicator identifies codes that describe diagnostic tests, e.g., pulmonary function tests, or therapeutic radiology procedures, e.g., radiation therapy. These codes generally have both a professional and technical component. Modifiers 26 and TC can be used with these codes.</p> <p>The total RVUs for codes reported with a 26 modifier include values for physician work, practice expense, and malpractice expense.</p> <p>The total RVUs for codes reported with a TC modifier include values for practice expense and malpractice expense only. The total RVUs for codes reported without a modifier equals the sum of RVUs for both the professional and technical component.</p> <p>2 = Professional component only codes:</p> | 1 Pic x(1)   | 34       |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | LENGTH & PIC | Position |
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| <p>This indicator identifies stand alone codes that describe the physician work portion of selected diagnostic tests for which there is an associated code that describes the technical component of the diagnostic test only and another associated code that describes the global test.</p> <p>An example of a professional component only code is 93010, Electrocardiogram; interpretation and report. Modifiers 26 and TC cannot be used with these codes. The total RVUs for professional component only codes include values for physician work, practice expense, and malpractice expense.</p> <p>3 = Technical component only codes: This indicator identifies stand alone codes that describe the technical component (i.e., staff and equipment costs) of selected diagnostic tests for which there is an associated code that describes the professional component of the diagnostic tests only.</p> <p>An example of a technical component code is 93005, Electrocardiogram, tracing only, without interpretation and report. It also identifies codes that are covered only as diagnostic tests and therefore do not have a related professional code. Modifiers 26 and TC cannot be used with these codes.</p> <p>The total RVUs for technical component only codes include values for practice expense and malpractice expense only.</p> <p>4 = Global test only codes: This indicator identifies stand alone codes for which there are associated codes that describe: a) the professional component of the test only and b) the technical component of the test only. Modifiers 26 and TC cannot be used with these codes. The total RVUs for global procedure only codes include values for physician work,</p> |              |          |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | LENGTH & PIC | Position |
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| <p>practice expense, and malpractice expense. The total RVUs for global procedure only codes equals the sum of the total RVUs for the professional and technical components only codes combined.</p> <p>5 = Incident to Codes: This indicator identifies codes that describe services covered incident to a physicians service when they are provided by auxiliary personnel employed by the physician and working under his or her direct supervision.</p> <p>Payment may not be made by carriers for these services when they are provided to hospital inpatients or patients in a hospital outpatient department. Modifiers 26 and TC cannot be used with these codes.</p> <p>6 = Laboratory physician interpretation codes: This indicator identifies clinical laboratory codes for which separate payment for interpretations by laboratory physicians may be made. Actual performance of the tests is paid for under the lab fee schedule. Modifier TC cannot be used with these codes. The total RVUs for laboratory physician interpretation codes include values for physician work, practice expense and malpractice expense.</p> <p>7 = Physical therapy service: Payment may not be made if the service is provided to either a hospital outpatient or inpatient by an independently practicing physical or occupational therapist.</p> <p>8 = Physician interpretation codes: This indicator identifies the professional component of clinical laboratory codes for which separate payment may be made only if the physician interprets an abnormal smear for hospital inpatient. This applies only to code 85060. No TC billing is recognized because payment for</p> |              |          |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | LENGTH & PIC | Position |
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| <p>the underlying clinical laboratory test is made to the hospital, generally through the PPS rate.</p> <p>No payment is recognized for code 85060 furnished to hospital outpatients or non-hospital patients. The physician interpretation is paid through the clinical laboratory fee schedule payment for the clinical laboratory test.</p> <p>9 = Concept of a professional/technical component does not apply</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |              |          |
| <p><b>Multiple Procedure (Modifier 51)</b></p> <p>Indicator indicates which payment adjustment rule for multiple procedures applies to the service.</p> <p>0 = No payment adjustment rules for multiple procedures apply. If the procedure is reported on the same day as another procedure, base payment on the lower of: (a) the actual charge or (b) the fee schedule amount for the procedure.</p> <p>1 = Standard payment adjustment rules in effect before January 1, 1996, for multiple procedures apply. In the 1996 MPFSDB, this indicator only applies to codes with procedure status of "D." If a procedure is reported on the same day as another procedure with an indicator of 1,2, or 3, rank the procedures by fee schedule amount and apply the appropriate reduction to this code (100 percent, 50 percent, 25 percent, 25 percent, 25 percent, and by report). Base payment on the lower of: (a) the actual charge or (b) the fee schedule amount reduced by the appropriate percentage.</p> <p>2 = Standard payment adjustment rules for multiple procedures apply. If the procedure is reported on the same day as another procedure with an indicator of 1, 2, or 3, rank the procedures by fee schedule amount and apply the appropriate reduction to this code (100</p> | 1 Pic (x)1   | 35       |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | LENGTH & PIC | Position |
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| <p>percent, 50 percent, 50 percent, 50 percent, 50 percent, and by report). Base payment on the lower of: (a) the actual charge or (b) the fee schedule amount reduced by the appropriate percentage.</p> <p>3 = Special rules for multiple endoscopic procedures apply if procedure is billed with another endoscopy in the same family (i.e., another endoscopy that has the same base procedure). The base procedure for each code with this indicator is identified in the endoscopic base code field.</p> <p>Apply the multiple endoscopy rules to a family before ranking the family with other procedures performed on the same day (for example, if multiple endoscopies in the same family are reported on the same day as endoscopies in another family or on the same day as a non-endoscopic procedure).</p> <p>If an endoscopic procedure is reported with only its base procedure, do not pay separately for the base procedure. Payment for the base procedure is included in the payment for the other endoscopy.</p> <p>4 = Subject to 25% reduction of the TC diagnostic imaging (effective for services January 1, 2006 through June 30, 2010). Subject to 50% reduction of the TC diagnostic imaging (effective for services July 1, 2010 and after). Subject to 25% reduction of the PC of diagnostic imaging (effective for services January 1, 2012 through December 31, 2016). Subject to 5% reduction of the PC of diagnostic imaging (effective for services January 1, 2017 and after).</p> <p>5 = Subject to 20% reduction of the practice expense component for certain therapy services furnished in office and other non-institutional settings, and 25%</p> |              |          |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | LENGTH & PIC | Position |
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| <p>reduction of the practice expense component for certain therapy services furnished in institutional settings (effective for services January 1, 2011 and after). Subject to 50% reduction of the practice expense component for certain therapy services furnished in both institutional and non-institutional settings (effective for services April 1, 2013 and after).</p> <p>6 = Subject to 25% reduction of the TC diagnostic cardiovascular services (effective for services January 1, 2013 and after).</p> <p>7 = Subject to 20% reduction of the TC diagnostic ophthalmology services (effective for services January 1, 2013 and after).</p> <p>9 = Concept does not apply.</p> <p>Codes with RVUs equal to zero are not included in the payment indicator file. These codes may have multiple procedure indicators not shown. See note below this table for instructions on these codes.</p> |              |          |
| <p><b>Bilateral Surgery Indicator (Modifier 50)</b></p> <p>This field provides an indicator for services subject to a payment adjustment.</p> <p>0 = 150 percent payment adjustment for bilateral procedures does not apply.</p> <p>The bilateral adjustment is inappropriate for codes in this category because of: (a) physiology or anatomy, or (b) because the code descriptor specifically states that it is a unilateral procedure and there is an existing code for the bilateral procedure.</p>                                                                                                                                                                                                                                                                                                                                                                                                    | 1 Pic (x)1   | 36       |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | LENGTH & PIC | Position |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|----------|
| <p>1 = 150 percent payment adjustment for bilateral procedures applies. If code is billed with the bilateral modifier base payment for these codes when reported as bilateral procedures on the lower of: (a) the total actual charge for both sides, or (b) 150 percent of the fee schedule amount for a single code.</p> <p>If code is reported as a bilateral procedure and is reported with other procedure codes on the same day, apply the bilateral adjustment before applying any applicable multiple procedure rules.</p> <p>2 = 150 percent payment adjustment for bilateral procedure does not apply. RVUs are already based on the procedure being performed as a bilateral procedure.</p> <p>The RVUs are based on a bilateral procedure because: (a) the code descriptor specifically states that the procedure is bilateral; (b) the code descriptor states that the procedure may be performed either unilaterally or bilaterally; or (c) the procedure is usually performed as a bilateral procedure.</p> <p>3 = The usual payment adjustment for bilateral procedures does not apply.</p> <p>Services in this category are generally radiology procedures or other diagnostic tests which are not subject to the special payment rules for other bilateral procedures. If a procedure is billed with the 50 modifier, base payment on the lesser of the total actual charges for each side or 100% of the fee schedule amount for each side.</p> <p>9 = Concept does not apply.</p> |              |          |
| <b>Assistant at Surgery (Modifiers AS, 80,</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | 1 Pic (x)1   | 37       |

| FIELD NAME & DESCRIPTION                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | LENGTH & PIC | Position |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|----------|
| <p><b>81 and 82)</b></p> <p>This field provides an indicator for services where an assistant at surgery may be paid:</p> <p>0 = Payment restriction for assistants at surgery applies to this procedure unless supporting documentation is submitted to establish medical necessity.</p> <p>1 = Statutory payment restriction for assistants at surgery applies to this procedure. Assistant at surgery may not be paid.</p> <p>2 = Payment restriction for assistants at surgery does not apply to this procedure. Assistant at surgery may be paid.</p> <p>9 = Concept does not apply.</p> |              |          |
| <p><b>Co-Surgeons (Modifier 62)</b></p> <p>This field provides an indicator for services for which two surgeons, each in a different specialty, may be paid.</p> <p>0 = Co-surgeons not permitted for this procedure.</p> <p>1 = Co-surgeons could be paid; supporting documentation required to establish medical necessity of two surgeons for the procedure.</p> <p>2 = Co-surgeons permitted; no documentation required if two specialty requirements are met.</p> <p>9 = Concept does not apply.</p>                                                                                    | 1 Pic (x)1   | 38       |
| <p><b>Team Surgeons (Modifier 66)</b></p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 1 Pic (x)1   | 39       |

| <b>FIELD NAME &amp; DESCRIPTION</b>                                                                                                                                                                                                                                                                                                                                           | <b>LENGTH &amp; PIC</b> | <b>Position</b> |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------|-----------------|
| <p>This field provides an indicator for services for which team surgeons may be paid.</p> <p>0 = Team surgeons not permitted for this procedure.</p> <p>1 = Team surgeons could be paid; supporting documentation required to establish medical necessity of a team; pay by report.</p> <p>2 = Team surgeons permitted; pay by report.</p> <p>9 = Concept does not apply.</p> |                         |                 |
| <p><b>Endoscopic Base Codes</b></p> <p>This field identifies an endoscopic base code for each code with a multiple surgery indicator of 3.</p>                                                                                                                                                                                                                                | 5 Pic (x) 5             | 40-44           |
| <p><b>Performance Payment Indicator</b><br/>(For future use)</p>                                                                                                                                                                                                                                                                                                              | 1 Pic x (1)             | 45              |
| <p><b>Diagnostic Imaging Family Indicator</b></p> <p>88 = Subject to the reduction for diagnostic imaging (effective for services January 1, 2011, and after).</p> <p>99 = Concept Does Not Apply</p>                                                                                                                                                                         | 2 Pic x (2)             | 46-47           |
| <p><b>Effective Date</b></p> <p>This field displays the effective date of the file in YYYYMMDD format.</p>                                                                                                                                                                                                                                                                    | 8 Pic x(8)              | 48 - 55         |
| <b>Filler</b>                                                                                                                                                                                                                                                                                                                                                                 | 30 Pic x(30)            | 56 -75          |

Multiple procedure indicator 5 is included in this file; however, the indicator represents the therapy multiple procedure payment reduction which never applies to professional service revenue codes on CAH outpatient claims. Multiple procedure indicators 6 and 7 are included in this file, however, since in these cases the reduction only applies to technical component services, they will not affect the cost-based payment on CAH claims. On CAH claims, technical components are paid on a cost basis and so are not subject to the reductions.

There may be cases when A/B MACs (A) must manually load a HCPCS code that is contractor priced which has a multiple procedure indicator that is not on the payment indicator file. In these cases, the MAC enters a multiple procedure indicator of 0.

## **50.7 - Institutional**

### **Claim Record Layout for the Mammography Fee Schedule**

(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)

This is a physician fee schedule abstract file for mammography services payment. The CMS will provide the specific file names when the prices are released. The file name will contain the label MAMMO.

|                |   |                                            |
|----------------|---|--------------------------------------------|
| Record Format  | - | FB                                         |
| Block Size     | - | 6000                                       |
| Character Code | - | EBCDIC                                     |
| Sort Sequence  | - | A/B MAC (B), Locality HCPCS Code, Modifier |

| <b>Data Element Name</b>                   | <b>Picture</b> | <b>Location</b> | <b>Comment</b>                                                                                                                                                                                                       |
|--------------------------------------------|----------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1-HCPCS                                    | X(05)          | 1 - 5           |                                                                                                                                                                                                                      |
| 2-Modifier                                 | X(02)          | 6 - 7           |                                                                                                                                                                                                                      |
| 3-Non-Facility Fee                         | 9(05)V99       | 8 - 14          |                                                                                                                                                                                                                      |
| 4-PCTC Indicator                           | X(01)          | 15 - 15         | This field is only applicable when pricing Critical Access Hospitals (CAHs) that have elected the optional method (Method 2) of payment.                                                                             |
| 5-Facility Fee                             | 9(05)V99       | 16 - 22         |                                                                                                                                                                                                                      |
| 6-Effective Date                           | X(08)          | 23 - 30         | Update effective date                                                                                                                                                                                                |
| 7- A/B MAC (B) Number                      | X(05)          | 31 - 35         |                                                                                                                                                                                                                      |
| 8-Locality                                 | X(02)          | 36- 37          |                                                                                                                                                                                                                      |
| 9-Filler                                   | X(05)          | 38- 42          |                                                                                                                                                                                                                      |
| 10-Status Code                             | X(1)           | 43- 43          | Separate instructions will be used for the use of this field at a later date. This field indicates whether the code is in the physician fee schedule and whether it is separately payable if the service is covered. |
| 11-Multiple Surgery                        | X(01)          | 44 - 44         | Indicator indicates which payment adjustment rule for multiple procedures apply to the service.                                                                                                                      |
| 12-Non Facility PE                         | 9(05)V99       | 45 - 51         | Non-facility practice expense RVU                                                                                                                                                                                    |
| 13-Filler                                  | X(01)          | 52 - 52         |                                                                                                                                                                                                                      |
| 14- A/B MAC (A) or (HHH) Therapy Reduction | 9(05)V99       | 53 - 59         | Reduces payment amount for multiple surgery                                                                                                                                                                          |
| 15-Filler                                  | X(01)          | 60 - 60         |                                                                                                                                                                                                                      |

**50.8 - Institutional Claim Record Layout for the Ambulance Fee Schedule**  
**(Rev. 2643, Issued: 01-31-13, Effective: 07-01-13, Implementation: 07-01-13)**

This is a physician fee schedule abstract file for ambulance services payment. The CMS will provide the specific file names when the prices are released. The file name will contain the label AMBFS.

|                |   |                              |
|----------------|---|------------------------------|
| Record Length  | - | 80                           |
| Record Format  | - | FB                           |
| Block Size     | - | 27920                        |
| Character Code | - | EBCDIC                       |
| Sort Sequence  | - | HCPCS, A/B MAC (B), Locality |

| Field Name             | Format  | Position | Description                                                        |
|------------------------|---------|----------|--------------------------------------------------------------------|
| 1-HCPCS                | X(05)   | 1 - 5    | HCFA Common Procedure Coding System                                |
| 2- A/B MAC (B) Number  | X(05)   | 6 - 10   |                                                                    |
| 3-Locality Code        | X(02)   | 11-12    |                                                                    |
| 4-Base RVU             | 9(4)v99 | 13 - 18  | Relative Value Unit                                                |
| 5-Non-Facility PE GPCI | 9v9(3)  | 19 - 22  | Geographic Adjustment Factor                                       |
| 6-Conversion Factor    | 9(5)v99 | 23 - 29  | Conversion Factor                                                  |
| 7-Urban Mileage        | 9(5)v99 | 30 - 36  | Urban payment rate or base rate mileage rate (determined by HCPCS) |
| 8-Rural Mileage        | 9(5)v99 | 37 - 43  | Rural payment rate or base rate mileage rate (determined by HCPCS) |
| 9-Current Year         | 9(04)   | 44- 47   | YYYY                                                               |
| 10-Current Quarter     | 9(01)   | 48       | Calendar Quarter-value 1-4                                         |
| 11-Effective Date      | 9(8)    | 49- 56   | Effective date of fee schedule file                                |
| 12-Filler              | X(24)   | 57 - 80  | Future Use                                                         |

## **60 - Durable Medical Equipment Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule**

**(Rev. 12068; Issued: 06-02-23; Effective: 07-01-23; Implementation: 07-03-23)**

The CMS issues instructions for implementing and/or updating the DMEPOS fee schedule payment amounts on an annual basis (January), with quarterly updates as necessary (April, July and October). In addition, special instructions may be issued at any time if necessary for implementation of changes in the statute, regulations, or for other reasons. The DMEPOS fee schedule is provided to DME MACs, the Pricing, Data Analysis and Coding Contractor (PDAC), and A/B MACs (B) via CMS' mainframe telecommunication system.

Upon successful receipt of the updated file(s), MACs send notification of receipt via email to [price\\_file\\_receipt@cms.hhs.gov](mailto:price_file_receipt@cms.hhs.gov) stating the name of the file received and the entities for which they were received (e.g., MAC name and A/B MAC (A) or (HHH) number).

A separate DMEPOS Fee Schedule file is released to A/B MACs (A) and (HHH), Railroad Retirement Board (RRB) and RRB's Specialty MAC, Indian Health Service, and United Mine Workers. This fee schedule is also available on the CMS site (<http://www.cms.gov/Medicare/Medicare-Fee-for-Service-Payment/DMEPOSFeeSched/DMEPOS-Fee-Schedule.html>) for other interested parties like the State Medicaid agencies and managed care organizations. The fee schedule for parenteral and enteral nutrition (PEN) is released to the PDAC and DME MACs in a separate file. All annual updates to fee schedules are to be implemented on January 1 for claims with dates of service on or after January 1.

Effective February 28, 2022, national fee schedule amounts for new DMEPOS items and services that are added to the DMEPOS or PEN fee schedule files are established by CMS using the process summarized below and outlined in the regulations at 42 CFR 414.114 and 414.240. As explained in more detail in §60.3 below, MACs shall establish interim local fee schedule amounts for use in paying claims for new items and services billed using codes for miscellaneous items and services and for use in paying claims on an interim basis until the national fee schedule amounts are established.

#### Process for Establishing National Fee Schedule Amounts – Effective February 28, 2022

If a preliminary determination is made by CMS that a new item or service without a fee schedule pricing history falls under a DMEPOS benefit category, CMS makes a preliminary payment determination for the item or service and posts the preliminary payment determination on the website [www.CMS.gov](http://www.CMS.gov) approximately 2 weeks prior to a public meeting. After consideration of public consultation provided at the public meeting on the preliminary benefit category and payment determinations for the item or service, CMS posts final determinations on the website [www.CMS.gov](http://www.CMS.gov) and implements the benefit category determinations and payment determinations for items and services through program instructions. Some determinations may be deferred to future cycles of the public consultation process if more time is needed to develop preliminary or final benefit category and payment determinations.

Benefit category determinations are made using the public meeting process described above for an item or service that is the subject of a request for a HCPCS code set modification. Benefit category determinations are also made using the public meeting process in cases where a manufacturer for a new item or service is not requesting a HCPCS code set modification for the item or service, but requests CMS to determine whether the item or service falls under a DMEPOS benefit category. Benefit category determinations are also made using the public meeting process when CMS identifies a need to address whether an item or service falls under a DMEPOS benefit category.

### **60.1 - Record Layout for DMEPOS Fee Schedule**

**(Rev. 12991; Issued: 12-13-24; Effective: 01-01-25; Implementation: 01-06-25)**

Sort Sequence: Category, HCPCS, 1st Modifier, 2nd Modifier State

| Field Name        | Pic     | Position | Comment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
|-------------------|---------|----------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Year              | X(4)    | 1 - 4    | Applicable Update Year                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
| HCPCS Code        | X(5)    | 5 - 9    | All current year active and deleted codes subject to DMEPOS floors and ceilings                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| 1st Modifier      | X(2)    | 10 - 11  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| 2nd Modifier      | X(2)    | 12 - 13  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| Jurisdiction      | X       | 14       | D = DME MAC Jurisdiction<br>L = Local A/B MAC (B) jurisdiction<br>J = Joint DME MAC/A/B MAC (B) jurisdiction                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| Category          | X(2)    | 15 - 16  | IN = Inexpensive/Routinely Purchased<br>FS = Frequently Serviced<br>CR = Capped Rental<br>OX = Oxygen & Oxygen Equipment<br>OS = Ostomy, Tracheostomy & Urologicals<br>SD = Surgical Dressings<br>PO = Prosthetics & Orthotics<br>SU = Supplies<br>TE = TENS<br>TS = Therapeutic Shoes<br>SC = Splints and Casts<br>IL = Intraocular Lenses<br>LC = Lymphedema Compression Treatment<br>Items<br>LT = Labor Rate                                                                                                                                                                                                                                                                                                   |
| HCPCS Action      | X       | 17       | Indicates active/delete status in HCPCS file<br>A = Active Code<br>D = Deleted Code, price provided for grace period processing only                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| Region            | X(2)    | 18 - 19  | This amount is not used for pricing claims. It is on file for informational purposes.<br>00 = For all non Prosthetic and Orthotic Services<br>01 - 10 = For Prosthetic and Orthotic Services only. This field denotes the applicable regional fee schedule.                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| State             | X(2)    | 20 - 21  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| Original Base Fee | 9(5)V99 | 22 - 28  | This amount is not used for pricing claims. It is on file for informational purposes. For capped rental services, this amount represents the base fee after adjustments for rebasing and statewide conversions. The base year for E0607 and L8603 is 1995. Since pricing amounts for E1405 and E1406 are developed by summing pricing amounts from source codes, they do not have a true base fee. For these codes, this field will be filled with zeros.<br>Beginning January 1, 2016, this field will include the Competitive Bidding Rural Fee Amount for specific HCPCS codes. A new indicator field (RURAL FEE INDICATOR) shall be populated with a value of "R" when the Rural Fee is present in this field. |

|                             |         |         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|-----------------------------|---------|---------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Ceiling                     | 9(5)V99 | 29 - 35 | <p>This amount is not used for pricing claims. It is on file for informational purposes and could be integrated into other processes (i.e., IR review, validation, inquiries). <b>NOTE:</b> Since E0607 is priced via national IR, it is not priced using floors and ceilings. For E0607, this field will be filled with zeros. Since pricing amounts for E1405 and E1406 are developed by summing pricing amounts from source codes, they are not subject to ceilings and floors. Since non-mail order (no-KL) codes A4233, A4234, A4235, A4236, A4253, A4256, A4258, and A4259 are priced using National Mail order single payment amounts, they are not subject to ceilings and floors. Splints, casts and intraocular lenses are national fee schedule amounts not subject to ceilings. For these codes, this field will be filled with zeros. Beginning January 1, 2016, this field will also be filled with zeros for codes whose fees are adjusted using DMEPOS competitive bid information. Labor rates and lymphedema compression treatment items are not subject to ceilings.</p> |
| Floor                       | 9(5)V99 | 36 - 42 | <p>This amount is not used for pricing claims. It is on file for informational purposes and could be integrated into other processes (i.e., IR review, validation, inquiries). <b>NOTE:</b> Since E0607 is priced via national IR, it is not priced using floors and ceilings. For E0607, this field will be filled with zeros. Since pricing amounts for E1405 and E1406 are developed by summing pricing amounts from source codes, they are not subject to ceilings and floors. Since non-mail order (no-KL) codes A4233, A4234, A4235, A4236, A4253, A4256, A4258, and A4259 are priced using National Mail order single payment amounts, they are not subject to ceilings and floors. Splints, casts and intraocular lenses are national fee schedule amounts not subject to floors. For these codes, this field will be filled with zeros. Beginning January 1, 2016, this field will also be filled with zeros for codes whose fees are adjusted using DMEPOS competitive bid information. Labor rates and lymphedema compression treatment items are not subject to floors.</p>     |
| Updated Fee Schedule Amount | 9(5)V99 | 43 - 49 | Amount used for pricing.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| Filler                      | X       | 50      | Set to spaces                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| Pricing Change Indicator    | X       | 51      | <p>0 = No change to the updated fee schedule amount since previous release.</p> <p>1 = A change has occurred to the updated fee schedule amount since the previous release.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
| Rural Fee Indicator         | X(01)   |         | R = Rural Fee is present in the Original Base Fee field                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |

## **60.2 - Quarterly Update Schedule for DMEPOS Fee Schedule**

**(Rev. 2902, Issued: 03-11-14, Effective: 04-01-14, Implementation: 04-07-14)**

Following is an approximate schedule for making additions (for new HCPCS codes) and corrections to base-year amounts for the DMEPOS fee schedule.

- The DME MACs identify instances where base year fees are incorrect and forward requests for revisions to their regional offices. The DME MACs also identify those instances where fee schedule amounts are replaced by inherent reasonableness (IR) limits/payment amounts, should the authority for making IR adjustments be restored. A/B MACs (B) and DME MACs must use the file layout in §60.1 above to submit all revisions. Regional offices will review those requests and, upon concurrence, forward them to the Division of Data Systems (DDS) and the Division of DMEPOS Policy (DDP) in CMS Central Office. Those transmissions must occur within the timeframes established by CMS.
- Requests for revisions must be accompanied by a narrative description.
- For inherent reasonableness (IR) changes, the effective date of the revised payment amount must be provided. The format provides a field for those dates.
- DDS will recalculate the current year fee schedule amounts as appropriate.
- DDS will transmit the entire DMEPOS file to the DME MACs, PDAC, and A/B MACs using the file layout described in §60.1 above. An indicator in the record field will identify those instances where pricing amounts have changed. These transmissions must occur within the dates specified each year by CMS. DDP must also receive a copy of the corrected fees.
- Concurrently, DDP issues instructions for implementing the revised fee schedule amounts.
- DME MACs and A/B MACs should give providers 30 days' notice before revised payment amounts are implemented. Dates for implementation are provided by CMS.
- DME MACs and A/B MACs should make adjustments on those claims that were processed incorrectly if brought to their attention. Adjustments may be made retroactively to January 1 unless otherwise specified.
- CMS will furnish the revised payment amounts to RRB, Indian Health Service and United Mine Workers. DME MACs and A/B MACs (A), (B), and (HHH), shall provide the data to the State Medicaid Agencies.

- Fee Schedule Disclaimer: Whenever the MACs publish the DMEPOS fee schedule in their bulletins/notices, a disclaimer must be added. The disclaimer is, “Inclusion or exclusion of a fee schedule amount for an item or service does not imply any health insurance coverage.”
- CMS will release specific timeframes for quarterly changes for DMEPOS Fees.

### **60.3 - Gap-filling DMEPOS Fees**

**(Rev. 13610; Issued: 01-30-26; Effective: 01-01-26; Implementation: 01-05-26)**

Gap-filling is used in establishing fee schedule amounts for new DMEPOS items or services that do not have a fee schedule pricing history. If a HCPCS code is new and describes items and services that have a fee schedule pricing history (classified and paid for previously under a different code, including codes for miscellaneous items, e.g., E1399, and including fee schedule amounts established by CMS or the MACs), the fee schedule amounts for the new code are established using the process included in section 60.3.1 of this manual.

All DMEPOS items and services subject to payment on a fee schedule basis as mandated by sections 1833(o)(2)(A), 1834(a), (h), and (i) of the Social Security and/or by regulations at 42 CFR 414.102 and 414.210 must have national fee schedule amounts established by CMS or interim local fee schedule amounts established by the MACs for use in paying claims for the items and services. Effective February 28, 2022, interim local fee schedule amounts established by the MACs for paying claims on an interim basis are considered a fee schedule pricing history for continuity of pricing purposes under §60.3.1 below for the time before national fee schedule amounts are established, but can be considered by CMS in developing national fee schedule amounts. Once national fee schedule amounts are established for an item or service, the national fee schedule amounts become the new fee schedule pricing history for the item or service for continuity of pricing purposes under §60.3.1 below. Local fee schedule amounts established by the MACs for use in paying claims prior to February 28, 2022 are considered a fee schedule pricing history for continuity of pricing purposes under §60.3.1 below.

The DME MACs or A/B MACs must establish fee schedule amounts for DMEPOS items and services billed using HCPCS codes for miscellaneous items not otherwise classified under the HCPCS (e.g., E1399, L2999, and L8699). Once the fee schedule amounts are established for DMEPOS items and services billed using HCPCS codes for miscellaneous items, these fee schedule amounts would only change when update factors are applied, to correct an error in the calculation of the fee schedule amounts, for based on program instructions.

For DME items, the DME MACs must apply the DME payment method depending on the DME class the item falls under (e.g., the item would be paid on a capped rental basis if it is expensive, not customized, not oxygen and oxygen equipment, and does not require frequent and substantial servicing in order to avoid risk to the patient).

National fee schedule amounts established by CMS and interim local fee schedule amounts established by the DME MACs and A/B MACs Part B shall be gap-filled for items for which charge data were unavailable during the fee schedule data base year using the fee schedule amounts for comparable equipment. Fee schedule amounts for new HCPCS codes for items and services without a fee schedule pricing history are established using existing fee schedule amounts for comparable items when items with existing fee schedule amounts are determined to be comparable to the new items and services. A comparison can be based on, but not limited to the following components: physical, mechanical, electrical, function and intended use, and additional attributes and features. When examining whether an item is comparable to another item, the analysis can be based on the items as a whole, its subcomponents, or a combination of items. A new product does not need to be comparable within each category, and there is no prioritization to the categories.

#### Examples of Attributes in Each Component Category

- Physical: Aesthetics, Design, Customized vs. Standard, Material, Portable, Size, Temperature Range/Tolerance, Weight
- Mechanical: Automated vs. Manual, Brittleness, Ductility, Durability, Elasticity, Fatigue, Flexibility, Hardness, Load Capacity, Flow-Control, Permeability, Strength
- Electrical: Capacitance, Conductivity, Dielectric Constant, Frequency, Generator, Impedance, Piezo-electric, Power, Power Source, Resistance
- Function and Intended Use: Function, Intended Use
- Additional Attributes and Features: “Smart”, Alarms, Constraints, Device Limitations, Disposable, Parts, Features, Invasive vs. Non-Invasive.

If unable to identify comparable item(s), other sources of pricing data can be used to calculate the gap-filled fee schedule amount for the new item. These sources include using supplier or commercial price lists with prices in effect during the fee schedule data base year. Data base “year” refers to the time period mandated by the statute and/or regulations from which Medicare allowed charge data is to be extracted in order to compute the fee schedule amounts for the various DMEPOS payment categories. For example, the fee schedule base year for inexpensive or routinely purchased durable medical equipment is the 12 month period ending June 30, 1987. Supplier price lists include catalogues and other retail price lists (such as internet retail prices) that provide information on commercial pricing for the item. Potential appropriate sources for such commercial pricing information can also include payments made by Medicare Advantage plans as well as verifiable information from supplier invoices and non-Medicare payer data (e.g., fee schedule amounts comprised of the median of the commercial pricing information adjusted as described below). DME MACs and A/B MACs shall gap-fill based on current instructions released each year for implementing and updating the payment amounts.

If the only available price information is from a period other than the base period, apply the deflation factors that are included in the current year implementation instructions against current pricing in order to approximate the base year price for gap-filling purposes.

The deflation factors for gap-filling purposes are shown below:

| <b>Year*</b> | <b>OX</b> | <b>CR</b> | <b>PO</b> | <b>SD</b> | <b>PE</b> | <b>SC</b> | <b>IL</b> |
|--------------|-----------|-----------|-----------|-----------|-----------|-----------|-----------|
| 1987         | 0.965     | 0.971     | 0.974     | n/a       | n/a       | n/a       | n/a       |
| 1988         | 0.928     | 0.934     | 0.936     | n/a       | n/a       | n/a       | n/a       |
| 1989         | 0.882     | 0.888     | 0.890     | n/a       | n/a       | n/a       | n/a       |
| 1990         | 0.843     | 0.848     | 0.851     | n/a       | n/a       | n/a       | n/a       |
| 1991         | 0.805     | 0.810     | 0.813     | n/a       | n/a       | n/a       | n/a       |
| 1992         | 0.781     | 0.786     | 0.788     | n/a       | n/a       | n/a       | n/a       |
| 1993         | 0.758     | 0.763     | 0.765     | 0.971     | n/a       | n/a       | n/a       |
| 1994         | 0.740     | 0.745     | 0.747     | 0.947     | n/a       | n/a       | n/a       |
| 1995         | 0.718     | 0.723     | 0.725     | 0.919     | n/a       | n/a       | n/a       |
| 1996         | 0.699     | 0.703     | 0.705     | 0.895     | 0.973     | n/a       | n/a       |
| 1997         | 0.683     | 0.687     | 0.689     | 0.875     | 0.951     | n/a       | n/a       |
| 1998         | 0.672     | 0.676     | 0.678     | 0.860     | 0.936     | n/a       | n/a       |
| 1999         | 0.659     | 0.663     | 0.665     | 0.844     | 0.918     | n/a       | n/a       |
| 2000         | 0.635     | 0.639     | 0.641     | 0.813     | 0.885     | n/a       | n/a       |
| 2001         | 0.615     | 0.619     | 0.621     | 0.788     | 0.857     | n/a       | n/a       |
| 2002         | 0.609     | 0.613     | 0.614     | 0.779     | 0.848     | n/a       | n/a       |
| 2003         | 0.596     | 0.600     | 0.602     | 0.763     | 0.830     | n/a       | n/a       |
| 2004         | 0.577     | 0.581     | 0.582     | 0.739     | 0.804     | n/a       | n/a       |
| 2005         | 0.563     | 0.567     | 0.568     | 0.721     | 0.784     | n/a       | n/a       |
| 2006         | 0.540     | 0.543     | 0.545     | 0.691     | 0.752     | n/a       | n/a       |
| 2007         | 0.525     | 0.529     | 0.530     | 0.673     | 0.732     | n/a       | n/a       |
| 2008         | 0.500     | 0.504     | 0.505     | 0.641     | 0.697     | n/a       | n/a       |
| 2009         | 0.508     | 0.511     | 0.512     | 0.650     | 0.707     | n/a       | n/a       |
| 2010         | 0.502     | 0.506     | 0.507     | 0.643     | 0.700     | n/a       | n/a       |
| 2011         | 0.485     | 0.488     | 0.490     | 0.621     | 0.676     | n/a       | n/a       |
| 2012         | 0.477     | 0.480     | 0.482     | 0.611     | 0.665     | n/a       | n/a       |
| 2013         | 0.469     | 0.472     | 0.473     | 0.600     | 0.653     | n/a       | 0.983     |
| 2014         | 0.459     | 0.462     | 0.464     | 0.588     | 0.640     | 0.980     | 0.963     |
| 2015         | 0.459     | 0.462     | 0.463     | 0.588     | 0.639     | 0.978     | 0.962     |
| 2016         | 0.454     | 0.457     | 0.458     | 0.582     | 0.633     | 0.969     | 0.952     |
| 2017         | 0.447     | 0.450     | 0.451     | 0.572     | 0.623     | 0.953     | 0.937     |
| 2018         | 0.435     | 0.437     | 0.439     | 0.556     | 0.605     | 0.927     | 0.911     |
| 2019         | 0.427     | 0.430     | 0.431     | 0.547     | 0.595     | 0.912     | 0.896     |
| 2020         | 0.425     | 0.427     | 0.429     | 0.544     | 0.592     | 0.906     | 0.891     |
| 2021         | 0.403     | 0.406     | 0.407     | 0.516     | 0.561     | 0.859     | 0.845     |
| 2022         | 0.370     | 0.372     | 0.373     | 0.473     | 0.515     | 0.788     | 0.774     |
| 2023         | 0.359     | 0.361     | 0.362     | 0.460     | 0.500     | 0.765     | 0.752     |
| 2024         | 0.349     | 0.351     | 0.352     | 0.446     | 0.485     | 0.743     | 0.730     |
| 2025         | 0.339     | 0.342     | 0.343     | 0.435     | 0.473     | 0.724     | 0.711     |

\* Year price in effect

Payment Category Key:

OX Oxygen & oxygen equipment (DME) CR

Capped rental (DME)

IN Inexpensive/routinely purchased (DME) FS

Frequently serviced (DME)

SU DME supplies

PO Prosthetics & orthotics SD

Surgical dressings

OS Ostomy, tracheostomy, and urological supplies PE

Parental and enteral nutrition

TS Therapeutic Shoes SC

Splints and Casts

IL Intraocular Lenses inserted in a physician's office

IN, FS, OS and SU category deflation factors=PO deflation factors

After deflation, the result must be increased by 1.7 percent and by the annual update factors shown below.

**DMEPOS Fee Schedule Update Factors for Gap-Filling Purposes (Updates applied to  
1986/87 base year amounts unless otherwise noted)**

**Update factors mandated by sections 1834(a)(14), 1834(h)(1)(E), 1834(h)(4)(A), 1834(i)(1)(B), and  
1842(s)(1)(B) of the Social Security Act**

| <b>Year</b> | <b>Class III<br/>DME</b> | <b>Other DME,<br/>Ostomy,<br/>Tracheostomy, and<br/>Urological Supplies</b> | <b>Parenteral and<br/>Enteral<br/>Nutrition<sup>1</sup></b> | <b>Surgical<br/>Dressings<sup>2</sup></b> | <b>Prosthetics,<br/>Orthotics, and<br/>Other Prosthetic<br/>Devices<sup>3</sup></b> |
|-------------|--------------------------|-----------------------------------------------------------------------------|-------------------------------------------------------------|-------------------------------------------|-------------------------------------------------------------------------------------|
| 1989        | 1.7%                     | 1.7%                                                                        | n/a                                                         | n/a                                       | 1.7%                                                                                |
| 1990        | 0.0%                     | 0.0%                                                                        | n/a                                                         | n/a                                       | 0.0%                                                                                |
| 1991        | 3.7%                     | 3.7%                                                                        | n/a                                                         | n/a                                       | 0.0%                                                                                |
| 1992        | 3.7%                     | 3.7%                                                                        | n/a                                                         | n/a                                       | 4.7%                                                                                |
| 1993        | 3.1%                     | 3.1%                                                                        | n/a                                                         | 3.1%                                      | 3.1%                                                                                |
| 1994        | 3.0%                     | 3.0%                                                                        | n/a                                                         | 3.0%                                      | 0.0%                                                                                |
| 1995        | 2.5%                     | 2.5%                                                                        | n/a                                                         | 2.5%                                      | 0.0%                                                                                |
| 1996        | 3.0%                     | 3.0%                                                                        | n/a                                                         | 3.0%                                      | 3.0%                                                                                |
| 1997        | 2.8%                     | 2.8%                                                                        | n/a                                                         | 2.8%                                      | 2.8%                                                                                |
| 1998        | 0.0%                     | 0.0%                                                                        | n/a                                                         | 0.0%                                      | 1.0%                                                                                |

|      |       |       |       |       |       |
|------|-------|-------|-------|-------|-------|
| 1999 | 0.0%  | 0.0%  | n/a   | 0.0%  | 1.0%  |
| 2000 | 0.0%  | 0.0%  | n/a   | 0.0%  | 1.0%  |
| 2001 | 3.7%  | 3.7%  | n/a   | 3.7%  | 3.7%  |
| 2002 | 0.0%  | 0.0%  | 0.0%  | 0.0%  | 1.0%  |
| 2003 | 1.1%  | 1.1%  | 1.1%  | 1.1%  | 1.1%  |
| 2004 | 2.1%  | 0.0%  | 2.1%  | 0.0%  | 0.0%  |
| 2005 | 3.3%  | 0.0%  | 3.3%  | 0.0%  | 0.0%  |
| 2006 | 2.5%  | 0.0%  | 2.5%  | 0.0%  | 0.0%  |
| 2007 | 0.0%  | 0.0%  | 4.3%  | 0.0%  | 4.3%  |
| 2008 | 2.7%  | 0.0%  | 2.7%  | 0.0%  | 2.7%  |
| 2009 | 5.0%  | 5.0%  | 5.0%  | 5.0%  | 5.0%  |
| 2010 | 0.0%  | 0.0%  | 0.0%  | 0.0%  | 0.0%  |
| 2011 | -0.1% | -0.1% | -0.1% | -0.1% | -0.1% |
| 2012 | 2.4%  | 2.4%  | 2.4%  | 2.4%  | 2.4%  |
| 2013 | 0.8%  | 0.8%  | 0.8%  | 0.8%  | 0.8%  |
| 2014 | 1.0%  | 1.0%  | 1.0%  | 1.0%  | 1.0%  |
| 2015 | 1.5%  | 1.5%  | 1.5%  | 1.5%  | 1.5%  |
| 2016 | -0.4% | -0.4% | -0.4% | -0.4% | -0.4% |

|      |      |      |      |      |      |
|------|------|------|------|------|------|
| 2017 | 0.7% | 0.7% | 0.7% | 0.7% | 0.7% |
| 2018 | 1.1% | 1.1% | 1.1% | 1.1% | 1.1% |
| 2019 | 2.3% | 2.3% | 2.3% | 2.3% | 2.3% |
| 2020 | 0.9% | 0.9% | 0.9% | 0.9% | 0.9% |
| 2021 | 0.2% | 0.2% | 0.2% | 0.2% | 0.2% |
| 2022 | 5.1% | 5.1% | 5.1% | 5.1% | 5.1% |
| 2023 | 8.7% | 8.7% | 8.7% | 8.7% | 8.7% |
| 2024 | 2.6% | 2.6% | 2.6% | 2.6% | 2.6% |
| 2025 | 2.4% | 2.4% | 2.4% | 2.4% | 2.4% |
| 2026 | 2.0% | 2.0% | 2.0% | 2.0% | 2.0% |

<sup>1</sup> Base Year is 1995 <sup>2</sup>

Base Year is 1992

<sup>3</sup> Artificial legs, arms, and eyes (prosthetics); leg, arm, back, and neck braces (orthotics); and all other prosthetic devices other than ostomy, tracheostomy, and urological supplies, parenteral and enteral nutrition, and intraocular lenses inserted in a physician's office

Note that when gap-filling for capped rental items, it is necessary to first gap-fill the purchase price and then compute the base period fee schedule at 10 percent of the base period purchase price.

For used equipment, establish fee schedule amounts at 75 percent of the fee schedule amount for new equipment.

Gap-filling is not used in establishing fee schedule amounts for new lymphedema compression treatment items that do not have a fee schedule pricing history. Additional information on payment for lymphedema compression treatment items is available at Pub. 100-04 Medicare Claims Processing Manual, Chapter 20, Section 181.1 Payment Policy for Lymphedema Compression Treatment Items.

If within 5 years of establishing fee schedule amounts using supplier or commercial prices, the supplier or commercial prices decrease by less than 15 percent, CMS can make a one- time adjustment to the fee schedule amounts using the new prices. The new supplier or commercial prices would be used to establish the new fee schedule amounts in the same way that the older prices were used, including application of the deflation formula of this section.

### **60.3.1 - Payment Concerns While Updating Codes**

**(Rev. 12068; Issued: 06-02-23; Effective: 07-01-23; Implementation: 07-03-23)**

The following instructions apply in situations where the CMS CO does NOT provide pricing guidance related to implementation of fee schedule for DMEPOS items and services.

Because a HCPCS code is new does not necessarily mean that Medicare payment on a fee schedule basis has never been made for the item and service described by the new code. In accordance with regulations at 42 CFR 414.110 and 414.236, if a new HCPCS code is added, CMS and/or MACs make efforts to determine whether the item and service has a fee schedule pricing history. If there is a fee schedule pricing history, the previous fee schedule amounts for the previous code(s) are mapped to the new code(s) to ensure continuity of pricing.

When the code for an item is divided into several codes for the components of that item, the total of the separate fee schedule amounts established for the components must not be higher than the fee schedule amount for the original item. When there is a single code that describes two or more distinct complete items (for example, two different but related or similar items), and separate codes are subsequently established for each item, the fee schedule amounts that applied to the single code continue to apply to each of the items described by the new codes. When the codes for the components of a single item are combined in a single global code, the fee schedule amounts for the new code are established by totaling the fee schedule amounts used for the components (that is, use the total of the fee schedule amounts for the components as the fee schedule amount for the global code). When the codes for several different items are combined into a single code, the fee schedule amounts for the new code are established using the average (arithmetic mean), weighted by allowed services, of the fee schedule amounts for the formerly separate codes.

### **60.4 - Process for Submitting Revisions to DMEPOS Fee Schedule to CMS**

**(Rev. 12068; Issued: 06-02-23; Effective: 07-01-23; Implementation: 07-03-23)**

The DME MACs identify instances where revisions to DMEPOS fees are needed and forward requests to CMS via email to the Division of Data Systems. The information must be contained in an ASCII file. The requests must include a narrative description.

The following file specifications are 2003 examples, the actual file names may change each year:

|               |              |                           |
|---------------|--------------|---------------------------|
| Data Set Name | DMEREV1A.TXT | First Quarter Submission  |
|               | DMEREV1B.TXT | Second Quarter Submission |

DMEREV1C.TXT      Third Quarter Submission

DMEREV1D.TXT      Fourth Quarter Submission

**Record Format**

| <b>Field Name</b>                   | <b>PIC</b> | <b>Position</b> | <b>Comment</b>                                                                                                                                                                                                                                                                                                   |
|-------------------------------------|------------|-----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| HCPCS Code                          | X(5)       | 1 - 5           |                                                                                                                                                                                                                                                                                                                  |
| Filler                              | X(1)       | 6 - 6           | Set to Spaces                                                                                                                                                                                                                                                                                                    |
| First Modifier                      | X(2)       | 7 - 8           |                                                                                                                                                                                                                                                                                                                  |
| Filler                              | X(1)       | 9               | Set to Spaces                                                                                                                                                                                                                                                                                                    |
| Second Modifier                     | X(2)       | 10 - 11         |                                                                                                                                                                                                                                                                                                                  |
| Filler                              | X(2)       | 12 - 13         | Set to Spaces                                                                                                                                                                                                                                                                                                    |
| State                               | X(3)       | 14 - 16         |                                                                                                                                                                                                                                                                                                                  |
| Filler                              | X(1)       | 17              | Set to Spaces                                                                                                                                                                                                                                                                                                    |
| Revised Base Fee                    | S9(5)V99   | 18 - 26         | 1992 level for surgical dressings;<br>1995 level for PEN;<br>2013 level for Splints & Casts;<br>2012 level for certain IOLs;<br>1989 for all other categories                                                                                                                                                    |
| Filler                              | X(1)       | 27              | Set to Spaces                                                                                                                                                                                                                                                                                                    |
| Capped Rental<br>Rebasing Indicator | X(1)       | 28              | For Capped Rental Services Only:<br>0 - IR not applied to original base<br>fee and base fee in effect prior to<br>1991, base fee is subject to<br>rebasing adjustment<br><br>1 - IR applied to original base fee<br>or base fee not in effect prior to<br>1991, base fee is exempted from<br>rebasing adjustment |
| Filler                              | X(1)       | 29              | Set to Spaces                                                                                                                                                                                                                                                                                                    |

| Field Name             | PIC  | Position | Comment                                                                                                                                                                           |
|------------------------|------|----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Nature of Fee Revision | X(1) | 30       | 0 = Correction<br>1 = IR Revision<br>2 = Other - Please submit supporting documentation.                                                                                          |
| Filler                 | X(1) | 31       | Set to Spaces                                                                                                                                                                     |
| IR - Effective Date    | 9(8) | 32 - 39  | Field is applicable only to those records where the fee has changed due to an inherent reasonableness decision and the previous field contains a value of "1." Format is YYYYMMDD |

The CMS will recalculate current year fee schedule amounts as appropriate and release the entire file in the record layout described in §60.1. An indicator in the record field (Pricing Change Indicator) will identify those instances where pricing amounts have changed. The CMS will also issue instructions for implementing the revised fee schedule amounts with the fee schedule.

### **60.5 – Rural ZIP Code Claim Record Layout for Medicare Contractors Processing Rural DMEPOS Fee Schedule Claims** (Rev. 3350, Issued: 09-11-15, Effective: 01-01-16, Implementation: 01-04-16)

This file contains rural area, as defined in 42 CFR §414.202, ZIP codes within the contiguous United States for use processing claims subject to the adjusted fee schedule methodology required by Section 1834(a)(1)(F) of the Social Security Act. The CMS will provide the specific file names when the rural ZIP code files are released.

| Field Name      | Pic   | Position | Comment                                                                                                                                                                                                                                                         |
|-----------------|-------|----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Effective Date  | 9(8)  | 1 - 8    | Effective date that ZIP code considered rural (CCYYMMDD).                                                                                                                                                                                                       |
| Expiration Date | 9(8)  | 9 - 16   | Final date for considering a ZIP code rural. Field will be filled with 99999999s if an end-date has not been established. Discontinued rural ZIP codes will be end dated as of the end of the quarter it was last an active rural ZIP. File Format is CCYYMMDD. |
| ZIP Code        | X(5)  | 17 - 21  | 5-digit ZIP code considered rural                                                                                                                                                                                                                               |
| State           | X(2)  | 22 - 23  |                                                                                                                                                                                                                                                                 |
| Filler          | X(17) | 24 -40   | Set to spaces                                                                                                                                                                                                                                                   |

### **70 - Parenteral and Enteral Nutrition (PEN) Fee Schedule**

**(Rev. 1, 10-01-03)**  
**PM B-01-54, AB-02-152**

The Balanced Budget Act of 1997 §4315 authorized the Secretary to implement a fee schedule for parenteral and enteral nutrition (PEN) items and services. These items were previously paid on a reasonable charge basis. The DME MACs will make payment based on the new PEN fee schedule effective for claims with dates of service on or after January 1, 2002.

The file layout for the PEN fee schedule is consistent with that of the DMEPOS fee schedule in §60.1 above except that “Region,” “Ceiling,” and “Floor” fields are not applicable and will be zero filled.

The CMS issues instructions for implementing an annual PEN fee schedule along with the DMEPOS fee schedule instructions. The PEN fee schedule is provided to DME MACs and the Pricing, Data Analysis, and Coding Contractor (PDAC) via CMS’ mainframe telecommunication system.

**70.1 - Record Layout for PEN Fee Schedule**

**(Rev. 3350, Issued: 09-11-15, Effective: 01-01-16, Implementation: 01-04-16)**

| <b>Field Name</b> | <b>Pic</b> | <b>Position</b> | <b>Comment</b>                                                                                                                                                                                                                                                                                                                                 |
|-------------------|------------|-----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Year              | X(4)       | 1 - 4           | Applicable Update Year                                                                                                                                                                                                                                                                                                                         |
| HCPCS Code        | X(5)       | 5 - 9           | All current year active and deleted codes                                                                                                                                                                                                                                                                                                      |
| 1st Modifier      | XX         | 10 - 11         |                                                                                                                                                                                                                                                                                                                                                |
| 2nd Modifier      | XX         | 12 - 13         |                                                                                                                                                                                                                                                                                                                                                |
| Jurisdiction      | X          | 14              | D = DME MAC Jurisdiction                                                                                                                                                                                                                                                                                                                       |
| Category          | XX         | 15 - 16         | PE = Parenteral and Enteral Nutrition.                                                                                                                                                                                                                                                                                                         |
| HCPCS Action      | X          | 17              | Indicates active/delete status in HCPCS file<br>A = Active Code<br>D = Deleted Code, price provided for grace period processing only                                                                                                                                                                                                           |
| Filler            | XX         | 18 - 19         | Value = 00                                                                                                                                                                                                                                                                                                                                     |
| State             | XX         | 20 - 21         |                                                                                                                                                                                                                                                                                                                                                |
| Original Base Fee | 9(5)V99    | 22 - 28         | This amount is not used for pricing claims. It is on file for informational purposes.<br>Beginning January 1, 2016, this field will include the Competitive Bidding Rural Fee Amount for specific HCPCS codes. A new indicator field (RURAL FEE INDICATOR) shall be populated with a value of “R” when the Rural Fee is present in this field. |
| Filler            | 9(5)V99    | 29 - 35         | This field is zero filled.                                                                                                                                                                                                                                                                                                                     |

| <b>Field Name</b>           | <b>Pic</b> | <b>Position</b> | <b>Comment</b>                                                                                                                                                       |
|-----------------------------|------------|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Filler                      | 9(5)V99    | 36 - 42         | This field is zero filled.                                                                                                                                           |
| Updated Fee Schedule Amount | 9(5)V99    | 43 - 49         | Amount used for pricing.                                                                                                                                             |
| Gap-Fill Indicator          | X          | 50              | 0 = No Gap-filling required.<br>1 = A/B MAC (B) Needs to Gap-fill Original Base Year Amount.                                                                         |
| Pricing Change Indicator    | X          | 51              | 0 = No change to the updated fee schedule amount since previous release.<br>1 = A change has occurred to the updated fee schedule amount since the previous release. |
| Rural Fee Indicator         | X(01)      | 52              | R = Rural Fee is present in the Original Base Fee field                                                                                                              |
| Filler                      | X(8)       | 53 - 60         |                                                                                                                                                                      |

**80 - Reasonable Charges as Basis for A/B MAC (B)/DME MAC Payments (Rev. 1, 10-01-03)**  
**B3-5000, AB-01-118, AB-02-136, B-02-089**

Effective with services furnished on or after January 1, 1992, A/B MACs (B) pay for physicians' services based on a fee schedule. Nonphysician services are also paid based on a percentage of fee schedule amounts depending on the type of nonphysician and service rendered. Other services and supplies (e.g., DMEPOS) are paid under the fee schedule designed specifically for those services. Beginning in 1992, only the services listed below are paid under the reasonable charge methodology.

Where payment continues to be made on a reasonable charge basis for items and services, other than ambulance and laboratory services, A/B MACs (B) compute customary and prevailing charge updates at the beginning of each fee screen year (the 12-month period beginning January), using available statistics on charges for services from claims processed or services rendered during the 12 month period ending June 30 immediately preceding the start of the fee screen year. For example, the customary and prevailing charge rates established for fee screen year 2003 (January 1, 2003, through December 31, 2003) are based on the charges made from July 1, 2001, through June 30, 2002.

Instructions regarding payment for ambulance and for laboratory services still subject to reasonable charges are found in Medicare Claims Processing Manual, Chapter 15, "Ambulance," and Chapter 16, "Laboratory Services from Independent Labs, Physicians, and Providers." Additional instructions regarding payment for dialysis supplies and equipment are provided in the Medicare Claims Processing Manual, Chapter 8, "Outpatient ESRD Hospital, Independent Facility, and Physician/Supplier Claims."

See specific year Program Memorandum instructions for a list of codes that are subject to the inflation-indexed charge (IIC) each year. A/B MACs (B) compute each year's IIC screen, by increasing the appropriate previous year's screen by the percentage amount

specified in annual Program Memorandum instructions. A/B MACs (B) must not compute IIC screens for items paid using gap-filled payment amounts in the previous year. See specific year Program Memorandum instructions for any variances to these rules and for each year's specific increase percentage.

Services paid on a reasonable charge basis continue to be paid for under the rules in this section. A/B MACs (B)/DME MACs have the primary responsibility for determining reasonable charges. The following guidelines are intended to assure overall consistency among A/B MACs (B)/DME MACs with respect to the concepts applied in making reasonable charge determinations.

### **80.1 - Criteria for Determining Reasonable Charge** **(Rev. 1, 10-01-03)** **B3-5002, B-03-007**

There are two criteria in §1842 of the Act that must be considered in determining the reasonable charge for a service. They are:

- The customary charges for similar services generally made by the physician or other person furnishing such services; and
- The prevailing charges in the locality for similar services.

Therefore, the reasonable charge for a specific service in the absence of unusual medical complications or circumstances, may not exceed the lowest of:

- The customary charge for that service;
- The prevailing charge made for similar services in the locality; or
- The actual charge for the service. (See §80.8.)

The law also provides that the reasonable charge for a service not exceed the charge applicable for a comparable service under comparable circumstances to the contractor's policyholders or subscribers. (See §80.7.) The A/B MAC (B) or DME MAC also determines if the charge for the specific item or service is inherently reasonable.

The income of an individual patient may not be considered in determining the amount of the reasonable charge.

Public Law 96-499 requires that reasonable charge payments be based on customary and prevailing charge screens in effect on the date the service is rendered. However, if the service was rendered at any time prior to the current fee year, payment is based on the screens in effect during the preceding fee screen year.

To implement this provision, the A/B MAC (B) must complete the following activities:

- Retain the prior year's pricing files in the system so that reasonable charge pricing data is available for two years. As of July 1, 2003, all A/B MACs (B) operating the MCS system must retain at least five pricing files (current period plus four prior periods); and
- Price the service based on the date of service on the claim using pricing files in effect for the same year as the date of service.

## **80.2 - Updating Customary and Prevailing Charges**

**(Rev. 1, 10-01-03)**

**B3-5003**

A/B MACs (B)/DME MACs update customary and prevailing charge limits at the beginning of each fee screen year (the 12-month period beginning January), using available statistics on charges for services from claims processed or services rendered during the 12-month period ending June 30 immediately preceding the start of the fee screen year. For example, the limits used during fee screen year 2003 (January 1, 2003, through December 31, 2003) were based on the charges made from July 1, 2001, through June 30, 2002.

Customary and prevailing charge screens for a fee screen year are not changed except for the following reasons:

- In individually identified and highly unusual situations where equity clearly indicates that the increases are warranted and requested by the entity furnishing the service;
- To correct erroneous calculations; or
- To establish screens for new services.

A/B MACs (B)/DME MACs must ensure that all data on the frequency of services, all data files and customary and prevailing charge files used to establish reasonable charge screens are retained and are available for use.

## **80.3 - The Customary Charge**

**(Rev. 1, 10-01-03)**

**B3-5010, B3-5205**

The customary charge is the amount that best represents the actual charges made for a given medical service or by other persons who supply other medical and health services to the general public. Therefore, obtain information on the customary charges from the following sources:

- Physicians and other persons not only from the Medicare program;
- Any other available sources;
- Other contractor programs;
- Other insurance programs;

- Federal Employee Health Benefit Program;
- TriCare;
- Any studies conducted by State or local medical societies;
- From public agencies;
- Any data volunteered from other sources.

Where circumstances warrant, MACs may also ask physicians or other persons for their charges for services rendered to the public in general. MACs should validate any information on charges obtained from sources against claims.

#### **A. Charges for Rare or Unusual Procedures**

A/B MACs (B) may incur situations where a new or rare procedure is performed and information on customary and prevailing charges is difficult to obtain. In such situations, in order to make the reasonable charge determination:

- Obtain data, if possible, on the charges made for the unusual or rare procedure in other areas similar to the locality in which the service was rendered; or
- Consult with the local medical society regarding the appropriate charge to be made for this procedure.

A relative value scale may be used together with available information about the physician's customary charges and about the prevailing charges for more frequently performed services in the locality in order to fill gaps in the data available. Where there is insufficient information, consult with any medical authority that would be helpful, such as the medical personnel within the A/B MAC (B), the local or State medical society, or hospital medical personnel. In assessing the value of the procedure, the medical personnel should take into consideration:

- Its complexity;
- The time needed to perform the procedure; and
- The prevailing charges in the locality for other procedures of comparable complexity.

A/B MACs (B) then determine reasonable charge for a given service on the best available medical opinion and information on customary and prevailing charges.

### **80.3.1 - Calculating Customary Charge**

(Rev. 1, 10-01-03)

**B3-5010.1, B3-5213**

A/B MACs (B) and DME MACs use to the extent possible, the actual charges for services rendered during the year ending June 30 immediately preceding the start of the fee screen

year. Use data either from claims processed or from claims for services rendered during that 12-month period.

The A/B MAC (B) or DME MAC arrays each charge the physician, supplier, or another entity made for a service in ascending order. The lowest actual charge which is high enough to include the median of the arrayed charge data is then selected as the physician's or other person's customary charge for the service. Include charges made to a class of patients (such as members of a Preferred Provider Organization) in the array of charges used to compute the customary charge.

Customary charges may be established using price lists when there is inadequate charge data. In this case, use only the fees charged and the price lists in effect as of December 31 of the data base year. The intent is to use a price list which reasonably replicates the median of the prices charged by the supplier for his items and services during the data base year.

Where the MAC permitted an increase in a customary charge under the equity provision (see §80.3.1.1 below), the increased amount is recognized as the customary charge for the next fee screen year if it exceeds the median of charges made by the physician or other person for the service during the 12 months ending June 30 immediately preceding the start of that fee screen year. The increased amount is the correct customary charge for use in the appropriate prevailing charge calculation(s).

#### **A. Inclusion of Sales Taxes in Reasonable Charges**

Sales taxes where appropriate were included in the calculation of reasonable charges computed. They were also accounted for in the calculation of the base fee schedules for DME and orthotic/prosthetic devices. The Consumer Price Index used to update fee schedules also accounts for sales tax. Therefore, MACs do not make any additional payment for sales taxes and do not make adjustments in fees to reflect local changes in tax rates.

### **80.3.1.1 - Equity Adjustments in Customary Charge Screens**

**(Rev. 1, 10-01-03)**

#### **B3-5010.2**

Once the MAC establishes the customary charge screens for a fee screen year, further increases (other than to correct errors) are permitted only in individually identified and highly unusual situations where equity clearly indicates that the increases are warranted. Requests for revisions in customary charge profiles are initiated only by physicians or other persons furnishing covered services. Such requests are neither encouraged nor discouraged, and each request is handled on the basis of its own merits.

All of the following considerations are taken into account, as applicable, in determining whether unusual circumstances warrant a revision in a customary profile in a particular situation.

- Time elapsed since last change was made in customary charge
  - Identify if the last change made in a customary charge for the service was in the past two or three years. Generally, the more time that has elapsed since the last change the stronger the case is for recognizing the current charge.
  - Take into consideration that a physician or other entity did not increase fees charged for a service because of the government's request for restraint and this can be verified. This should be taken into account in determining whether unusual circumstances are present in a particular case.
- Consider the amount of the requested increase and the relationship of the new and old charges to the customary charges of other physicians or other persons in the locality for the service.
- Increases in Operating Expenses Used to Justify an Increase in Charges
  - The entity must specifically prove that increases in operating expenses are substantially above those resulting from general economic factors and the situation is unique.

### **80.3.2 - Customary Charge Profile**

**(Rev. 1, 10-01-03)**  
**B3-5010.3**

A/B MACs (B)/DME MACs establish and maintain adequate data on the customary charges for specific services and procedures made by individual physicians and other persons and organizations which render covered services. This data must be readily available. The customary charge record for each entity rendering covered services must include a minimum of the following data:

- Entity name, address, identification code, locality and specialty status;
- A continuing history of the entity's charges for specific services, supplies, etc;
- A customary charge for each specific service or item derived from the entity's history of charges for that service or item (and which is to be used as the basis for the determination of reasonable charges in conjunction with the prevailing charge criterion); and
- For a charge made under unusual circumstances, include a description of the circumstances.

A/B MACs (B)/DME MACs develop and include in the entity's profile a customary charge conversion factor for use with a relative value scale when there is insufficient data to

establish the actual customary charge or when an infrequently performed service or item is involved.

## **80.4 - Prevailing Charge**

**(Rev. 1, 10-01-03)**

**B3-5020**

Prevailing charges are those charges that fall within the range of charges most frequently and widely used in a locality for a particular procedure or service. The top of this range establishes an overall limitation on the charges that the A/B MAC (B) accepts as reasonable for a given procedure or service, except where unusual circumstances or medical complications warrant an additional charge.

For any fee screen year, the prevailing charge limit in a locality for a service must be calculated as the 75th percentile of the customary charges determined for that service. Array each customary charge for the service in ascending order and weight by how often the physician or other person rendered the service (as reflected by the charge data used to calculate the customary charge. The lowest customary charge which is high enough to include the customary charges of the physicians or other persons who rendered 75 percent of the cumulative services is then determined as the prevailing charge for the service. The proper procedure for establishing revised prevailing charge screens based on the 75th percentile is illustrated by the following example:

| <b>Customary Charge</b> | <b>Number of the item/<br/>service rendered</b> | <b>Cumulative Services</b> |
|-------------------------|-------------------------------------------------|----------------------------|
| \$5                     | 1402                                            | 1402                       |
| \$6                     | 1115                                            | 2517                       |
| \$7                     | 1680                                            | 4197                       |
| \$8                     | 803                                             | 5000                       |
| Total 5000              |                                                 |                            |

In the above example, 75 percent of the total of 5,000 services equals 3,750 services. The prevailing charge is, therefore, \$7. (A total of 2,517 services were rendered with \$5 and \$6 customary charges, and an additional 1,680 services were rendered with \$7 customary charges. The 3,750th service was thus rendered with a \$7 customary charge.)

A/B MACs (B)/DME MACs establish four customary charges as the minimum number of customary charges for the same service needed to calculate the prevailing charge for a particular service in a locality or in an A/B MAC (B) service area.

If it is necessary to establish customary charges using price lists, A/B MACs (B)/DME MACs use these customary charges to also establish the required prevailing charges. (See §80.3.1.) If the A/B MAC (B)/DME MAC cannot derive precise data on the frequency of services from their records, they may use any information they have about the volume of business done by various suppliers in their area in order to weight the customary charges used to calculate the prevailing charges.

If the A/B MAC (B)/DME MAC has not established a locality prevailing charge for the service by specialty or group of specialties, they select the locality prevailing charge from the remaining sources of data (items 2 through 5 below inclusive). That sequence must be followed in strict order except as otherwise permitted by item 6. The first charge available in the sequence of sources of data is selected as the locality prevailing charge. Payments must be based on the fully adjusted locality prevailing charges.

#### Approved Sources of Data in the Required Sequence

- 1 Prevailing charge by locality and by specialty or group of specialties;
- 2 Prevailing charge by A/B MAC (B) service area and by specialty or group of specialties;
- 3 Prevailing charge by locality without regard to specialty;
- 4 Prevailing charge by A/B MAC (B) service area without regard to specialty; and
- 5 Array of actual charges - Where items 1 through 4 have not produced an acceptable prevailing charge and where at least four physicians or other persons in your service area (without regard to locality or specialty) have submitted charges for a service (but four “qualified customary charges” are not available), array all the available actual charges for that service. Set the prevailing charge for that service at the 75th percentile of that array.
- 6 Item 4 may be omitted with RO approval from the sources of data listed above, e.g., because this simplifies claims processes and reduces the related expenses. Also, when there is insufficient charge data to establish valid prevailing charges for substantial numbers of services or items, request RO assistance in working out mergers of charge data with that from other A/B MACs (B). Use another A/B MAC (B)’s data that has a similar service area with regards to medical prices, and other economic and demographic characteristics. Such data mergers must receive prior approval from CMS.

The steps described above may not always achieve a perfect result. However, beneficiaries, physicians, and suppliers have the right to question the determination in a given case. In such instances, A/B MACs (B) should use their best judgment based on the available data to resolve the issue.

The CMS publishes A/B MAC (B) and DME MAC localities. These designations should be used in making reasonable charge determinations. Where appropriate, multiple localities may be used with respect to different types and levels of services. This may occur where there are not enough members of a specialty group in any one locality to establish a valid basis for deriving the prevailing charges for their services. Prior CO approval must be obtained when deviating from the established CMS localities.

### **80.4.1 - Rounding of Reasonable Charge Calculation** **(Rev. 1, 10-01-03)** **B3-5021**

A/B MAC (B) screens should include both dollars and cents. Rounding of customary and prevailing charges should not normally take place with one exception.

Rounding is permitted in situations where the A/B MAC (B)'s customary charge calculation is clearly erroneous and results in a peculiar or odd cent amount, e.g., \$8.33. In such an instance, rounding to the nearest 10 cents is permissible, pending the required correction of the calculation, where the data result in a customary charge that is not in 25-cent or 75-cent units. For other than the 25-cent or 75-cent amounts, fractional amounts ending in 5-cent increments should be rounded up to the nearest 10 cents. However, where the A/B MAC (B) knows the odd cents in a customary charge calculation are correct, e.g., because a sales tax is involved in the charges made by a physician, then no rounding of the odd cents should take place.

The above exception applies only to customary charge calculations, and not to prevailing charges. The exception is permissible only as an interim measure to deal with an immediate problem. The A/B MAC (B) should promptly identify the cause of the error and take appropriate corrective action.

There should be no need for rounding initial prevailing charge calculations since actual customary charges are used to establish these screens.

### **80.5 - Filling Gaps in A/B MAC (B) Reasonable Charge Screens** **(Rev. 1, 10-01-03)** **B3-5022, AB-02-136, B-02-089**

Where there is insufficient actual charge data for determining the customary charge and/or the prevailing charge in the locality for a particular medical procedure or service, A/B MACs (B)/DME MACs must use one of the following gap-filling methods to establish the needed screen.

#### **A. Gaps in Customary Charge Screens**

A/B MACs (B)/DME MACs fill a gap in a customary charge screen:

1. By applying a conversion factor derived from the physician's (or supplier's) known customary charges for other services in the same category of service (medicine, surgery, etc.) to a relative value scale in the manner suggested in §§80.3, or
2. By using the 50th percentile in the array of customary charges (weighted by frequency) that was used to establish the prevailing charges for similar services for the physicians in the same specialty and locality.

A/B MACs (B)/DME MACs must ensure that, where a 50th percentile charge is used as a customary charge for payment purposes, the reasonable charge does not exceed either that amount or the locality prevailing charge. Also, all A/B MACs (B)/DME MACs that use 50th percentile charges to fill gaps in their customary charge data must retain the capacity to calculate and use conversion factors with relative value scales when this is appropriate.

## **B. Gaps in Prevailing Charge Screens**

Where the A/B MAC (B) has not been able to establish a prevailing charge for a service based on the prevailing charge screen procedures, then A/B MACs (B) must gap-fill the payment amounts using a prevailing charge conversion factor (§80.5.1) and a relative value scale, manually if necessary. (See §80.3 regarding charges for rare or unusual procedures. This is to be used as a last resort.

### **80.5.1 - Use of Relative Value Scale and Conversion Factors for Reasonable Charge Gap-Filling (Rev. 1, 10-01-03) B3-5022.1**

The relative value scales used to fill gaps in charge data for the Medicare program should, to the extent possible, be those that contractors use in their own programs. Relative value scales developed by contractors or by medical societies for States other than those in which your Medicare service area is located should be carefully reviewed and validated before they are used. Ensure that a relative value scale, which is used to estimate customary charges or prevailing charges, accurately reflects charge patterns in the area you service. Similarly, the conversion factor used with the relative value scale should reflect the known customary charges of the physician or other person for whom a customary charge is being estimated, or the known prevailing charges for services in the locality, as appropriate.

As a result of consent agreements with the Federal Trade Commission (FTC), relative value scales formerly published by the American Academy of Orthopedic Surgeons, the American College of Obstetricians and Gynecologists, the American College of Radiology, and the California Medical Association have been withdrawn from circulation. The FTC consent agreements did not bar the use of the related procedural terminology and coding systems. However, do not use these four relative value scales to fill gaps in customary and prevailing charge data.

This does not mean that you must discontinue using relative value scales to fill gaps in customary charges or in determining prevailing charges. Instead use a system of relative value units that has not been the subject of a consent agreement, or develop your own system. A potential source of information in this regard is the relative value scale that has been included with the CMS HCPCS. This relative value scale is included with HCPCS as an informational item only. You may use it, revise it, or use some other relative value scale if you deem it more appropriate.

You may, if necessary develop a relative value scale for gap-filling purposes by dividing the unadjusted prevailing charges in a locality by a common denominator and filling the gaps in the resulting “relative value scale” by relying on medical staff judgment or on other persons with knowledge of the charging patterns and practices in your service area. Alternatively, you may use a service area wide approach and/or the unweighted average of the customary charges that have been made for a service, divided by a common denominator.

For radiology codes (codes beginning with 7, local radiology codes, and R-codes in HCPCS), you may use the national and local relative values used to make payment under the radiology fee schedule.

Customary and/or prevailing charge conversion factors used with relative value scales to fill gaps in reasonable charge screens should be calculated as outlined in A and B below. (Develop separate customary charge conversion factors for each physician or supplier from his known customary charges in the same category of services, e.g., medicine, surgery, radiology, etc. Similarly, separate prevailing charge conversion factors, by locality (and by specialty or groups of specialties as applicable), should be calculated based on the known prevailing charges, by locality and specialty, or groups of specialties within the same category of service. Customary charge conversion factors may only be calculated for a physician for a category of service if the physician has at least seven customary charges for services in that category of service upon which to base the conversion factor calculation. If a physician does not have sufficient customary charges to calculate a conversion factor in one category of service, this does not preclude the calculation of his customary charge conversion factors for other categories of service for which he does have sufficient customary charges).

#### **A. Customary Charge**

Use the following formula for the calculation of a customary charge conversion factor:

C/F = Customary charge conversion factor

CHG = The physician’s customary charge for a procedure

SVC = Number of times the physician performed the procedure

l-n = The different procedures the physician performed within a category of service

RVU = The relative value unit assigned to a procedure

SIGMA = Sum of

$$C/F = \frac{\frac{CHG_1}{RVU_1} \times SVC_1 + \frac{CHG_2}{RVU_2} \times SVC_2 + \dots + \frac{CHG_n}{RVU_n} \times SVC_n}{\text{SIGMA}}$$

### SIGMA SVC<sub>1-n</sub>

**EXAMPLE:** Compute a customary charge conversion factor for a physician with the following charge history: (May be for medicine, surgery, radiology, pathology.)

| Procedure | Frequency | Customary Charge | Relative Value |
|-----------|-----------|------------------|----------------|
| 1         | \$ 3      | 5.00             | 1              |
| 2         | 7         | 12.00            | 2              |
| 3         | 5         | 35.00            | 4              |
| 4         | 4         | 20.00            | 3              |
| 5         | <u>6</u>  | 8.00             | 1.5            |
|           | 25        |                  |                |

#### Method

1. For each procedure, divide the customary charge by the relative value and multiply the result by the frequency of that procedure in the physician's charge history.
2. Add all the results of these computations.
3. Divide the result by the sum of all the frequencies.

#### Solution

$$\begin{array}{ccccccc} \frac{(5 \times 3)}{1} & + & \frac{(12 \times 7)}{2} & + & \frac{(35 \times 5)}{4} & + & \frac{(20 \times 4)}{3} & + & \frac{(8 \times 6)}{1.5} & \text{divided by } 25 = \\ (5 \times 3) & + & (6 \times 7) & + & (8.75 \times 5) & + & (6.67 \times 4) & + & (5.33 \times 6) & = 25 \\ 15 & + & 42 & + & 43.75 & + & 26.68 & + & 31.98 & = 25 \\ 159.41 & = & \$6.40 & (\text{i.e., } \$6.38 \text{ rounded to the 25 nearest 10 cents}) \end{array}$$

To determine a physician's customary charge for a particular procedure where there is no reliable statistical basis, multiply the relative value of the procedure by the physician's customary charge conversion factor for the appropriate category of service (e.g., radiology, medicine, surgery).

### B. Prevailing Charges

The prevailing charge conversion factors used with the appropriate relative value scale are developed from the same formula used for customary charge conversion factors, except that:

CHG = The fully adjusted locality prevailing charge for a procedure by locality and by specialty or group of specialties (regardless of the source of data from which the locality prevailing charge was developed).

SVC = The number of times the procedure was performed by all physicians in the same specialty or group of specialties and locality.

l-n = The different procedures within a category of service for which prevailing charges have been established by specialty or group of specialties and locality.

The conversion factors calculated for any fee screen year reflect customary and prevailing charges calculated on the basis of charge data for the year ending June 30 immediately preceding the start of the fee screen year. Also, reasonable charge screens established through the use of a relative value scale and conversion factors consist of two components. Consequently, the conversion factors must be recalculated when there is any change in the relative value units assigned to procedures (as may occur if you use a different or updated relative value scale) in order to assure that the change(s) in unit values do not violate the integrity of the reasonable charge screens. The economic index limitation, the no rollback provision, and the Administrative Savings Clause are not applied directly to prevailing charge conversion factors calculated in accordance with this section.

## **80.6 - Inflation Indexed Charge (IIC) for Nonphysician Services** (Rev. 2837, Issued: 12-13-13, Effective: 01-01-14 - payment reasonable charge basis/04-01-14 - payment national fee schedule basis, Implementation: 01-06-14 - payment reasonable charge basis/04-07-14 - payment national fee schedule basis)

### **A General**

Effective for services rendered on or after October 1, 1985, an additional factor - the inflation indexed charge (IIC), is added to the factors taken into consideration in determining reasonable charges for non-physician services. Non-physician services are defined as those Part B medical services, supplies, and equipment reimbursed on a reasonable charge basis and not subject to the application of the Medicare Economic Index (MEI).

Examples of items affected by the IIC are:

- Prosthetic and orthotic devices not subject to the fee schedules [Therapeutic Shoes (2005 and prior), Intraocular Lenses (2014 and prior)];
- Blood products and transfusion medicine;
- Certain medical supplies used in connection with home dialysis (2011 and prior); and
- Ambulance services (2001 and prior)

## **80.7 - Determination of Comparable Circumstances** (Rev. 1, 10-01-03) **B3-5026**

A/B MACs (B) do not make a reasonable charge determination that would be higher than the charge upon which they would base payment to their own policyholders for a

comparable service under comparable circumstances. The charge upon which payment is based does not mean the amount the A/B MAC (B) would be obligated to pay. Under certain circumstances, some A/B MACs (B) pay amounts on behalf of their policyholders, which are below the customary and prevailing charges physicians or other persons usually make to the general public. Payments under the medical insurance program are not limited to these lower amounts.

“Comparable circumstances” refers to the circumstances under which services are rendered to individuals and the nature of your health insurance programs, and the method used to determine the amounts of payment under these programs. Generally, comparability exists where:

- Payment is made under the contractor’s own program on the customary charges of physicians or other persons, and on current prevailing charges in a locality, and
- The determination does not preclude recognition of factors such as specialty status and unusual circumstances that affect the amount charged for a service.

However, even where there is comparability, coverage limitations applicable under the contractor’s own programs do not necessarily apply to reasonable charge determinations for Medicare purposes.

The “current” customary and/or prevailing charges of an A/B MAC (B)’s private health plan refer to the payment screens that are presently in effect, e.g., payment levels actually being used in the A/B MAC (B)’s private business for settling claims submitted by its policy holders or subscribers. A/B MACs (B) must, therefore, continue to apply the comparability limitation based upon their payment screens that are presently in effect, even where an update under their private insurance plans has been deferred. If an A/B MAC (B)’s private health plan allowances are later revised, it will be necessary for the A/B MAC (B) to reexamine the relationship of these new payment levels to those under the Medicare program and initiate the necessary changes through their routine maintenance operations.

Responsibility for determining whether an A/B MAC (B)’s program has comparability will fall upon the A/B MAC (B) in reporting pertinent information about its programs to CMS. When the pertinent information has been reported, CMS will advise the A/B MAC (B) whether any of its programs have comparability.

## **80.8 - Applying Criteria for Reasonable Charge Determinations**

**(Rev. 1, 10-01-03)**

**B3-5030, B3-5030.2**

The reasonable charge determination for a covered service should be based on the actual charge for the service when that charge is no higher than the applicable customary and prevailing charges. The A/B MAC (B) should exercise judgment so that its reasonable charge determinations are realistic and equitable. A/B MACs (B) should take into account special factors in individual cases that might affect the reasonableness of charges. The

special factors could involve travel, medical complications, or other unusual circumstances such as prolonged time and attention required by a patient's condition.

Different services and supplies for which charges are made under Part B, may not be grouped together for the purpose of making one overall reasonable charge determination. Thus, in processing a claim for payment a separate reasonable charge determination must be made with respect to each item for which a charge is made.

### **80.8.1 - Waiver of Deductible and Coinsurance**

**(Rev. 1, 10-01-03)**

**B3-5220**

Physicians or suppliers who routinely waive the collection of deductible or coinsurance from a beneficiary constitute a violation of the law pertaining to false claims and kickbacks. These situations should be referred to Program Integrity area for additional investigation according to the procedures in the Medicare Program Integrity Manual.

Deductible and coinsurance amounts are taken into account (included) in determining the reasonable charge for a service or item. In this regard, a billed amount that is not reasonably related to an expectation of payment is not considered the "actual" charge for the purpose of processing a claim or for the purpose of determining customary charges.

Where a physician/supplier makes a reasonable collection effort for the payment of coinsurance/deductibles, failure to collect payment is not considered a reduction in the physician's/supplier's charge. To be considered a reasonable collection effort, the effort to collect Medicare coinsurance/deductible amounts must be similar to the effort made to collect comparable amounts from non-Medicare patients. It must also involve the issuance of a bill to the beneficiary or to the party responsible for the patient's personal financial obligations. In addition, it may include other actions, such as subsequent billings, collection letters and telephone calls or personal contacts which constitute a genuine, rather than token, collection effort.

### **90 - Inherent Reasonableness Used for Payment of Nonphysician Services**

**(Rev. 1, 10-01-03)**

**B3-5246, AB-98-9 (Obsolete instructions)**

Placeholder for instruction when current regulation is finalized.

### **100 - Competitive Bidding Durable Medical Equipment Prosthetics, Orthotics and Supplies (DMEPOS) Single Payment Amounts**

**(Rev. 2682, Issued: 04-05-13, Effective: 07-01-13, Implementation: 07-01-13)**

Section 1847 (b)(5) of the Social Security Act mandates that a single payment amount be established for each item in each competitive bid area based on the bids submitted and accepted for that item. Section 1847(a)(6) of the Act requires that this payment basis be substituted for the fee schedule payment basis otherwise applied under section 1834(a) of

the Act for DME, section 1834(h) of the Act for off-the-shelf orthotics, or section 1842(s) of the Act for enteral nutrients, equipment and supplies, as appropriate.

For the Round One Rebid, the DME MACs will make payment based on the new competitive bidding single payment amounts effective for claims with dates of service on or after January 1, 2011. The CMS issues instructions for implementing the single payment amounts and/or updating the payment amounts quarterly for competitive bidding items. The following files will be provided to the DME MACs and the Pricing, Data Analysis and Coding (PDAC) Contractor via CMS' mainframe telecommunication system in order to implement a competitive bidding round: a Healthcare Common Procedure Coding System (HCPCS) category file, a bid pricing file, a ZIP Code file, and a contract supplier file. All four files will contain data only for the current competitive bidding round.

The DME MACs will make manual updates to the competitive bidding files, as instructed through Recurring Update Notifications. The following files will be provided on a quarterly basis to the DME MACs and the Pricing, Data Analysis and Coding (PDAC) Contractor via CMS' mainframe telecommunication system: HCPCS category file, bid pricing file, and ZIP Code file. These files will only contain the changes for that quarter.

Upon successful receipt of the mainframe files, DME MACs will send notification of receipt via e-mail to [price\\_file\\_receipt@cms.hhs.gov](mailto:price_file_receipt@cms.hhs.gov), stating the name of the files received and the entities for which they were received (e.g., DME MAC name).

Following program implementation and on an as needed basis, updates to the contract supplier file will be accomplished via an Excel file emailed directly from the Competitive Bidding Implementation Contractor (CBIC) to the DME MACs. Upon successful receipt of the files, the DME MACs will send notification of receipt via email stating the name of the file received and the entities for which they were received (e.g., DME MAC name) to the CBIC. E-mails should be sent to [cbic.dmemac@palmettoGBA.com](mailto:cbic.dmemac@palmettoGBA.com) at the CBIC. The DME MACs shall manually update the contract supplier file with the changes by adding and/or end dating each supplier record.

Public use files containing the competitive bidding HCPCS categories, single payment amounts and ZIP Codes are available on the CBIC Website at <http://www.dmecompetitivebid.com/palmetto/cbic.nsf>.

### **100.1 - Record Layout for Competitive Bidding HCPCS Category File (Rev. 2088, Issued: 11-05-10, Effective: 01-01-11, Implementation: 01-03-11)**

| <b>Field Name</b>                       | <b>Pic</b> | <b>Position</b> | <b>Comment</b> |
|-----------------------------------------|------------|-----------------|----------------|
| CBA Bid Category Effective Date         | X(8)       | 1 - 8           |                |
| Bid Product Category Effective Date- CC | 9(2)       | 1 - 2           |                |

| <b>Field Name</b>                           | <b>Pic</b> | <b>Position</b> | <b>Comment</b>                        |
|---------------------------------------------|------------|-----------------|---------------------------------------|
| Bid Product Category Effective Date-Year    | 9(2)       | 3 - 4           |                                       |
| Bid Product Category Effective Date -Month  | 9(2)       | 5 - 6           |                                       |
| Bid Product Category Effective-Day          | 9(2)       | 7 - 8           |                                       |
| Bid Category Expiration Date                | X(8)       | 9 - 16          |                                       |
| Bid Product Category Expiration Date -CC    | 9(2)       | 9 - 10          |                                       |
| Bid Product Category Expiration Date - Year | 9(2)       | 11 - 12         |                                       |
| Bid Product Category Expiration Date-Month  | 9(2)       | 13 - 14         |                                       |
| Bid Product Category Expiration Date - Day  | 9(2)       | 15 - 16         |                                       |
| HCPCS Procedure Codes                       | X(13)_     | 17 - 29         |                                       |
| HCPCS Procedure Code                        | X(5)       | 17 - 21         | All Competitive Bidding Active Codes. |
| Modifier 1                                  | X(2)       | 22 - 23         |                                       |
| Modifier 2                                  | X(2)       | 24 - 25         |                                       |
| Modifier 3                                  | X(2)       | 26 - 27         |                                       |
| Modifier 4                                  | X(2)       | 28 - 29         |                                       |

| Field Name                  | Pic  | Position | Comment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
|-----------------------------|------|----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CBA Area                    | X(5) | 30 - 34  | 16740 = Charlotte NC-SC<br>16741 = Charlotte NC-SC Mail Order<br>17140 = Cincinnati OH<br>17141 = Cincinnati OH Mail Order<br>17460 = Cleveland OH<br>17461 = Cleveland OH Mail Order<br>19100 = Dallas TX<br>19101 = Dallas TX Mail Order<br>28140 = Kansas MO-KS<br>28141 = Kansas MO- KS Mail Order<br>33100 = Miami FL<br>33101 = Miami FL Mail Order<br>36740 = Orlando FL<br>36741 = Orlando FL Mail Order<br>38300 = Pittsburgh PA<br>38301 = Pittsburgh PA Mail Order<br>40140 = Riverside CA<br>40141 = Riverside CA Mail Order |
| Bid Product Category Number | X(3) | 35 - 37  | 001 = Oxygen Supplies & Equipment<br>002 = Standard Power Wheelchairs<br>003 = Complex Rehab Power Wheelchairs<br>004 = Mail Order Diabetic Supplies<br>005 = Enteral Nutrients, Equipment<br>006 = CPAPs, RADs and Related Supplies<br>007 = Hospital Beds & Related Supplies<br>009 = Walkers & Related Supplies<br>010 = Support Surfaces                                                                                                                                                                                             |

| <b>Field Name</b>   | <b>Pic</b> | <b>Position</b> | <b>Comment</b>                                                                                                                          |
|---------------------|------------|-----------------|-----------------------------------------------------------------------------------------------------------------------------------------|
| Bid Business Rule 1 | X(1)       | 38 - 38         | When field contains “S”, permits non-contract physicians to bill selected CB items or “H”, permits Hospitals to bill selected CB items  |
| Bid Business Rule 2 | X(1)       | 39 - 39         | When field contains “S”, permits non-contract physicians to bill selected CB items or “H”, permits Hospitals to bill selected CB items. |
| Bid Business Rule 3 | X(1)       | 40 - 40         | For future use                                                                                                                          |
| Bid Business Rule 4 | X(1)       | 41 - 41         | For future use                                                                                                                          |
| Bid Business Rule 5 | X(1)       | 42 - 42         | For future use                                                                                                                          |
| Filler              | X(8)       | 43 - 50         |                                                                                                                                         |

**100.2 - Record Layout for Competitive Bidding Pricing File**  
**(Rev. 2088, Issued: 11-05-10, Effective: 01-01-11, Implementation: 01-03-11)**

| <b>Field Name</b>                | <b>Pic</b> | <b>Position</b> | <b>Comment</b> |
|----------------------------------|------------|-----------------|----------------|
| CBA Bid Round Effective Date     | X(8)       | 1-8             |                |
| Bid Round Effective Date - CC    | 9(2)       | 1 -2            |                |
| Bid Round Effective Date -Year   | 9(2)       | 3-4             |                |
| Bid Round Effective Date - Month | 9(2)       | 5 - 6           |                |
| Bid Round Effective Date - Day   | 9(2)       | 7 - 8           |                |

| <b>Field Name</b>                   | <b>Pic</b> | <b>Position</b> | <b>Comment</b>                                                                                                                                                                                                                                                  |
|-------------------------------------|------------|-----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Bid Round Expiration                | X(8)       | 9-16            |                                                                                                                                                                                                                                                                 |
| Bid Round Expiration<br>Date -CC    | 9(2)       | 9 - 10          |                                                                                                                                                                                                                                                                 |
| Bid Round Expiration<br>Date - Year | 9(2)       | 11 - 12         |                                                                                                                                                                                                                                                                 |
| Bid Round Expiration<br>Date- Month | 9(2)       | 13 - 14         |                                                                                                                                                                                                                                                                 |
| Bid Round Expiration<br>Date - Day  | 9(2)       | 15 - 16         |                                                                                                                                                                                                                                                                 |
| HCPCS Procedure<br>Code             | X(5)       | 17 - 21         |                                                                                                                                                                                                                                                                 |
| Modifier 1                          | X(2)       | 22 - 23         |                                                                                                                                                                                                                                                                 |
| Modifier 2                          | X(2)       | 24 - 25         |                                                                                                                                                                                                                                                                 |
| Modifier 3                          | X(2)       | 26 - 27         |                                                                                                                                                                                                                                                                 |
| Modifier 4                          | X(2)       | 28 - 29         |                                                                                                                                                                                                                                                                 |
| DMEPOS Category                     | X(2)       | 30 - 31         | IN = Inexpensive/Routinely Purchased<br>FS = Frequently Serviced<br>CR = Capped Rental<br>OX = Oxygen & Oxygen Equipment<br>OS = Ostomy, Tracheostomy &<br>Urologicals<br>SD = Surgical Dressings<br>PO = Prosthetics & Orthotics<br>SU = Supplies<br>TE = TENS |

| Field Name                       | Pic      | Position | Comment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
|----------------------------------|----------|----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CBA Area                         | X(5)     | 32 - 36  | 16740 = Charlotte NC-SC<br>16741 = Charlotte NC-SC Mail Order<br>17140 = Cincinnati OH<br>17141 = Cincinnati OH Mail Order<br>17460 = Cleveland OH<br>17461 = Cleveland OH Mail Order<br>19100 = Dallas TX<br>19101 = Dallas TX Mail Order<br>28140 = Kansas MO-KS<br>28141 = Kansas MO- KS Mail Order<br>33100 = Miami FL<br>33101 = Miami FL Mail Order<br>36740 = Orlando FL<br>36741 = Orlando FL Mail Order<br>38300 = Pittsburgh PA<br>38301 = Pittsburgh PA Mail Order<br>40140 = Riverside CA<br>40141 = Riverside CA Mail Order |
| Bid Price                        | 9(6)V99. | 37 - 44  | Competitive Bidding Single Payment Amount for HCPCS code                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| Bid Price Effective Date         | X(8)     | 45 - 52  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Price Effective Date - CC    | 9(2)     | 45 - 46  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Price Effective Date -Year   | 9(2)     | 47 - 48  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Price Effective Date - Month | 9(2)     | 49 - 50  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Price Effective Date - Day   | 9(2)     | 51 - 52  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |

| Field Name                       | Pic   | Position | Comment                                                                                                                                                                                                                                                                                                                                      |
|----------------------------------|-------|----------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Bid Price Expiration Date        | X(8)  | 53 - 60  |                                                                                                                                                                                                                                                                                                                                              |
| Bid Price Expiration Date -CC    | 9(2)  | 53 - 54  |                                                                                                                                                                                                                                                                                                                                              |
| Bid Price Expiration Date - Year | 9(2)  | 55 - 56  |                                                                                                                                                                                                                                                                                                                                              |
| Bid Price Expiration Date- Month | 9(2)  | 57 - 58  |                                                                                                                                                                                                                                                                                                                                              |
| Bid Price Expiration Date - Day  | 9(2)  | 59 - 60  |                                                                                                                                                                                                                                                                                                                                              |
| Price Change Indicator           | X(1)  | 61 - 61  |                                                                                                                                                                                                                                                                                                                                              |
| Bid Product Category             | X(3)  | 62 - 64  | 001 = Oxygen Supplies & Equipment<br>002 = Standard Power Wheelchairs<br>003 = Complex Rehab Power Wheelchairs<br>004 = Mail Order Diabetic Supplies<br>005 = Enteral Nutrients, Equipment<br>006 = CPAPs, RADs and Related Supplies<br>007 = Hospital Beds & Related Supplies<br>009 = Walkers & Related Supplies<br>010 = Support Surfaces |
| Filler                           | X(12) | 65 - 76  |                                                                                                                                                                                                                                                                                                                                              |

**100.3 - Record Layout for Competitive Bidding ZIP Code File**  
(Rev. 2088, Issued: 11-05-10, Effective: 01-01-11, Implementation: 01-03-11)

| Field Name                  | Pic  | Position | Comment |
|-----------------------------|------|----------|---------|
| CBA ZIP Code Effective Date | X(8) | 1- 8     |         |

| <b>Field Name</b>                         | <b>Pic</b> | <b>Position</b> | <b>Comment</b>       |
|-------------------------------------------|------------|-----------------|----------------------|
| Bid ZIP Code<br>Effective Date- CC        | 9(2)       | 1 - 2           |                      |
| Bid ZIP Code<br>Effective Date-Year       | 9(2)       | 3-4             |                      |
| Bid ZIP Code<br>Effective Date -Month     | 9(2)       | 5 - 6           |                      |
| Bid ZIP Code<br>Effective Date - Day      | 9(2)       | 7- 8            |                      |
| Bid ZIP Code<br>Expiration Date           | X(8)       | 9-16            |                      |
| Bid ZIP Code<br>Expiration Date -CC       | 9(2)       | 9 - 10          |                      |
| Bid ZIP Code<br>Expiration Date -<br>Year | 9(2)       | 11 - 12         |                      |
| Bid ZIP Code<br>Expiration Date-<br>Month | 9(2)       | 13 - 14         |                      |
| Bid ZIP Code<br>Expiration Date - Day     | 9(2)       | 15 - 16         |                      |
| CBA ZIP Code                              | X(5)       | 17 - 21         | ZIP Code for the CBA |
| CBA State                                 | X(2)       | 22 - 23         |                      |

| Field Name | Pic   | Position | Comment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
|------------|-------|----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CBA Area   | X(5)  | 24- 28   | 16740 = Charlotte NC-SC<br>16741 = Charlotte NC-SC Mail Order<br>17140 = Cincinnati OH<br>17141 = Cincinnati OH Mail Order<br>17460 = Cleveland OH<br>17461 = Cleveland OH Mail Order<br>19100 = Dallas TX<br>19101 = Dallas TX Mail Order<br>28140 = Kansas MO-KS<br>28141 = Kansas MO- KS Mail Order<br>33100 = Miami FL<br>33101 = Miami FL Mail Order<br>36740 = Orlando FL<br>36741 = Orlando FL Mail Order<br>38300 = Pittsburgh PA<br>38301 = Pittsburgh PA Mail Order<br>40140 = Riverside CA<br>40141 = Riverside CA Mail Order |
| Filler     | X(12) | 29 - 40  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |

**100.4 - Record Layout for Competitive Bidding Contract Supplier File**  
(Rev. 2088, Issued: 11-05-10, Effective: 01-01-11, Implementation: 01-03-11)

| Field Name          | Pic   | Position | Comment |
|---------------------|-------|----------|---------|
| Supplier NSC Number | X(10) | 1 - 10   |         |
| Supplier NPI Number | X(10) | 11 - 20  |         |

| Field Name                    | Pic  | Position | Comment                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
|-------------------------------|------|----------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CBA Area                      | X(5) | 21 - 25  | 16740 = Charlotte NC-SC<br>16741 = Charlotte NC-SC Mail Order<br>17140 = Cincinnati OH<br>17141 = Cincinnati OH Mail Order<br>17460 = Cleveland OH<br>17461 = Cleveland OH Mail Order<br>19100 = Dallas TX<br>19101 = Dallas TX Mail Order<br>28140 = Kansas MO-KS<br>28141 = Kansas MO- KS Mail Order<br>33100 = Miami FL<br>33101 = Miami FL Mail Order<br>36740 = Orlando FL<br>36741 = Orlando FL Mail Order<br>38300 = Pittsburgh PA<br>38301 = Pittsburgh PA Mail Order<br>40140 = Riverside CA<br>40141 = Riverside CA Mail Order |
| Bid Effective Date            | X(8) | 26 - 33  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Effective Date-<br>CC     | 9(2) | 26 - 27  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Effective Date-<br>Year   | 9(2) | 28 - 29  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Effective Date -<br>Month | 9(2) | 30 - 31  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Effective Date-<br>Day    | 9(2) | 32 - 33  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Expiration Date           | X(8) | 34 - 41  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| Bid Expiration Date -<br>CC   | 9(2) | 34 - 35  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |

| Field Name                    | Pic    | Position  | Comment                                                                                                                          |
|-------------------------------|--------|-----------|----------------------------------------------------------------------------------------------------------------------------------|
| Bid Expiration Date - Year    | 9(2)   | 36 - 37   |                                                                                                                                  |
| Bid Expiration Date- Month    | 9(2)   | 38 - 39   |                                                                                                                                  |
| Bid Expiration Date - Day     | 9(2)   | 40 - 41   |                                                                                                                                  |
| Bid Switches Occurs 500 Times | X(500) | 42-541    | The switch occurrence equates to the 500 Times Bid category; i.e., Bid Switch (1) denotes the supplier contract for category 001 |
| Bid Switch (1)                | X(1)   | 42 - 42   |                                                                                                                                  |
| Bid Switch (2)                | X(1)   | 43 - 43   |                                                                                                                                  |
| Filler                        | X(59)  | 542 - 600 |                                                                                                                                  |

### **100.5 - Adjustments to the Single Payment Amounts to Reflect Changes in HCPCS Codes**

**(Rev. 1535, Issued: 06-13-08, Effective: 07-01-08, Implementation: 07-07-08)**

If new HCPCS codes are established following the start of a competitive bidding contract period that describe new technology items that did not previously fall under HCPCS codes for competitive bidding items, they will not be added to the competitive bidding HCPCS, CBA pricing and public use files.

If a HCPCS code descriptor for a competitive bidding item is revised to clarify the item described by that code after the competitive bidding contract period begins, a corresponding code descriptor change will be made in the competitive bidding HCPCS, CBA pricing and public use files.

Should a HCPCS code be deleted from the HCPCS after the competitive bidding contract period begins, the deleted code will be considered invalid for claims submission under the competitive bidding program and will be removed from the competitive bidding HCPCS, CBA pricing and public use files. Although the code is discontinued, as explained below, the items that previously fell under the code may continue to be subject to the competitive bidding program.

If a HCPCS code for a competitive bidding item is revised after the contract period for a competitive bidding program begins, CMS will adjust the single payment amount(s) for the item(s) on the CBA pricing file for that Round using one of the following methods:

- a. If a single HCPCS code for an item is divided into two or more HCPCS codes for the components of that item, the sum of the single payment amounts for the new HCPCS codes will equal the single payment amount for the original item. In accordance with instructions provided in future recurring update notifications, the payment amounts for the HCPCS codes for the components will be established based on the corresponding fee schedule amounts for these codes that are established in accordance with section 60.3 of chapter 23 of the Claims Processing Manual. These amounts for the components of the item will then be adjusted by the same percentage to the level where the sum of the payment amounts for the HCPCS codes for the components equals the single payment amount for the item.
- b. If a single HCPCS code is divided into two or more separate HCPCS codes for different but similar items, the single payment amount for each of the new separate HCPCS codes is equal to the single payment amount applied to the original, single HCPCS code.
- c. If the HCPCS codes for components of an item are merged into a single HCPCS code for the item, the single payment amount for the new HCPCS code is equal to the total of the separate single payment amounts for the components.
- d. If multiple HCPCS codes for different but similar items are merged into a single HCPCS code, the items to which the new HCPCS codes apply may be furnished by any supplier that has a valid Medicare billing number. Payment for the new code will be based on the fee schedule methodology, even if single payment amounts were established for the discontinued multiple HCPCS codes. The old codes will be considered invalid and no longer included in the competitive bidding program for the remainder of the contract term.

Contract suppliers must furnish the item(s) described by the new HCPCS code(s) in scenarios (a) through (c) above and submit claims using the new codes. Notification of a competitive bidding HCPCS code change will occur through program instruction.

### **Addendum - MPFSDB File Record Layout and Field Descriptions (Rev. 12823, Issued:09-05- 24, Effective: 10-08-24, Implementation Date:10-08-24)**

The CMS MPFSDB includes the total fee schedule amount, related component parts, and payment policy indicators. The record layout is provided below. Beginning with the 2019 MPFSDB, and thereafter, the MPFSDB File Record Layout will no longer be revised annually in this section for the sole purpose of changing the calendar year, but will only be revised when there is a change to a field. Previous MPFSDB file layouts (for 2018 and prior) can be found on the CMS web site on the Physician Fee Schedule web page at: <https://www.cms.gov/Medicare/Medicare-Fee-for-Service-Payment/PhysicianFeeSched/index.html>.

## MPFSDB File Layout

### HEADER RECORD

| FIELD # | DATA ELEMENT NAME                                                | LOCATION | PIC               |
|---------|------------------------------------------------------------------|----------|-------------------|
| 1       | Header ID                                                        | 1-4      | x(4) Value "Head" |
| 2       | Header Number                                                    | 5        | x(1)              |
| 3       | Data Set Name                                                    | 6-50     | x(45)             |
| 4       | Record Length                                                    | 51-53    | x(3)              |
| 5       | Filler                                                           | 54-54    | x(1)              |
| 6       | Block size                                                       | 55-58    | x(4)              |
| 7       | Filler                                                           | 59-59    | x(1)              |
| 8       | Number of Records<br>Number does not include this header record. | 60-69    | 9(10)             |
| 9       | Date Created                                                     | 70-77    | x(8) YYYYMMDD     |
| 10      | Blanks                                                           | 78-345   | x(268)            |

### DATA RECORD

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                        | LENGTH & PIC |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|
| 1<br>File Year<br>This field displays the effective year of the file.                                                                                                                                                                                                                                                                                                                                                                                                 | 4 Pic x(4)   |
| 2<br>A/B MAC (B) Number<br>This field represents the 5-digit number assigned to the A/B MAC (B).                                                                                                                                                                                                                                                                                                                                                                      | 5 Pic x(5)   |
| 3<br>Locality<br>This 2-digit code identifies the pricing locality used.                                                                                                                                                                                                                                                                                                                                                                                              | 2 Pic x(2)   |
| 4<br>HCPCS Code<br>This field represents the procedure code. Each A/B MAC (B) Current Procedural Terminology (CPT) code (other than codes for Multianalyte Assays with Algorithmic Analyses (MAAA) and Proprietary Laboratory Analyses (PLA)) and alpha-numeric HCPCS codes other than B, C, E, K, L and U codes will be included. The standard sort for this field is blanks, alpha, and numeric in ascending order. Note: MAAA and PLA are alpha-numeric CPT codes. | 5 Pic x(5)   |
| 5<br>Modifier                                                                                                                                                                                                                                                                                                                                                                                                                                                         | 2 Pic x(2)   |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | LENGTH & PIC    |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------|
| <p>For diagnostic tests, a blank in this field denotes the global service and the following modifiers identify the components:</p> <p>26 = Professional component</p> <p>TC = Technical component</p> <p>For services other than those with a professional and/or technical component, a blank will appear in this field with one exception: the presence of CPT modifier -53 which indicates that separate Relative Value Units (RVUs) and a fee schedule amount have been established for procedures which the physician terminated before completion. This modifier is used only with colonoscopy through stoma code 44388, colonoscopy code 45378 and screening colonoscopy codes G0105 and G0121. Any other codes billed with modifier -53 are subject to medical review and priced by individual consideration.</p> <p>Modifier-53 = Discontinued Procedure - Under certain circumstances, the physician may elect to terminate a surgical or diagnostic procedure. Due to extenuating circumstances, or those that threaten the well being of the patient, it may be necessary to indicate that a surgical or diagnostic procedure was started but discontinued.</p> |                 |
| <p>6</p> <p>Descriptor</p> <p>This field will include a brief description of each procedure code.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | 50 Pic x(50)    |
| <p>7</p> <p>Code Status</p> <p>This 1 position field provides the status of each code under the full fee schedule. Each status code is explained in §30.2.2.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | 1 Pic x(1)      |
| <p>8</p> <p>Conversion Factor</p> <p>This field displays the multiplier which transforms relative values into payment amounts. The file will contain the conversion factor for the File Year which will reflect all adjustments.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | 8 Pic 9(4)v9999 |
| <p>9</p> <p>Update Factor</p> <p>This update factor has been included in the conversion factor in Field 8.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | 6 Pic 9(2)v9999 |
| <p>10</p> <p>Work Relative Value Unit</p> <p>This field displays the unit value for the physician work RVU.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | 9 Pic 9(7)v99   |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | LENGTH & PIC  |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------|
| 11<br>Filler                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | 9 Pic 9(7)v99 |
| 12<br>Malpractice Relative Value Unit<br>This field displays the unit value for the malpractice expense RVU.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | 9 Pic 9(7)v99 |
| 13<br>Work Geographic Practice Cost Indices (GPCIs)<br>This field displays a work geographic adjustment factor used in computing the fee schedule amount.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | 5 Pic 99v999  |
| 14<br>Practice Expense GPCI<br>This field displays a practice expense geographic adjustment factor used in computing the fee schedule amount.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | 5 Pic 99v999  |
| 15<br>Malpractice GPCI<br>This field displays a malpractice expense geographic adjustment factor used in computing the fee schedule amount.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | 5 Pic 99v999  |
| 16<br>Global Surgery<br>This field provides the postoperative time frames that apply to payment for each surgical procedure or another indicator that describes the applicability of the global concept to the service.<br>000 = Endoscopic or minor procedure with related preoperative and postoperative relative values on the day of the procedure only included in the fee schedule payment amount; evaluation and management services on the day of the procedure generally not payable.<br>010 = Minor procedure with preoperative relative values on the day of the procedure and postoperative relative values during a 10-day postoperative period included in the fee schedule amount; evaluation and management services on the day of the procedure and during this 10-day postoperative period generally not payable.<br>090 = Major surgery with a 1-day preoperative period and 90-day postoperative period included in the fee schedule payment amount.<br>MMM = Maternity codes; usual global period does not apply.<br>XXX = Global concept does not apply.<br>YYY = A/B MAC (B) determines whether global concept applies and establishes postoperative period, if appropriate, at time of pricing. | 3 Pic x(3)    |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | LENGTH & PIC |
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| <p>ZZZ = Code related to another service and is always included in the global period of the other service. (Note: Physician work is associated with intra-service time and in some instances the post service time.)</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |              |
| <p>17<br/>Preoperative Percentage (Modifier 56)<br/>This field contains the percentage (shown in decimal format) for the preoperative portion of the global package. For example, 10 percent will be shown as 010000. The total of fields 17, 18, and 19 will usually equal one. Any variance is slight and results from rounding.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | 6 Pic 9v9(5) |
| <p>18<br/>Intraoperative Percentage (Modifier 54)<br/>This field contains the percentage (shown in decimal format) for the intraoperative portion of the global package including postoperative work in the hospital. For example, 63 percent will be shown as 063000. The total of fields 17, 18, and 19 will usually equal one. Any variance is slight and results from rounding.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 6 Pic 9v9(5) |
| <p>19<br/>Postoperative Percentage (Modifier 55)<br/>This field contains the percentage (shown in decimal format) for the postoperative portion of the global package that is provided in the office after discharge from the hospital. For example, 17 percent will be shown as 017000. The total of fields 17, 18, and 19 will usually equal one. Any variance is slight and results from rounding.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | 6 Pic 9v9(5) |
| <p>20<br/>Professional Component (PC)/Technical Component (TC) Indicator<br/>0 = Physician service codes: This indicator identifies codes that describe physician services. Examples include visits, consultations, and surgical procedures. The concept of PC/TC does not apply since physician services cannot be split into professional and technical components. Modifiers 26 &amp; TC cannot be used with these codes. The total Relative Value Units (RVUs) include values for physician work, practice expense and malpractice expense. There are some codes with no work RVUs.<br/>1 = Diagnostic tests or radiology services: This indicator identifies codes that describe diagnostic tests, e.g., pulmonary function tests, or therapeutic radiology procedures, e.g., radiation therapy. These codes generally have both a professional and technical component. Modifiers 26 and TC can be used with these codes.<br/>The total RVUs for codes reported with a 26 modifier include values for physician work, practice expense, and malpractice expense.</p> | 1 Pic x(1)   |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | LENGTH & PIC |
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| <p>The total RVUs for codes reported with a TC modifier include values for practice expense and malpractice expense only. The total RVUs for codes reported without a modifier equals the sum of RVUs for both the professional and technical component.</p> <p>2 = Professional component only codes: This indicator identifies stand alone codes that describe the physician work portion of selected diagnostic tests for which there is an associated code that describes the technical component of the diagnostic test only and another associated code that describes the global test.</p> <p>An example of a professional component only code is 93010, Electrocardiogram; interpretation and report. Modifiers 26 and TC cannot be used with these codes. The total RVUs for professional component only codes include values for physician work, practice expense, and malpractice expense.</p> <p>3 = Technical component only codes: This indicator identifies stand alone codes that describe the technical component (i.e., staff and equipment costs) of selected diagnostic tests for which there is an associated code that describes the professional component of the diagnostic tests only.</p> <p>An example of a technical component code is 93005, Electrocardiogram, tracing only, without interpretation and report. It also identifies codes that are covered only as diagnostic tests and therefore do not have a related professional code. Modifiers 26 and TC cannot be used with these codes.</p> <p>The total RVUs for technical component only codes include values for practice expense and malpractice expense only.</p> <p>4 = Global test only codes: This indicator identifies stand alone codes for which there are associated codes that describe: a) the professional component of the test only and b) the technical component of the test only. Modifiers 26 and TC cannot be used with these codes. The total RVUs for global procedure only codes include values for physician work, practice expense, and malpractice expense. The total RVUs for global procedure only codes equals the sum of the total RVUs for the professional and technical components only codes combined.</p> <p>5 = Incident to codes: This indicator identifies codes that describe services covered incident to a physicians service when they are provided by auxiliary personnel employed by the physician and working under his or her direct supervision.</p> <p>Payment may not be made by A/B MACs (B) for these services when they are provided to hospital inpatients or patients in a hospital outpatient department. Modifiers 26 and TC cannot be used with these codes.</p> |              |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | LENGTH & PIC      |
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| <p>6 = Laboratory physician interpretation codes: This indicator identifies clinical laboratory codes for which separate payment for interpretations by laboratory physicians may be made. Actual performance of the tests is paid for under the lab fee schedule. Modifier TC cannot be used with these codes. The total RVUs for laboratory physician interpretation codes include values for physician work, practice expense and malpractice expense.</p> <p>7 = Private practice therapist's service: Payment may not be made if the service is provided to either a hospital outpatient or a hospital inpatient by a physical therapist, occupational therapist, or speech-language pathologist in private practice.</p> <p>8 = Physician interpretation codes: This indicator identifies the professional component of clinical laboratory codes for which separate payment may be made only if the physician interprets an abnormal smear for hospital inpatient. This applies only to code 85060. No TC billing is recognized because payment for the underlying clinical laboratory test is made to the hospital, generally through the PPS rate.</p> <p>No payment is recognized for code 85060 furnished to hospital outpatients or non-hospital patients. The physician interpretation is paid through the clinical laboratory fee schedule payment for the clinical laboratory test.</p> <p>9 = Concept of a professional/technical component does not apply.</p> |                   |
| <p>21</p> <p>Multiple Procedure (Modifier 51)</p> <p>Indicator indicates which payment adjustment rule for multiple procedures applies to the service.</p> <p>0 = No payment adjustment rules for multiple procedures apply. If procedure is reported on the same day as another procedure, base payment on the lower of: (a) the actual charge or (b) the fee schedule amount for the procedure.</p> <p>1 = Standard payment adjustment rules in effect before January 1, 1996, for multiple procedures apply. In the 1996 MPFSDB, this indicator only applies to codes with procedure status of "D." If a procedure is reported on the same day as another procedure with an indicator of 1,2, or 3, rank the procedures by fee schedule amount and apply the appropriate reduction to this code (100 percent, 50 percent, 25 percent, 25 percent, 25 percent, and by report). Base payment on the lower of: (a) the actual charge or (b) the fee schedule amount reduced by the appropriate percentage.</p> <p>2 = Standard payment adjustment rules for multiple procedures apply. If procedure is reported on the same day as another procedure with an indicator of 1, 2, or 3, rank the procedures by fee</p>                                                                                                                                                                                                                                                            | <p>1 Pic (x)1</p> |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | LENGTH & PIC      |
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| <p>schedule amount and apply the appropriate reduction to this code (100 percent, 50 percent, 50 percent, 50 percent, 50 percent, and by report). Base payment on the lower of: (a) the actual charge or (b) the fee schedule amount reduced by the appropriate percentage.</p> <p>3 = Special rules for multiple endoscopic procedures apply if procedure is billed with another endoscopy in the same family (i.e., another endoscopy that has the same base procedure). The base procedure for each code with this indicator is identified in field 31G. Apply the multiple endoscopy rules to a family before ranking the family with other procedures performed on the same day (for example, if multiple endoscopies in the same family are reported on the same day as endoscopies in another family or on the same day as a non-endoscopic procedure).</p> <p>If an endoscopic procedure is reported with only its base procedure, do not pay separately for the base procedure. Payment for the base procedure is included in the payment for the other endoscopy.</p> <p>4 = Subject to 25% reduction of the TC diagnostic imaging (effective for services January 1, 2006 through June 30, 2010). Subject to 50% reduction of the TC diagnostic imaging (effective for services July 1, 2010 and after). Subject to 25% reduction of the PC of diagnostic imaging (effective for services January 1, 2012 through December 31, 2016). Subject to 5% reduction of the PC of diagnostic imaging (effective for services January 1, 2017 and after).</p> <p>5 = Subject to 20% reduction of the practice expense component for certain therapy services furnished in office and other non-institutional settings, and 25% reduction of the practice expense component for certain therapy services furnished in institutional settings (effective for services January 1, 2011 and after). Subject to 50% reduction of the practice expense component for certain therapy services furnished in both institutional and non-institutional settings (effective for services April 1, 2013 and after).</p> <p>6 = Subject to 25% reduction of the TC diagnostic cardiovascular services (effective for services January 1, 2013 and after).</p> <p>7 = Subject to 20% reduction of the TC diagnostic ophthalmology services (effective for services January 1, 2013 and after).</p> <p>9 = Concept does not apply.</p> |                   |
| <p>22</p> <p>Bilateral Surgery Indicator (Modifier 50)</p> <p>This field provides an indicator for services subject to a payment adjustment.</p> <p>0 = 150 percent payment adjustment for bilateral procedures does not apply. If procedure is reported with modifier -50 or with</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | <p>1 Pic (x)1</p> |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | LENGTH & PIC |
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| <p>modifiers RT and LT, base payment for the two sides on the lower of: (a) the total actual charge for both sides or (b) 100 percent of the fee schedule amount for a single code. Example: The fee schedule amount for code XXXXX is \$125. The physician reports code XXXXX-LT with an actual charge of \$100 and XXXXX-RT with an actual charge of \$100.</p> <p>Payment would be based on the fee schedule amount (\$125) since it is lower than the total actual charges for the left and right sides (\$200).</p> <p>The bilateral adjustment is inappropriate for codes in this category because of (a) physiology or anatomy or (b) because the code descriptor specifically states that it is a unilateral procedure and there is an existing code for the bilateral procedure.</p> <p>1 = 150 percent payment adjustment for bilateral procedures applies. If code is billed with the bilateral modifier or is reported twice on the same day by any other means (e.g., with RT and LT modifiers or with a 2 in the units field), base payment for these codes when reported as bilateral procedures on the lower of: (a) the total actual charge for both sides or (b) 150 percent of the fee schedule amount for a single code.</p> <p>If code is reported as a bilateral procedure and is reported with other procedure codes on the same day, apply the bilateral adjustment before applying any applicable multiple procedure rules.</p> <p>2 = 150 percent payment adjustment for bilateral procedure does not apply. RVUs are already based on the procedure being performed as a bilateral procedure. If procedure is reported with modifier -50 or is reported twice on the same day by any other means (e.g., with RT and LT modifiers with a 2 in the units field), base payment for both sides on the lower of (a) the total actual charges by the physician for both sides or (b) 100 percent of the fee schedule amount for a single code.</p> <p>Example: The fee schedule amount for code YYYYYY is \$125. The physician reports code YYYYYY-LT with an actual charge of \$100 and YYYYYY-RT with an actual charge of \$100. Payment would be based on the fee schedule amount (\$125) since it is lower than the total actual charges for the left and right sides (\$200).</p> <p>The RVUs are based on a bilateral procedure because: (a) the code descriptor specifically states that the procedure is bilateral; (b) the code descriptor states that the procedure may be performed either unilaterally or bilaterally; or (c) the procedure is usually performed as a bilateral procedure.</p> <p>3 = The usual payment adjustment for bilateral procedures does not apply. If procedure is reported with modifier -50 or is reported for both sides on the same day by any other means (e.g., with RT and</p> |              |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | LENGTH & PIC |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|
| <p>LT modifiers or with a 2 in the units field), base payment for each side or organ or site of a paired organ on the lower of: (a) the actual charge for each side or (b) 100% of the fee schedule amount for each side. If procedure is reported as a bilateral procedure and with other procedure codes on the same day, determine the fee schedule amount for a bilateral procedure before applying any applicable multiple procedure rules.</p> <p>Services in this category are generally radiology procedures or other diagnostic tests which are not subject to the special payment rules for other bilateral procedures.</p> <p>9 = Concept does not apply.</p> |              |
| <p>23</p> <p>Assistant at Surgery</p> <p>This field provides an indicator for services where an assistant at surgery is never paid for per IOM.</p> <p>0 = Payment restriction for assistants at surgery applies to this procedure unless supporting documentation is submitted to establish medical necessity.</p> <p>1 = Statutory payment restriction for assistants at surgery applies to this procedure. Assistant at surgery may not be paid.</p> <p>2 = Payment restriction for assistants at surgery does not apply to this procedure. Assistant at surgery may be paid.</p> <p>9 = Concept does not apply.</p>                                                  | 1 Pic (x)1   |
| <p>24</p> <p>Co-Surgeons (Modifier 62)</p> <p>This field provides an indicator for services for which two surgeons, each in a different specialty, may be paid.</p> <p>0 = Co-surgeons not permitted for this procedure.</p> <p>1 = Co-surgeons could be paid; supporting documentation required to establish medical necessity of two surgeons for the procedure.</p> <p>2 = Co-surgeons permitted; no documentation required if two specialty requirements are met.</p> <p>9 = Concept does not apply.</p>                                                                                                                                                             | 1 Pic (x)1   |
| <p>25</p> <p>Team Surgeons (Modifier 66)</p> <p>This field provides an indicator for services for which team surgeons may be paid.</p> <p>0 = Team surgeons not permitted for this procedure.</p> <p>1 = Team surgeons could be paid; supporting documentation required to establish medical necessity of a team; pay by report.</p>                                                                                                                                                                                                                                                                                                                                     | 1 Pic (x)1   |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | LENGTH & PIC  |
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| 2 = Team surgeons permitted; pay by report.<br>9 = Concept does not apply.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |               |
| 26<br>Filler                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | 1 Pic (x)1    |
| 27<br>Site of Service Differential<br>For 1999 and beyond, the site of service differential no longer applies. The following definitions will apply for all years after 1998:<br>0 = Facility pricing does not apply.<br>1 = Facility pricing applies.<br>9 = Concept does not apply.                                                                                                                                                                                                                                                                                                                                                                                   | 1 Pic (x)1    |
| 28<br>Non-Facility Fee Schedule Amount<br>This field shows the fee schedule amount for the non-facility setting. This amount equals Field 34.<br><b>Note:</b> Field 33 D indicates if an additional adjustment should be applied to this formula.<br>Non-Facility Pricing Amount for the File Year<br>$[(\text{Work RVU} * \text{Work GPCI}) + (\text{Non-Facility PE RVU} * \text{PE GPCI}) + (\text{MP RVU} * \text{MP GPCI})] * \text{Conversion Factor}$                                                                                                                                                                                                            | 9 Pic 9(7)v99 |
| 29<br>Facility Fee Schedule Amount<br>This field shows the fee schedule amount for the facility setting. This amount equals Field 35.<br><b>Note:</b> Field 33D indicates if an additional adjustment should be applied to this formula.<br>Facility Pricing Amount for the File Year<br>$[(\text{Work RVU} * \text{Work GPCI}) + (\text{Facility PE RVU} * \text{PE GPCI}) + (\text{MP RVU} * \text{MP GPCI})] * \text{Conversion Factor}$ Place of service codes to be used to identify facilities.<br>02 – Telehealth-Provided Other than in Patient’s Home.<br>19 – Off Campus-Outpatient Hospital<br>21 - Inpatient Hospital<br>22 – On Campus-Outpatient Hospital | 9 Pic 9(7)v99 |

| <b>FIELD # &amp; ITEM</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | <b>LENGTH &amp; PIC</b> |
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| 23 - Emergency Room - Hospital<br>24 - Ambulatory Surgical Center – In a Medicare approved ASC, for an approved procedure on the ASC list, Medicare pays the lower facility fee to physicians. Beginning with dates of service January 1, 2008, in a Medicare approved ASC, for procedures NOT on the ASC list of approved procedures, contractors will also pay the lower facility fee to physicians.<br>26 - Military Treatment Facility<br>31 - Skilled Nursing Facility<br>34 - Hospice<br>41 - Ambulance - Land<br>42 - Ambulance Air or Water<br>51 - Inpatient Psychiatric Facility<br>52 - Psychiatric Facility Partial Hospitalization<br>53 - Community Mental Health Center<br>56 - Psychiatric Residential Treatment Facility<br>61 - Comprehensive Inpatient Rehabilitation Facility |                         |
| 29A<br>Anti-markup Test Indicator<br>This field provides an indicator for Anti-markup Test HCPCS codes:<br>‘1’ = Anti-markup Test HCPCS.<br>‘9’ = Concept does not apply.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | 1 Pic x                 |
| 30<br>Record Effective Date<br>This field identifies the effective date for the MPFSDB record for each HCPCS. The field is in YYYYMMDD format.<br><br>NOTE: This is not the date the HCPCS code was created. It is the date the code was updated or added to the MPFSDB file for the current file year. This field is set to January 1 for all codes during the annual update process.                                                                                                                                                                                                                                                                                                                                                                                                            | 8 Pic x(8)              |
| 31<br>Filler                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | 28 Pic x(28)            |
| 31EE<br>Reduced therapy fee schedule amount                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | 9Pic(7)v99              |
| 31DD<br>Filler                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 1Pic x(2)               |

| <b>FIELD # &amp; ITEM</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | <b>LENGTH &amp; PIC</b> |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------|
| 31CC<br>Imaging Cap Indicator<br>A value of “1” means subject to OPPS payment cap determination.<br>A value of “9” means not subject to OPPS payment cap determination.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | 1Pic x(1)               |
| 31BB<br>Non-Facility Imaging Payment Amount                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | 9Pic(7)v99              |
| 31AA<br>Facility Imaging Payment Amount                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | 9Pic(7)v99              |
| 31A<br>Physician Supervision of Diagnostic Procedures<br>This field is for use in post payment review.<br>01 = Procedure must be performed under the general supervision of a physician.<br>02 = Procedure must be performed under the direct supervision of a physician.<br>03 = Procedure must be performed under the personal supervision of a physician.<br><br>(Diagnostic imaging procedures performed by a Registered Radiologist Assistant (RRA) who is certified and registered by The American Registry of Radiologic Technologists (ARRT) or a Radiology Practitioner Assistant (RPA) who is certified by the Certification Board for Radiology Practitioner Assistants (CBRPA), and is authorized to furnish the procedure under state law, may be performed under direct supervision.)<br><br>04 = Physician supervision policy does not apply when procedure is furnished by a qualified, independent psychologist or a clinical psychologist; otherwise must be performed under the general supervision of a physician.<br>05 = Not subject to supervision when furnished personally by a qualified audiologist, physician or non physician practitioner. Direct supervision by a physician is required for those parts of the test that may be furnished by a qualified technician when appropriate to the circumstances of the test.<br>06 = Procedure must be personally performed by a physician or a physical therapist (PT) who is certified by the American Board of Physical Therapy Specialties (ABPTS) as a qualified electrophysiological clinical specialist and is permitted to provide | 2 Pic x(2)              |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | LENGTH & PIC |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|
| <p>the procedure under State law. Procedure may also be performed by a PT with ABPTS certification without physician supervision.</p> <p>21 = Procedure may be performed by a technician with certification under general supervision of a physician; otherwise must be performed under direct supervision of a physician. Procedure may also be performed by a PT with ABPTS certification without physician supervision.</p> <p>22 = May be performed by a technician with on-line real-time contact with physician.</p> <p>66 = May be personally performed by a physician or by a physical therapist with ABPTS certification and certification in this specific procedure.</p> <p>6A = Supervision standards for level 66 apply; in addition, the PT with ABPTS certification may personally supervise another PT, but only the PT with ABPTS certification may bill.</p> <p>77 = Procedure must be performed by a PT with ABPTS certification (TC &amp; PC) or by a PT without certification under direct supervision of a physician (TC &amp; PC), or by a technician with certification under general supervision of a physician (TC only; PC always physician).</p> <p>7A = Supervision standards for level 77 apply; in addition, the PT with ABPTS certification may personally supervise another PT, but only the PT with ABPTS certification may bill.</p> <p>09 = Concept does not apply.</p> |              |
| <p>31B</p> <p>This field has been deleted to allow for the expansion of field 31A.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |              |
| <p>31C</p> <p>Facility Setting Practice Expense Relative Value Units</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 9 Pic(7)v99  |
| <p>31D</p> <p>Non-Facility Setting Practice Expense Relative Value Units</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | 9 Pic(7)v99  |
| <p>31E</p> <p>Filler</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 9 Pic(7)v99  |
| <p>31F</p> <p>Filler</p> <p>Reserved for future use.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 1 Pic x(1)   |
| <p>31G</p> <p>Endoscopic Base Codes</p> <p>This field identifies an endoscopic base code for each code with a multiple surgery indicator of 3.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | 5 Pic x(5)   |

| FIELD # & ITEM                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | LENGTH & PIC         |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------|
| <p>32A</p> <p>1996 Transition/Fee Schedule Amount</p> <p>This field is no longer applicable since transitioning ended in 1996. This field will contain a zero.</p>                                                                                                                                                                                                                                                                                                                                                                                              | <p>9 Pic 9(7)v99</p> |
| <p>32B</p> <p>1996 Transition/Fee Schedule</p> <p>This field is no longer applicable since transitioning ended in 1996. This field will contain spaces.</p>                                                                                                                                                                                                                                                                                                                                                                                                     | <p>1 Pic x(1)</p>    |
| <p>32C</p> <p>1996 Transition/Fee Schedule Amount When Site or Service Differential Applies</p> <p>This field is no longer applicable since transitioning ended in 1996. This field will contain a zero.</p>                                                                                                                                                                                                                                                                                                                                                    | <p>9 Pic 9(7)v99</p> |
| <p>33A</p> <p>Units Payment Rule Indicator</p> <p>Reserved for future use.</p> <p>9 = Concept does not apply.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                               | <p>1 Pic x(1)</p>    |
| <p>33B</p> <p>Mapping Indicator</p> <p>This field is no longer applicable since transitioning ended in 1996. This field will contain spaces.</p>                                                                                                                                                                                                                                                                                                                                                                                                                | <p>1 Pic x(1)</p>    |
| <p>33C</p> <p>Anti-markup Locality—Informational Use—Locality used for reporting utilization of anti-markup services.</p> <p>NOT FOR A/B MAC (B) USE: These Medicare Advantage encounter pricing localities are for Shared System Maintainer purposes only. The locality values were developed to facilitate centralized processing of encounter data by the Medicare Advantage organizations.</p>                                                                                                                                                              | <p>2 Pic x(2)</p>    |
| <p>33D</p> <p>Calculation Flag</p> <p>This field is informational only; the SSMs do not need to add this field. The intent is to assist A/B MACs (B) to understand how the fee schedule amount in fields 28 and 29 are calculated. The MMA mandates an additional adjustment to selected HCPCS codes. A value of “1” indicates an additional fee schedule adjustment of 1.32 in 2004 and 1.03 in 2005. A value of “0” indicates no additional adjustment needed. A value of “2” indicates an additional fee schedule adjustment of 1.05 effective 7/1/2008.</p> | <p>1 Pic x(1)</p>    |

| <b>FIELD # &amp; ITEM</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | <b>LENGTH &amp; PIC</b> |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------|
| 33 E<br>Diagnostic Imaging Family Indicator<br>For services effective January 1, 2011, and after, family indicators 01 - 11 will not be populated.<br>01 = Family 1 Ultrasound (Chest/Abdomen/Pelvis – Non Obstetrical<br>02 = Family 2 CT and CTA (Chest/Thorax/Abd/Pelvis)<br>03 = Family 3 CT and CTA (Head/Brain/Orbit/Maxillofacial/Neck)<br>04 = Family 4 MRI and MRA (Chest/Abd/Pelvis)<br>05 = Family 5 MRI and MRA (Head/Brain/Neck)<br>06 = Family 6 MRI and MRA (spine)<br>07 = Family 7 CT (spine)<br>08 = Family 8 MRI and MRA (lower extremities)<br>09 = Family 9 CT and CTA (lower extremities)<br>10 = Family 10 Mr and MRI (upper extremities and joints)<br>11 = Family 11 CT and CTA (upper extremities)<br>88 = Subject to the reduction of the TC diagnostic imaging (effective for services January 1, 2011, and after). Subject to the reduction of the PC diagnostic imaging (effective for services January 1, 2012 and after).<br>99 = Concept Does Not Apply | 2Pic x(2)               |
| 33F<br>Performance Payment Indicator<br>(For future use)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | 1 Pic x (1)             |
| 33G<br>National Level Future Expansion                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | 3 Pic x (3)             |
| 34<br>Non-Facility Fee Schedule Amount<br>This field replicates field 28.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | 9 Pic 9(7)v99           |
| 35<br>Facility Fee Schedule Amount<br>This field replicates field 29.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | 9 Pic 9(7)v99           |
| 36<br>Filler                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | 1 Pic x(1)              |
| 37                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | 7 Pic x(7)              |

| FIELD # & ITEM                                                                                                                                                                                                                                                                   | LENGTH & PIC |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|
| <p>Future Local Level Expansion**</p> <p>The Updated 1992 Transition Amount was previously stored in this field. A/B MACs (B) can continue to maintain the updated transition amount in this field.</p>                                                                          |              |
| <p>38A</p> <p>Future Local Level Expansion**</p> <p>The adjusted historical payment basis (AHPB) was previously stored in this field. A/B MACs (B) can continue to maintain the AHPB in this field.</p>                                                                          | 7 Pic x(7)   |
| <p>38 B</p> <p>Filler</p> <p>This field was originally established for 15 spaces. Since AHPB data will only use 7 of the 15 spaces, A/B MACs (B) have 8 remaining spaces for their purposes.</p> <p>** These fields will be appended by each A/B MAC (B) at the local level.</p> | 8 Pix x(8)   |

## Transmittals Issued for this Chapter

| Rev #                           | Issue Date | Subject                                                                                                                                                                                                               | Impl Date  | CR#   |
|---------------------------------|------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------|-------|
| <a href="#"><u>R13701CP</u></a> | 06/16/2026 | Update to Several Sections of the Internet-Only Manual (IOM) Publication (Pub.) 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements                           | 07/16/2026 | 14428 |
| <a href="#"><u>R13610CP</u></a> | 01/30/2026 | Calendar Year 2026 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                    | 01/05/2026 | 14326 |
| <a href="#"><u>R13519CP</u></a> | 12/19/2025 | Calendar Year 2026 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule-Rescinded and replaced by Transmittal 10233                                                        | 01/05/2026 | 14326 |
| <a href="#"><u>R13216CP</u></a> | 05/09/2025 | Update to Several Sections of the Internet-Only Manual (IOM) Publication (Pub.) 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements                           | 06/06/2025 | 14048 |
| <a href="#"><u>R13150CP</u></a> | 04/11/2025 | Internet Only Manual (IOM) Update to 100-04 Chapter 13, Section 90.3 - Transportation Component (HCPCS Codes R0070 - R0075) and Chapter 23, Section 30.5 - Payment Amounts for Portable X-Ray Transportation Services | 05/12/2025 | 13909 |
| <a href="#"><u>R12991CP</u></a> | 12/13/2024 | Calendar Year 2025 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                    | 01/06/2025 | 13888 |
| <a href="#"><u>R12823CP</u></a> | 09/05/2024 | Update to Internet-Only Manual Publication (Pub.) 100-04, Chapters 12 and 23, to Update Place of Service (POS) Code Descriptions to Match the POS Descriptions Found in Pub. 100-04, Chapter 26                       | 10/08/2024 | 13744 |
| <a href="#"><u>R12645CP</u></a> | 05/16/2024 | Update to Pub. 100-04 Medicare Claims Processing Manual, Chapter 23, Section 60.3 Gap-filling Durable Medical Equipment Prosthetics Orthotics Supplies (DMEPOS) Fees                                                  | 06/17/2024 | 13617 |

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| <u>R12601CP</u> | 05/02/2024        | Update to Several Sections of the Internet-only Manual (IOM) Publication (Pub.) 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements                                                 | 06/04/2024       | 13599      |
| <u>R12573CP</u> | 04/09/2024        | Manual Update to Section 20.7 in Chapter 23 of Publication (Pub) 100-04                                                                                                                                                                     | 08/10/2020       | 11833      |
| <u>R12398CP</u> | 12/07/2023        | Calendar Year 2024 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                          | 01/02/2024       | 13463      |
| <u>R12326CP</u> | 10/26/2023        | Internet-Only Manual Update, Pub. 100-04, Chapter 23 (Fee Schedule Administration and Coding Requirements), Section 50.6                                                                                                                    | 01/29/2024       | 13410      |
| <u>R12068CP</u> | 06/02/2023        | July Quarterly Update for 2023 Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                         | 07/03/2023       | 13235      |
| <u>R11630CP</u> | 10/06/2022        | Instructions to the Fiscal Intermediary Shared System [FISS] to Add Additional Multiple Procedure Indicators 6 and 7 Into the Physician Fee Schedule Payment Policy Indicator File Record Layout                                            | 04/03/2023       | 12871      |
| <u>R11137CP</u> | 12/02/2021        | Calendar Year 2022 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                          | 01/03/2022       | 12521      |
| <u>R10878CP</u> | 07/15/2021        | Update to the Internet-only Manual (IOM) Publication (Pub.) 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements, Section 20.9 - Fee Schedule Administration and Coding Requirements | 08/16/2021       | 12311      |

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| <u>R10504CP</u> | 12/04/2020        | CY 2021 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                                                      | 01/04/2021       | 12063      |
| <u>R10320CP</u> | 08/28/2020        | Updates to Chapter 23 - Fee Schedule Administration and Coding Requirements                                                                                                                                                                                  | 12/01/2020       | 11941      |
| <u>R10356CP</u> | 09/18/2020        | Update to the Medicare Claims Processing Manual                                                                                                                                                                                                              | 10/19/2020       | 11958      |
| <u>R10233CP</u> | 07/24/2020        | Update to the (IOM) Publication 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements, Section 20.9 - Fee Schedule Administration and Coding Requirements                                              | 06/16/2020       | 11782      |
| <u>R10211CP</u> | 07/10/2020        | Manual Update to Section 20.7 in Chapter 23 of Publication (Pub) 100-04                                                                                                                                                                                      | 08/10/2020       | 11833      |
| <u>R10136CP</u> | 05/15/2020        | Update to the (IOM) Publication 100-04, Medicare Claims Processing Manual, Chapter 23 - Fee Schedule Administration and Coding Requirements, Section 20.9 - Fee Schedule Administration and Coding Requirements– Rescinded and replaced by Transmittal 10233 | 06/16/2020       | 11782      |
| <u>R4465CP</u>  | 11/15/2019        | Medicare Claims Processing Manual Chapter 23 - Fee Schedule Administration and Coding Requirements                                                                                                                                                           | 01/30/2019       | 10868      |
| <u>R4418CP</u>  | 10/18/2019        | Medicare Physician Fee Schedule Database (MPFSDB) Update to Status Indicators                                                                                                                                                                                | 11/19/2019       | 11453      |
| <u>R4298CP</u>  | 05/03/2019        | Medicare Physician Fee Schedule Database (MPFSDB) File Record Layout                                                                                                                                                                                         | 10/07/2019       | 11191      |
| <u>R4188CP</u>  | 12/28/2018        | Medicare Claims Processing Manual Chapter 23 - Fee Schedule Administration                                                                                                                                                                                   | 01/30/2019       | 10868      |

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|                |                   | and Coding Requirements- Rescinded and replaced by Transmittal 4465                                                                                           |                  |            |
| <u>R4131CP</u> | 09/14/2018        | Updates to Chapter 23 Fee File Instructions                                                                                                                   | 12/17/2018       | 10926      |
| <u>R4130CP</u> | 09/14/2018        | Update to the Medicare Claims Processing Manual, Chapter 23, Section 60.3                                                                                     | 10/15/2018       | 10924      |
| <u>R3931CP</u> | 12/01/2017        | CY 2018 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                       | 01/02/2018       | 10395      |
| <u>R3903CP</u> | 11/03/2017        | Annual Medicare Physician Fee Schedule (MPFS) Files Delivery and Implementation and Medicare Physician Fee Schedule Database (MPFSDB) 2018 File Layout Manual | 01/02/2018       | 10218      |
| <u>R3876CP</u> | 10/06/2017        | Decommission the MCS Maintained HBCRB081 Report (“Correct Coding Quarterly Savings Report”)                                                                   | 04/02/2018       | 10284      |
| <u>R3721CP</u> | 02/24/2017        | Updates to Pub. 100-04, Chapters 12, 17 and 23 to Correct Remittance Advice Messages                                                                          | 05/25/2017       | 9906       |
| <u>R3693CP</u> | 01/13/2017        | Medicare Physician Fee Schedule Database (MPFSDB) 2017 File Layout Manual                                                                                     | 01/03/2017       | 9784       |
| <u>R3671CP</u> | 12/05/2016        | CY 2017 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                       | 01/03/2016       | 9854       |
| <u>R3613CP</u> | 09/16/2016        | Medicare Physician Fee Schedule Database (MPFSDB) 2017 File Layout Manual – Rescinded and replaced by Transmittal 3693.                                       | 01/03/2017       | 9784       |

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| <u>R3416CP</u> | 11/23/2015        | CY 2016 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                                    | 01/04/2016       | 9431       |
| <u>R3371CP</u> | 10/09/2015        | Medicare Physician Fee Schedule Database (MPFSDB) 2016 File Layout Manual                                                                                                                                                                  | 01/04/2016       | 9346       |
| <u>R3350CP</u> | 09/11/2015        | Implementation of Adjusted Durable Medical Equipment Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule Amounts Using Information from the National Competitive Bidding Program (CBP)                                              | 01/04/2016       | 9239       |
| <u>R3316CP</u> | 08/07/2015        | Implementation of Adjusted Durable Medical Equipment Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule Amounts Using Information from the National Competitive Bidding Program (CBP) – Rescinded and replaced by Transmittal 3350 | 01/04/2016       | 9239       |
| <u>R3190CP</u> | 02/06/2015        | CY 2015 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                                                                                                                    | 01/05/2015       | 8999       |
| <u>R3144CP</u> | 12/05/2014        | Medicare Physician Fee Schedule Database (MPFSDB) 2015 File Layout Manual                                                                                                                                                                  | 01/05/2015       | 8958       |
| <u>R3129CP</u> | 11/21/2014        | CY 2015 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule – Rescinded and replaced by Transmittal 3190                                                                                       | 01/05/2015       | 8999       |
| <u>R3124CP</u> | 11/13/2014        | 2015 Healthcare Common Procedure Coding System (HCPCS) Annual Update Reminder                                                                                                                                                              | 01/05/2014       | 8975       |

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| <u>R3100CP</u> | 10/24/2014        | 2015 Healthcare Common Procedure Coding System (HCPCS) Annual Update Reminder – Rescinded and replaced by Transmittal 3124               | 01/05/2014                    | 8975       |
| <u>R3093CP</u> | 10/10/2014        | Medicare Physician Fee Schedule Database (MPFSDB) 2015 File Layout Manual – Rescinded and replaced by Transmittal 3144                   | 01/05/2015                    | 8958       |
| <u>R3081CP</u> | 09/26/2014        | Update to Pub. 100-04, Chapter 23 to Provide Language-Only Changes for Conversion to ICD-10                                              | Upon Implementation of ICD-10 | 8692       |
| <u>R3020CP</u> | 08/08/2014        | Update to Pub. 100-04, Chapter 23 to Provide Language-Only Changes for Conversion to ICD-10 – Rescinded and replaced by Transmittal 3081 | Upon Implementation of ICD-10 | 8692       |
| <u>R2902CP</u> | 03/11/2014        | April Quarterly Update for 2014 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule                    | 04/07/2014                    | 8645       |
| <u>R2893CP</u> | 02/28/2014        | April Quarterly Update for 2014 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule                    | 04/07/2014                    | 8645       |
| <u>R2837CP</u> | 12/13/2013        | Change to the Reasonable Charge Update for 2014 for Splints, Casts and Certain Intraocular Lenses                                        | 01/06/2014                    | 8523       |
| <u>R2836CP</u> | 01/06/2013        | CY 2014 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                  | 01/06/2014                    | 8531       |
| <u>R2790CP</u> | 09/20/2013        | Medicare Physician Fee Schedule Database (MPFSDB) 2014 File Layout Manual                                                                | 01/06/2014                    | 8455       |

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| <u>R2783CP</u> | 09/10/2013        | Corrections to the Medicare Claims Processing Manual                                                                                                 | 09/17/2013       | 8343       |
| <u>R2725CP</u> | 06/14/2013        | Corrections to the Medicare Claims Processing Manual – Rescinded and replaced by Transmittal 2783                                                    | 09/17/2013       | 8343       |
| <u>R2709CP</u> | 05/17/2013        | July Quarterly Update for 2013 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule                                 | 07/01/2013       | 8325       |
| <u>R2682CP</u> | 04/05/2013        | Quarterly Update for the Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Competitive Bidding Program (CBP) - July 2013       | 07/01/2013       | 8232       |
| <u>R2643CP</u> | 01/31/2013        | Streamlining the Process for Updating the Abstract Files Used to Price Institutional Claims                                                          | 07/01/2013       | 8128       |
| <u>R2632CP</u> | 01/11/2013        | CY 2013 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                                              | 01/07/2013       | 8133       |
| <u>R2608CP</u> | 12/07/2012        | CY 2013 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule – Rescinded and replaced by Transmittal 2632 | 01/07/2013       | 8133       |
| <u>R2566CP</u> | 10/12/2012        | Medicare Physician Fee Schedule Database (MPFSDB) 2013 File Layout Manual                                                                            | 01/07/2013       | 8064       |
| <u>R2549CP</u> | 09/21/2012        | Medicare Physician Fee Schedule Database (MPFSDB) 2013 File Layout Manual – Rescinded and replaced by Transmittal 2566                               | 01/07/2013       | 8064       |

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| <u>R2340CP</u> | 11/04/2011        | CY 2012 Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule                 | 01/03/2012       | 7635       |
| <u>R2288CP</u> | 08/26/2011        | Establishing a Quarterly Recurring Update Notification Process for Temporary “K” and “Q” Codes                          | 01/03/2012       | 7493       |
| <u>R2287CP</u> | 08/26/2011        | Medicare Physician Fee Schedule Database (MPFSDB) 2012 File Layout Manual                                               | 01/03/2012       | 7554       |
| <u>R2236CP</u> | 06/03/2011        | July Quarterly Update for 2011 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule    | 07/05/2011       | 7416       |
| <u>R2148CP</u> | 02/04/2011        | Auto Denial of Claim Line(s) Items Submitted With a GZ Modifier                                                         | 07/05/2011       | 7228       |
| <u>R2119CP</u> | 12/14/2010        | Medicare Physician Fee Schedule Database (MPFSDB) 2011 File Layout Manual                                               | 01/03/2011       | 7243       |
| <u>R2101CP</u> | 11/19/2010        | Medicare Physician Fee Schedule Database (MPFSDB) 2011 File Layout Manual – Rescinded and replaced by Transmittal 2119  | 01/03/2011       | 7243       |
| <u>R2088CP</u> | 11/05/2010        | January 2011 Quarterly Update for the DMEPOS Competitive Bidding Program                                                | 01/03/2011       | 7181       |
| <u>R2006CP</u> | 07/23/2010        | October Quarterly Update for 2010 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) Fee Schedule | 10/04/2010       | 7070       |
| <u>R1853CP</u> | 11/13/2009        | CY 2010 Fee Schedule Update for Durable Medical Equipment, Prosthetics, Orthotics and Supplies (DMEPOS) Fee Schedule    | 01/04/2010       | 6720       |

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| <u>R1825CP</u> | 10/08/2009        | Medicare Physician Fee Schedule Database (MPFSDB) 2010 File Layout                                                                                                                           | 01/04/2010                | 6648       |
| <u>R1822CP</u> | 10/02/2009        | Medicare Physician Fee Schedule Database (MPFSDB) 2010 File Layout – Rescinded and replaced by Transmittal 1825                                                                              | 01/04/2010                | 6648       |
| <u>R1717CP</u> | 04/24/2009        | Speech-Language Pathology Practice Payment Policy                                                                                                                                            | 07/06/2009                | 6381       |
| <u>R1709CP</u> | 04/03/2009        | Manualization of the Medicare Physician Fee Schedule (MPFS) Record Layouts for Contractors Processing Institutional Claims                                                                   | 07/06/2009                | 6443       |
| <u>R1644CP</u> | 12/05/2008        | 2008 Jurisdiction List for Durable Medical Equipment Prosthetics, Orthotics, and Supply Healthcare Common Procedure Coding System (HCPCS) Codes                                              | 10/27/2008 and 12/12/2008 | 6062       |
| <u>R1632CP</u> | 11/07/2008        | Medicare Physician Fee Schedule Database (MPFSDB) 2009 File Layout                                                                                                                           | 01/05/2009                | 6227       |
| <u>R1630CP</u> | 11/07/2008        | Fee Schedule Update for 2009 for Durable Medical Equipment, Prosthetics, Orthotics and Supplies                                                                                              | 01/05/2009                | 6270       |
| <u>R1605CP</u> | 09/26/2008        | 2008 Jurisdiction List for Durable Medical Equipment Prosthetics, Orthotics, and Supply Healthcare Common Procedure Coding System (HCPCS) Codes – Rescinded and replaced by Transmittal 1644 | 10/27/2008                | 6062       |
| <u>R1535CP</u> | 06/13/2008        | July 2008 Quarterly Update for the DMEPOS Competitive Bidding Program                                                                                                                        | 07/07/2008                | 6101       |
| <u>R1472CP</u> | 03/06/2008        | Update of Institutional Claims References                                                                                                                                                    | 04/07/2008                | 5893       |

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| <u>R1421CP</u> | 01/25/2008        | Update of Institutional Claims References - Rescinded and Replaced by Transmittal 1472                                                                              | 04/07/2008       | 5893       |
| <u>R1358CP</u> | 10/26/2007        | Medicare Physician Fee Schedule Database (MPFSDB) 2008 File                                                                                                         | 01/07/2008       | 5774       |
| <u>R1005CP</u> | 07/21/2006        | Medicare Physician Fee Schedule Database (MPFSDB) 2007 File Layout                                                                                                  | 01/02/2007       | 5206       |
| <u>R823CP</u>  | 02/01/2006        | New Temporary Code for Battery for Power Mobility Devices                                                                                                           | 07/03/2006       | 4253       |
| <u>R614CP</u>  | 07/22/2005        | Medicare Physician Fee Schedule Database (MPFSDB) 2006 File Layout                                                                                                  | 01/03/2006       | 3889       |
| <u>R341CP</u>  | 10/29/2004        | Implementation of the Medicare Physician Fee Schedule (MPFS) National Abstract File for Purchased Diagnostic Tests and Interpretations                              | 04/04/2005       | 3481       |
| <u>R323CP</u>  | 10/22/2004        | Update Regarding the Use of American Dental Association's (ADA) Current Dental Terminology (CDT) Codes on Medicare Contractors Web Sites and Other Electronic Media | 11/22/2004       | 3499       |
| <u>R286CP</u>  | 08/27/2004        | Medicare Physician Fee Schedule Database (MPFSDB) 2005 File Layout                                                                                                  | 01/03/2005       | 3421       |
| <u>R283CP</u>  | 08/27/2004        | 2005 Healthcare Common Procedure Coding System (HCPCS) Annual Update Reminder                                                                                       | 01/03/2005       | 3422       |
| <u>R236CP</u>  | 07/23/2004        | 2005 DMEPOS Pricing File Record Layout Expansion and New Pricing Procedures for Certain DMEPOS Items Based on Modifiers                                             | 01/03/2005       | 3300       |

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| <u>R210CP</u> | 06/18/2004        | CMS Annual Reminder to Contractors of The ICD-9-CM Update for October 1, 2004                                                                                                                                                               | 10/04/2004       | 3303       |
| <u>R127CP</u> | 03/26/2004        | Annual Spreadsheet Containing an Updated List of HCPCS Codes for Durable Medical Equipment Regional Carrier (DMERC) and Part B Local Carrier Jurisdictions to Reflect Codes That Have Been Added or Discontinued (Deleted) During Each Year | 05/26/2004       | 3139       |
| <u>R126CP</u> | 03/26/2004        | ICD-9 Coding for Beneficiary Claims                                                                                                                                                                                                         | N/A              | 2857       |
| <u>R095CP</u> | 02/06/2004        | Eliminate The 90-Day Grace Period for Billing Discontinued ICD-9-CM Codes                                                                                                                                                                   | 10/01/2004       | 3094       |
| <u>R089CP</u> | 02/06/2004        | Eliminating The 90-Day Grace Period for Billing Discontinued HCPCS Codes                                                                                                                                                                    | 07/06/2004       | 3093       |
| <u>R066CP</u> | 01/16/2004        | Quarterly Update to Correct Coding Initiative (CCI) Edits, Version 10 1, Effective April 1, 2004                                                                                                                                            | 04/05/2004       | 2997       |
| <u>R050CP</u> | 12/19/2003        | Clarification That Medicare Contractors Will Be Receiving Quarterly Updates for Temporary HCPCS Codes                                                                                                                                       | 04/05/2004       | 2967       |
| <u>R043CP</u> | 12/19/2003        | Using the American Dental Association's (ADA's) Current Dental Terminology-Fourth Edition (CDT-4) Codes of Contractors Web Sites and Other Electronic Media.                                                                                | 01/20/2004       | 3003       |
| <u>R037CP</u> | 12/08/2003        | Adding place of service (POS) code 42 to Field 29 of MFSDB; and 38B, and adding the year 2004 to field 8 Conversion Factor                                                                                                                  | 01/05/2004       | 2970       |
| <u>R001CP</u> | 10/01/2003        | Initial Publication of Manual                                                                                                                                                                                                               | NA               | NA         |

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